#1 · Assessment and Diagnosis

Which findings in a client with low back pain suggest cauda equina syndrome, and what is the first action?

Answer

Saddle anesthesia, new bladder retention or overflow incontinence, bowel dysfunction, and progressive bilateral leg weakness. This is a surgical emergency, so stop the examination and arrange immediate emergency referral.

#2 · Assessment and Diagnosis

Which history items raise suspicion of a spinal fracture as the cause of low back pain?

Answer

Major trauma, minor trauma in an older or osteoporotic client, prolonged corticosteroid use, and age over 70. Fracture risk rises with each item, so screen for these before loading the spine.

#3 · Assessment and Diagnosis

List the screening red flags for spinal malignancy in a client with low back pain.

Answer

Past history of cancer, unexplained weight loss, age over 50, constant night pain unrelieved by rest or position, and failure to improve after about one month of appropriate care. Several together warrant medical referral.

#4 · Assessment and Diagnosis

Which questions screen for spinal infection in a client with low back pain?

Answer

Ask about fever or chills, a recent bacterial infection, intravenous drug use, immunosuppression, and recent spinal surgery or injection. Constant pain with these features needs prompt medical assessment.

#5 · Assessment and Diagnosis

Which presentation of back pain should make you suspect an abdominal aortic aneurysm?

Answer

An older male smoker with deep, throbbing, non-mechanical back or abdominal pain, possibly with a pulsatile abdominal mass. Movement does not change the pain. Refer urgently, because rupture is life-threatening.

#6 · Assessment and Diagnosis

Match the lumbar myotomes to their test movements: L2, L3, L4, L5, S1.

Answer

L2 hip flexion, L3 knee extension, L4 ankle dorsiflexion, L5 great toe extension, S1 ankle plantarflexion (or heel raise). Test bilaterally and compare for subtle weakness.

#7 · Assessment and Diagnosis

Which deep tendon reflexes assess the lumbosacral nerve roots, and what root does each reflect?

Answer

The patellar reflex reflects L3 and L4, and the Achilles reflex reflects S1 (S1 and S2). A reduced reflex supports a root lesion at that level, but absence can be normal in older adults.

#8 · Assessment and Diagnosis

What is a positive straight leg raise, and what does it suggest?

Answer

Reproduction of the client's leg symptoms between roughly 30 and 70 degrees of hip flexion, increased by dorsiflexion. It is sensitive but not specific for lumbar disc herniation with nerve root irritation. A positive crossed test is more specific.

#9 · Assessment and Diagnosis

Describe the slump test and what makes it positive.

Answer

The client sits slumped, flexes the neck, then extends the knee and dorsiflexes the ankle. It is positive when symptoms are reproduced and ease when neck flexion is released, indicating neural tissue mechanosensitivity.

#10 · Assessment and Diagnosis

Which skin areas are used to test L4, L5 and S1 dermatomes?

Answer

L4 the medial leg and medial malleolus, L5 the dorsum of the foot including the great toe web space, S1 the lateral border of the foot. Compare light touch side to side.

#11 · Assessment and Diagnosis

Under the Canadian C-spine rule, which three high-risk factors mandate radiography?

Answer

Age 65 or older, a dangerous mechanism of injury, or paresthesias in the extremities. If any is present, radiography is required before cervical range of motion is assessed.

#12 · Assessment and Diagnosis

Which low-risk factors allow safe cervical range-of-motion assessment under the Canadian C-spine rule?

Answer

Simple rear-end collision, sitting in the emergency department, ambulatory at any time, delayed onset of neck pain, and no midline cervical tenderness. If present, test whether the client can actively rotate 45 degrees each way.

#13 · Assessment and Diagnosis

What are the four items of the Wainner cluster for cervical radiculopathy?

Answer

Positive Spurling A test, positive distraction test, positive upper limb tension test A, and ipsilateral cervical rotation under 60 degrees. Three of four positive raises probability, and all four positive makes it high.

#14 · Assessment and Diagnosis

Name the symptoms that suggest cervical arterial dysfunction and the correct response.

Answer

The 5 Ds and 3 Ns: dizziness, diplopia, dysarthria, dysphagia, drop attacks, nausea, numbness, nystagmus, plus sudden severe unfamiliar headache or neck pain. Stop treatment and arrange urgent medical assessment.

#15 · Assessment and Diagnosis

Which upper motor neuron signs support cervical myelopathy?

Answer

Hyperreflexia, positive Hoffmann sign, inverted supinator sign, clonus, positive Babinski, gait disturbance and hand clumsiness. These signs indicate spinal cord compression and call for prompt medical referral.

#16 · Assessment and Diagnosis

How does a costovertebral joint or rib dysfunction typically present?

Answer

Localized unilateral thoracic or rib pain that is sharp with deep inspiration, coughing or trunk rotation, with tenderness over the joint and reproduction on rib springing. Rule out cardiopulmonary causes first.

#17 · Assessment and Diagnosis

Which features make thoracic or chest pain suspicious for a cardiac cause?

Answer

Pressure or heaviness that spreads to the left arm or jaw, brought on by exertion, with sweating, nausea or shortness of breath, and unchanged by movement or palpation. Call emergency services without delay.

#18 · Assessment and Diagnosis

Describe the rib motion pattern of the upper ribs versus the lower ribs during breathing.

Answer

Upper ribs (1 to 6) move in a pump-handle pattern, increasing anteroposterior chest diameter. Lower ribs (7 to 10) move in a bucket-handle pattern, increasing transverse diameter. Ribs 11 and 12 move in a caliper pattern.

#19 · Assessment and Diagnosis

Which visceral condition refers pain to the right shoulder or interscapular area and relates to meals?

Answer

Gallbladder disease. Pain follows fatty meals, may come with nausea, and does not change with spinal movement or palpation. A mechanical examination that fails to reproduce the pain should prompt medical referral.

#20 · Assessment and Diagnosis

What is the commonly cited normal range of thoracic kyphosis and a rigid, severe kyphosis in an adolescent suggests what?

Answer

Normal thoracic kyphosis is commonly cited as about 20 to 40 degrees. A rigid, angular kyphosis that does not correct on extension in an adolescent suggests Scheuermann disease.

#21 · Assessment and Diagnosis

What does a painful arc suggest in shoulder assessment?

Answer

Pain during mid-range arm elevation, roughly 60 to 120 degrees of abduction, that eases above and below that range. It suggests subacromial pain syndrome, with compression of the rotator cuff or bursa under the acromion.

#22 · Assessment and Diagnosis

How is the empty can (Jobe) test performed and which muscle does it test?

Answer

The arm is elevated to 90 degrees in the scapular plane, thumb pointing down, then resisted downward pressure is applied. Pain or weakness suggests supraspinatus tendinopathy or tear.

#23 · Assessment and Diagnosis

What does a positive drop arm test indicate?

Answer

The client cannot lower the abducted arm slowly from 90 degrees or the arm drops suddenly. It indicates a large full-thickness rotator cuff tear, most often involving supraspinatus.

#24 · Assessment and Diagnosis

What is the Park cluster for subacromial impingement?

Answer

Three tests: positive Hawkins-Kennedy test, positive painful arc, and weakness on resisted external rotation (infraspinatus test). When all three are positive, the likelihood of subacromial impingement rises substantially.

#25 · Assessment and Diagnosis

Which tests assess infraspinatus, teres minor and subscapularis?

Answer

Resisted external rotation with the arm at the side tests infraspinatus. The Hornblower sign tests teres minor. The lift-off test or belly press test assesses subscapularis.

#26 · Assessment and Diagnosis

What is the capsular pattern of the glenohumeral joint?

Answer

External rotation is most limited, then abduction, then internal rotation. A capsular pattern with painful, restricted passive movement suggests adhesive capsulitis or joint arthritis rather than a muscle problem.

#27 · Assessment and Diagnosis

Describe the stages and typical client for adhesive capsulitis.

Answer

Painful freezing stage, then stiff frozen stage, then thawing stage. It is most common in people aged 40 to 60, more often women, and is strongly associated with diabetes.

#28 · Assessment and Diagnosis

How do the apprehension and relocation tests identify anterior shoulder instability?

Answer

Supine, the arm is placed in 90 degrees abduction and external rotation, and apprehension is a positive sign. A posteriorly directed force then reduces the apprehension, which is the positive relocation test.

#29 · Assessment and Diagnosis

What does a positive sulcus sign indicate?

Answer

A visible gap below the acromion when inferior traction is applied to the arm at the side. It indicates inferior glenohumeral laxity and is a hallmark of multidirectional instability.

#30 · Assessment and Diagnosis

How is the O'Brien active compression test performed for a SLAP lesion?

Answer

The arm is flexed to 90 degrees, adducted 10 degrees, and internally rotated with the thumb down. Resisted elevation causes pain that is reduced with the palm up. Diagnostic accuracy is limited, so interpret with the history.

#31 · Assessment and Diagnosis

Which findings support lateral epicondylalgia (tennis elbow)?

Answer

Pain over the lateral epicondyle on resisted wrist and finger extension (Cozen test) or gripping, with local tenderness over the common extensor origin. The extensor carpi radialis brevis tendon is most often involved.

#32 · Assessment and Diagnosis

List the five items of the Wainner carpal tunnel syndrome cluster.

Answer

Shaking the hands relieves symptoms, wrist ratio index over 0.67, symptom severity scale over 1.9, reduced median sensation in the thumb, and age over 45. Four of five positive makes carpal tunnel syndrome likely.

#33 · Assessment and Diagnosis

Which structures are involved in De Quervain tenosynovitis and what test is used?

Answer

Abductor pollicis longus and extensor pollicis brevis in the first dorsal compartment. Pain at the radial styloid is reproduced by the Finkelstein test: thumb in the fist, then ulnar deviation of the wrist.

#34 · Assessment and Diagnosis

What is Froment sign and which nerve does it indicate?

Answer

When asked to hold a sheet of paper between thumb and index finger, the client flexes the thumb interphalangeal joint to compensate. It shows weak adductor pollicis and indicates an ulnar nerve lesion.

#35 · Assessment and Diagnosis

How is Phalen test performed and what is a positive result?

Answer

The wrists are held in full flexion, backs of the hands together, for up to 60 seconds. Reproduction of numbness or tingling in the median nerve distribution suggests carpal tunnel syndrome.

#36 · Assessment and Diagnosis

How is the FADIR test performed and how should a positive result be interpreted?

Answer

The hip is flexed, adducted and internally rotated. Groin pain is a positive result. The test is sensitive for femoroacetabular impingement or intra-articular pathology but not specific, so correlate with the rest of the examination.

#37 · Assessment and Diagnosis

What is the typical capsular pattern of hip osteoarthritis?

Answer

Internal rotation is limited first and most, followed by flexion and abduction. A painful, stiff hip with restricted internal rotation in an older client suggests osteoarthritis.

#38 · Assessment and Diagnosis

How do cam and pincer femoroacetabular impingement differ?

Answer

Cam impingement is a bump at the femoral head-neck junction, typically in young active males. Pincer impingement is acetabular overcoverage, more often in middle-aged women. Many people have a combined pattern.

#39 · Assessment and Diagnosis

What does a positive Trendelenburg sign show?

Answer

When standing on one leg, the pelvis drops on the opposite, unsupported side. It indicates weakness of the gluteus medius on the stance side, or a superior gluteal nerve problem.

#40 · Assessment and Diagnosis

Which findings support greater trochanteric pain syndrome?

Answer

Lateral hip pain with tenderness over the greater trochanter, pain lying on the affected side, and pain reproduced by single leg stance held for about 30 seconds. Gluteal tendinopathy is a common cause.

#41 · Assessment and Diagnosis

Why is the Lachman test preferred for assessing the anterior cruciate ligament?

Answer

With the knee at 20 to 30 degrees of flexion, anterior tibial translation with a soft end feel is assessed. It is the most sensitive ACL test and is easier than the anterior drawer in an acute, swollen knee.

#42 · Assessment and Diagnosis

What does the pivot shift test detect and what is a positive sign?

Answer

Anterolateral rotatory instability from ACL deficiency. With valgus and internal rotation as the knee flexes from extension, the tibia subluxes and then reduces with a clunk at about 20 to 40 degrees. It is highly specific.

#43 · Assessment and Diagnosis

How do you perform the posterior drawer test and which ligament does it assess?

Answer

With the knee at 90 degrees, a posterior force is applied to the proximal tibia. Excessive posterior translation indicates posterior cruciate ligament injury, and a posterior sag sign supports it.

#44 · Assessment and Diagnosis

Which collateral ligaments do valgus and varus stress tests assess, and why test at 0 and 30 degrees?

Answer

Valgus stress tests the medial collateral ligament and varus stress tests the lateral collateral ligament. Laxity at 30 degrees is isolated collateral injury. Laxity also at 0 degrees suggests additional cruciate or posterior capsule damage.

#45 · Assessment and Diagnosis

State the Ottawa knee rule criteria for radiography.

Answer

Radiography is needed with age 55 or older, isolated patellar tenderness, fibular head tenderness, inability to flex to 90 degrees, or inability to bear weight for four steps both immediately and in the emergency department.

#46 · Assessment and Diagnosis

Which history and examination findings suggest a meniscal tear?

Answer

Joint line tenderness, mechanical symptoms such as catching or locking, delayed swelling after a twisting injury, and pain with deep squatting. Joint line tenderness is sensitive, while locking is more specific.

#47 · Assessment and Diagnosis

How is the Thessaly test performed and what is a positive result?

Answer

The client stands on the affected leg with the knee flexed about 20 degrees and rotates the body in and out three times. Joint line pain or a locking or catching sensation suggests a meniscal tear.

#48 · Assessment and Diagnosis

What is the J sign in patellar tracking?

Answer

Abrupt lateral deviation of the patella near terminal knee extension, tracing a J shape. It suggests maltracking with lateral patellar displacement, often from weak vastus medialis or tight lateral structures.

#49 · Assessment and Diagnosis

How does infrapatellar fat pad impingement present?

Answer

Anterior knee pain with tenderness on both sides of the patellar tendon, worse with end-range knee extension or prolonged standing. Pain reproduction with tendon-sided palpation and passive hyperextension supports Hoffa fat pad syndrome.

#50 · Assessment and Diagnosis

What does a positive patellar apprehension test indicate?

Answer

With the knee slightly flexed, a lateral glide of the patella provokes apprehension or quadriceps contraction. It indicates patellar instability, commonly after a previous lateral dislocation.

#51 · Assessment and Diagnosis

According to the Ottawa ankle rules, when is an ankle radiograph series indicated in a patient with malleolar zone pain?

Answer

Bone tenderness at the posterior edge or tip of either malleolus (distal 6 cm), or inability to bear weight for four steps both immediately after injury and at assessment.

#52 · Assessment and Diagnosis

Which findings call for a foot (midfoot) radiograph under the Ottawa ankle rules?

Answer

Midfoot pain plus bone tenderness at the base of the fifth metatarsal or at the navicular, or inability to bear weight for four steps. The rules are highly sensitive, so a negative result safely rules out fracture.

#53 · Assessment and Diagnosis

How is a lateral ankle sprain graded I, II and III?

Answer

Grade I: mild stretch, minimal swelling, no laxity. Grade II: partial tear, moderate swelling and bruising, mild to moderate laxity. Grade III: complete rupture, marked swelling and instability, often loss of function.

#54 · Assessment and Diagnosis

How is the Thompson test performed and interpreted?

Answer

With the patient prone and the foot over the table edge, squeeze the calf. Absent or reduced plantarflexion compared with the other side indicates a complete Achilles tendon rupture.

#55 · Assessment and Diagnosis

What is the classic presentation of plantar fasciopathy (plantar heel pain)?

Answer

Sharp pain with the first steps in the morning or after rest, easing then returning with load. Tenderness is at the medial calcaneal tubercle, and the windlass test (passive great toe extension) may reproduce it.

#56 · Assessment and Diagnosis

What is the Laslett cluster for sacroiliac joint pain?

Answer

Three or more positive pain provocation tests from distraction, thigh thrust, compression, sacral thrust and Gaenslen tests. The cluster has good accuracy for identifying the SIJ as a pain source.

#57 · Assessment and Diagnosis

How is the thigh thrust test for the SIJ performed?

Answer

Patient supine with the hip flexed to 90 degrees. The examiner applies a posteriorly directed force through the femur, stressing the SIJ. Reproduction of the familiar buttock pain is positive.

#58 · Assessment and Diagnosis

Where does the patient point to in the Fortin finger test, and what does it suggest?

Answer

The patient points with one finger to pain just inferior and medial to the posterior superior iliac spine. A single-finger point within about 1 cm suggests the SIJ as the source.

#59 · Assessment and Diagnosis

How does centralization of symptoms with repeated movements help separate lumbar from SIJ pain?

Answer

Centralization with repeated lumbar movements points to a lumbar discogenic source. SIJ pain does not centralize and is instead reproduced by SIJ provocation tests.

#60 · Assessment and Diagnosis

How can hip joint pain be distinguished from SIJ or buttock pain at assessment?

Answer

Hip pathology gives groin pain, painful limited internal rotation, and a positive FADIR test. SIJ pain is usually buttock pain below L5, with a positive provocation cluster and normal hip range.

#61 · Assessment and Diagnosis

What features suggest a tibial stress fracture in a runner?

Answer

Localized bony tenderness, pain that builds with impact activity and eases with rest, and pain with the hop test. Early radiographs are often normal, so MRI is the most sensitive imaging.

#62 · Assessment and Diagnosis

Why is a femoral neck stress fracture treated as high risk, and what is the first action?

Answer

It can displace and cause avascular necrosis. Suspect it with groin pain on hopping or weight bearing, stop loading, and arrange urgent medical referral with protected weight bearing.

#63 · Assessment and Diagnosis

What are the key signs of a scaphoid fracture after a fall on an outstretched hand?

Answer

Tenderness in the anatomical snuffbox and pain on axial compression of the thumb. The first radiograph may be normal, so immobilize and arrange repeat imaging or MRI.

#64 · Assessment and Diagnosis

What is the typical limb position and finding in a displaced hip fracture?

Answer

The leg is shortened and externally rotated, with groin pain and inability to bear weight. This needs emergency medical transfer, and the patient should not be walked.

#65 · Assessment and Diagnosis

Which risk factors raise suspicion of osteoporosis and fragility fracture risk?

Answer

Prior fragility fracture, older age, long-term glucocorticoid use, low body weight, parental hip fracture, smoking and heavy alcohol use. Risk tools such as FRAX combine these with bone density.

#66 · Assessment and Diagnosis

How are muscle strains graded I to III?

Answer

Grade I: few fibres torn, mild pain, no real strength loss. Grade II: partial tear with moderate pain, swelling and weakness. Grade III: complete rupture with major weakness and often a palpable gap.

#67 · Assessment and Diagnosis

Which findings suggest acute compartment syndrome, and what is the first action?

Answer

Pain out of proportion, pain on passive stretch, and a tense swollen limb. Pulselessness is a late sign. Remove tight bandages and arrange emergency medical referral for fasciotomy assessment.

#68 · Assessment and Diagnosis

When does delayed onset muscle soreness typically peak, and what causes it?

Answer

It peaks 24 to 72 hours after unaccustomed or eccentric exercise, from microscopic muscle damage. Pain is diffuse and tender on palpation, and strength recovers within about a week.

#69 · Assessment and Diagnosis

Which warning signs after intense exertion suggest rhabdomyolysis?

Answer

Severe muscle pain and swelling, marked weakness, and dark tea-coloured urine. Creatine kinase is very high, and the risk is kidney injury, so refer urgently for medical care.

#70 · Assessment and Diagnosis

What presentation after a thigh contusion suggests myositis ossificans?

Answer

A firm, tender mass with increasing pain and falling range of motion weeks after a deep contusion. Avoid aggressive massage and passive stretching early, and refer for imaging if suspected.

#71 · Assessment and Diagnosis

How is manual muscle test grade 3 defined?

Answer

The muscle moves the joint through full available range against gravity but cannot tolerate any added resistance. Grade 4 tolerates moderate resistance, and grade 5 is normal strength.

#72 · Assessment and Diagnosis

What are manual muscle test grades 2, 1 and 0?

Answer

Grade 2: full range with gravity eliminated. Grade 1: palpable or visible contraction without joint movement. Grade 0: no contraction detected at all.

#73 · Assessment and Diagnosis

How is a universal goniometer aligned to measure joint range?

Answer

Centre the axis over the joint axis, align the stationary arm with the proximal segment, and align the moving arm with the distal segment. Use bony landmarks and consistent patient positioning.

#74 · Assessment and Diagnosis

How do you calculate the minimal detectable change from the standard error of measurement?

Answer

MDC95 = 1.96 x square root of 2 x SEM. A change smaller than the MDC may be measurement error rather than true change in the patient.

#75 · Assessment and Diagnosis

In hand-held dynamometry, how do make and break tests differ?

Answer

In a make test the patient pushes against a fixed examiner resistance. In a break test the examiner overcomes the contraction. Break tests give higher values and more risk of injury, so use one method consistently.

#76 · Assessment and Diagnosis

What are the main passive end-feel types, and which one suggests acute pathology?

Answer

Hard (bone on bone), soft (tissue approximation), firm (capsule or ligament stretch), and empty. An empty end-feel means pain stops movement before tissue resistance, suggesting acute inflammation or serious pathology.

#77 · Assessment and Diagnosis

What is the capsular pattern at the glenohumeral joint?

Answer

External rotation is most limited, then abduction, then internal rotation. A capsular pattern points to a joint-wide problem such as adhesive capsulitis or arthritis.

#78 · Assessment and Diagnosis

What does a non-capsular pattern of restriction suggest?

Answer

A mechanical or localized cause such as ligament sprain, loose body, internal derangement or muscle lesion. Only some motions are limited, in a pattern not matching the joint's capsular proportions.

#79 · Assessment and Diagnosis

How are Maitland mobilization grades I to IV defined?

Answer

Grades I and II are small and large amplitude oscillations in the free range, without tissue resistance. Grades III and IV are large and small amplitude oscillations into resistance. Grade V is a thrust.

#80 · Assessment and Diagnosis

What does the Beighton score measure, and how many points are available?

Answer

Generalized joint hypermobility, scored out of 9 (little finger extension, thumb to forearm, elbow and knee hyperextension, and palms to floor). Cut-offs vary by age, commonly 5 or more in younger adults.

#81 · Assessment and Diagnosis

What is the scoring range and minimal clinically important difference for the LEFS?

Answer

Twenty items scored 0 to 4 give a maximum of 80, where higher means better function. The minimal clinically important difference is 9 scale points.

#82 · Assessment and Diagnosis

How is the DASH scored, and does a higher score mean better or worse function?

Answer

The 30-item DASH is scored from 0 to 100, and a higher score means greater disability. The QuickDASH uses 11 items. Minimal clinically important change is roughly 10 points.

#83 · Assessment and Diagnosis

What does the Neck Disability Index (NDI) consist of and how is it interpreted?

Answer

Ten sections scored 0 to 5, for a maximum of 50, often reported as a percentage. A higher score means greater neck-related disability.

#84 · Assessment and Diagnosis

What is the Oswestry Disability Index and what change is considered meaningful?

Answer

A 10-section low back pain disability questionnaire scored as a percentage from 0 to 100, where higher means worse. A change of about 10 points is commonly considered clinically important.

#85 · Assessment and Diagnosis

How does the Patient-Specific Functional Scale (PSFS) work?

Answer

The patient names three to five important activities they struggle with and rates each from 0 (unable) to 10 (able at pre-injury level). Averaging gives the score, and a 2 point change is meaningful.

#86 · Assessment and Diagnosis

What are the typical findings in scapular dyskinesis?

Answer

Prominence of the medial border or inferior angle, early or excessive elevation (shrug), and rapid downward rotation on lowering the arm. Winging with a push-up suggests serratus anterior weakness or long thoracic nerve palsy.

#87 · Assessment and Diagnosis

What is seen in a Trendelenburg gait, and which muscle is weak?

Answer

The pelvis drops on the side opposite to the stance leg because the stance-side gluteus medius is weak. A compensated version shows a trunk lean over the weak side.

#88 · Assessment and Diagnosis

What causes a steppage gait, and which nerve is commonly involved?

Answer

Dorsiflexor weakness causes foot drop, so the patient lifts the knee high to clear the toes. A common cause is common peroneal nerve injury or an L4 to L5 radiculopathy.

#89 · Assessment and Diagnosis

What gait speed is a useful warning threshold in older adults?

Answer

Walking speed below about 0.8 m/s is linked to a higher risk of falls, disability, hospitalization and mortality. Gait speed is often called the sixth vital sign.

#90 · Assessment and Diagnosis

How is the Functional Movement Screen scored?

Answer

Seven movement patterns are each scored from 0 to 3, for a maximum of 21. A score of 0 is given when pain occurs during the movement, which needs further clinical assessment.

#91 · Assessment and Diagnosis

Which signs suggest central sensitization in a patient with chronic pain?

Answer

Pain out of proportion to the injury, diffuse or non-anatomical spread, allodynia, hyperalgesia, and sensitivity to light, sound or stress. The Central Sensitization Inventory screens for it, with 40 as a common cut-off.

#92 · Assessment and Diagnosis

What are yellow flags?

Answer

Psychosocial risk factors for chronicity, such as fear avoidance, catastrophizing, depression, belief that pain means harm, low recovery expectations and passive coping. They are addressed alongside physical treatment.

#93 · Assessment and Diagnosis

How does the STarT Back Screening Tool stratify risk?

Answer

Nine items. A total of 3 or less is low risk. A total of 4 or more is medium risk, or high risk if the 5-item psychosocial subscale is 4 or more.

#94 · Assessment and Diagnosis

What three components does the Pain Catastrophizing Scale measure?

Answer

Rumination, magnification and helplessness, across 13 items scored 0 to 52. A total of 30 or more is commonly considered clinically relevant catastrophizing.

#95 · Assessment and Diagnosis

What is the core message of pain neuroscience education?

Answer

Pain is a protective output of the brain, not a direct measure of tissue damage. Understanding this reduces threat and fear, and supports graded activity and movement.

#96 · Assessment and Diagnosis

What is the typical joint pattern and stiffness in rheumatoid arthritis?

Answer

Symmetrical polyarthritis of small joints (MCP, PIP, wrists, MTP), sparing the DIP joints, with morning stiffness lasting longer than 60 minutes and warm swollen joints.

#97 · Assessment and Diagnosis

What clinical features suggest inflammatory back pain from ankylosing spondylitis?

Answer

Insidious onset before age 40, morning stiffness over 30 minutes, night pain, and improvement with exercise but not rest. Chest expansion under 2.5 cm and reduced modified Schober increase are typical findings.

#98 · Assessment and Diagnosis

What is the classic presentation of acute gout?

Answer

Sudden severe monoarthritis, often of the first metatarsophalangeal joint (podagra), with hot, red, exquisitely tender swelling. Crystals in joint fluid are needle-shaped and negatively birefringent.

#99 · Assessment and Diagnosis

Which features point to psoriatic arthritis rather than rheumatoid arthritis?

Answer

Dactylitis (sausage digit), DIP joint involvement, nail pitting, enthesitis such as Achilles insertion pain, and an asymmetric joint pattern in a patient with psoriasis.

#100 · Assessment and Diagnosis

Which features suggest systemic lupus erythematosus in a patient with joint pain?

Answer

Malar (butterfly) rash, photosensitivity, fatigue, oral ulcers, and non-erosive symmetrical arthralgia, often in a young woman. Kidney involvement is serious, and exercise is adjusted during flares.

#101 · Assessment and Diagnosis

Which ACR clinical criteria support a diagnosis of knee osteoarthritis without imaging or labs?

Answer

Knee pain plus at least 3 of 6: age over 50, morning stiffness under 30 minutes, crepitus, bony tenderness, bony enlargement, and no palpable warmth.

#102 · Assessment and Diagnosis

How does morning stiffness help separate osteoarthritis from inflammatory arthritis such as rheumatoid arthritis?

Answer

Osteoarthritis stiffness usually settles within 30 minutes. Inflammatory arthritis stiffness typically lasts over 60 minutes, often with swelling and systemic fatigue, which should prompt referral for medical workup.

#103 · Assessment and Diagnosis

Which hip motions are lost first in hip osteoarthritis, and which passive test is most provocative?

Answer

Internal rotation is usually lost first, followed by flexion and abduction (capsular pattern). Passive internal rotation, especially at 90 degrees of hip flexion, commonly reproduces groin pain.

#104 · Assessment and Diagnosis

What is the capsular pattern of the knee, and what does it suggest?

Answer

Flexion is limited more than extension. It indicates an arthritic or capsular problem such as osteoarthritis, as opposed to a mechanical block like a displaced meniscal tear.

#105 · Assessment and Diagnosis

A client with knee osteoarthritis reports a sudden flare with a large warm effusion and fever. What should you consider?

Answer

Septic arthritis or crystal arthropathy (gout, pseudogout). A hot, swollen joint with fever is not a routine osteoarthritis flare and needs same day medical referral.

#106 · Assessment and Diagnosis

What is the difference between phantom limb sensation and phantom limb pain after amputation?

Answer

Phantom sensation is painless awareness of the missing limb (tingling, position, itch). Phantom pain is a painful sensation felt in the absent limb, often burning, cramping, or shooting.

#107 · Assessment and Diagnosis

Which K-level describes a limited community ambulator able to negotiate low-level barriers such as curbs and stairs?

Answer

K2. K1 is level surface household ambulation at fixed cadence, K3 is variable cadence and most barriers, and K4 is high-impact or high-demand activity such as sport or children.

#108 · Assessment and Diagnosis

Which K-level applies to a prosthetic user with high-impact demands, such as an athlete or active child?

Answer

K4. These users exceed basic ambulation skills and need high-energy storing components. K0 means no ability or potential to ambulate or transfer safely, so a prosthesis is not indicated.

#109 · Assessment and Diagnosis

A residual limb has a very tender, focal point and a positive Tinel sign over the cut nerve end. What is suspected?

Answer

A symptomatic neuroma. Pressure from the socket can trigger sharp, shooting pain along the nerve. Management includes socket modification and referral for medical or surgical review if pain persists.

#110 · Assessment and Diagnosis

What does pistoning of the residual limb in the socket usually indicate?

Answer

Poor suspension or loss of limb volume (shrinkage). Excess vertical movement causes friction and skin breakdown, so check sock ply, liner fit, and suspension before referring to the prosthetist.

#111 · Assessment and Diagnosis

How do Ortolani and Barlow maneuvers differ in screening an infant for developmental hip dysplasia?

Answer

Ortolani reduces a dislocated hip into the socket (abduct and lift, felt as a clunk). Barlow provokes dislocation of an unstable hip (adduct with posterior pressure).

#112 · Assessment and Diagnosis

What is the typical presentation of Legg-Calve-Perthes disease?

Answer

A boy aged about 4 to 8 with a painless or mildly painful limp, often with referred thigh or knee pain, and limited hip abduction and internal rotation. It is avascular necrosis of the femoral head.

#113 · Assessment and Diagnosis

What are the classic features of slipped capital femoral epiphysis (SCFE)?

Answer

An overweight adolescent (about 10 to 16 years) with hip or referred knee pain, limited internal rotation, and obligatory external rotation on passive hip flexion. Refer urgently and keep non-weight-bearing.

#114 · Assessment and Diagnosis

What does a positive Adams forward bend test show, and what does it indicate?

Answer

A rib or paraspinal prominence on one side when bending forward. It indicates vertebral rotation, meaning a structural scoliosis, and warrants imaging and referral rather than observation alone.

#115 · Assessment and Diagnosis

What Cobb angle on standing radiograph defines scoliosis?

Answer

10 degrees or more of lateral spinal curvature. The Cobb angle is measured between the most tilted vertebrae at the curve ends and guides monitoring, bracing, and surgical decisions.

#116 · Assessment and Diagnosis

What is the difference between stress urinary incontinence and urge urinary incontinence?

Answer

Stress incontinence is leakage with raised abdominal pressure (cough, sneeze, jump). Urge incontinence is leakage preceded by a sudden, hard-to-delay urge, typically linked to overactive bladder.

#117 · Assessment and Diagnosis

What information does a bladder diary record, and for how long is it typically kept?

Answer

Fluid intake, voiding times and volumes, urgency, leaks, and triggers, usually over 3 days. It objectively shows frequency, nocturia, and fluid habits that guide the treatment plan.

#118 · Assessment and Diagnosis

How do cystocele and rectocele differ on examination of pelvic organ prolapse?

Answer

Cystocele is descent of the bladder into the anterior vaginal wall. Rectocele is bulging of the rectum into the posterior vaginal wall. Both are graded on straining or bearing down.

#119 · Assessment and Diagnosis

How is pelvic floor muscle strength graded on the modified Oxford scale?

Answer

0 to 5: 0 no contraction, 1 flicker, 2 weak, 3 moderate with some lift, 4 good with resisted lift, 5 strong with firm resistance and lift against pressure.

#120 · Assessment and Diagnosis

During palpation, what finding indicates an incorrect pelvic floor muscle contraction strategy?

Answer

Bearing down (perineal descent) or gluteal, abdominal, or breath-holding substitution instead of an inward and upward squeeze and lift. Retrain with feedback before progressing strengthening.

#121 · Assessment and Diagnosis

What are the Kocher criteria for predicting septic arthritis of the hip in a child?

Answer

Non-weight-bearing on the affected side, ESR over 40 mm/h, fever over 38.5 C, and WBC over 12,000 per microlitre. More positives raise the probability sharply.

#122 · Assessment and Diagnosis

A hot, swollen joint with fever and severe pain on any movement is present. What is the action?

Answer

Suspect septic arthritis, a surgical emergency. Do not treat. Arrange immediate emergency medical assessment, since joint destruction can begin within days.

#123 · Assessment and Diagnosis

Where does acute hematogenous osteomyelitis most often start in children?

Answer

In the metaphysis of long bones such as the distal femur and proximal tibia. Signs include focal bone pain, fever, refusal to bear weight, and local tenderness.

#124 · Assessment and Diagnosis

A young child refuses to walk or sit, with low-grade fever and back pain. What diagnosis is suspected?

Answer

Discitis, an infection of the intervertebral disc. ESR and CRP are usually raised. Refer for imaging (MRI) and medical care rather than starting treatment.

#125 · Assessment and Diagnosis

How does transient synovitis of the hip differ from septic arthritis in a child?

Answer

Transient synovitis often follows a viral illness, with no or low fever, normal or mildly raised inflammatory markers, and the child can usually bear some weight. Septic arthritis does not fit this picture.

#126 · Assessment and Diagnosis

Which features of left arm, jaw, or chest pain suggest a cardiac source needing urgent referral?

Answer

Exertional onset relieved by rest, diaphoresis, nausea, dyspnea, and pain not changed by movement or palpation. If present at rest or severe, call 911 and do not continue treatment.

#127 · Assessment and Diagnosis

What is Kehr sign, and what does it suggest?

Answer

Acute left shoulder tip pain, referred from diaphragmatic irritation, most often from splenic rupture or intra-abdominal bleeding. Recent abdominal trauma makes it an emergency.

#128 · Assessment and Diagnosis

Right shoulder or scapular pain after a fatty meal, with nausea, suggests which organ source?

Answer

The gallbladder (cholecystitis or gallstones), referred through the phrenic nerve. The shoulder exam is typically normal and movement does not change symptoms. Refer for medical assessment.

#129 · Assessment and Diagnosis

An older male smoker has deep back pain and a pulsatile abdominal mass. What is suspected and what is done?

Answer

Abdominal aortic aneurysm. Pain that is constant and unaffected by movement raises concern. Do not palpate deeply, and arrange urgent or emergency medical care.

#130 · Assessment and Diagnosis

A client has unilateral calf swelling, warmth, and tenderness after surgery. What tool guides your decision?

Answer

The Wells score for deep vein thrombosis, based on risk factors and signs. Moderate or high probability needs same day medical referral for imaging. Do not massage the calf.

#131 · Assessment and Diagnosis

What are the four stages of fracture healing in order?

Answer

Inflammation (hematoma), soft callus, hard callus, then remodelling. Remodelling can take months to years, and loading is progressed according to the stage and stability.

#132 · Assessment and Diagnosis

What is the Salter-Harris SALTR mnemonic for pediatric growth plate fractures?

Answer

I Straight across the physis, II Above (through physis and metaphysis, most common), III beLow (epiphysis), IV Through (metaphysis, physis, epiphysis), V cRush. Higher types risk growth disturbance.

#133 · Assessment and Diagnosis

A fall on an outstretched hand causes anatomical snuffbox tenderness. What is the concern and management?

Answer

Scaphoid fracture, which risks avascular necrosis of the proximal pole due to retrograde blood supply. Immobilize and refer, because early radiographs may be negative.

#134 · Assessment and Diagnosis

Why is avascular necrosis a major risk after a displaced femoral neck fracture?

Answer

The femoral head blood supply runs retrograde from the medial femoral circumflex artery, which a displaced intracapsular fracture can disrupt. Watch for worsening groin pain and loss of function.

#135 · Assessment and Diagnosis

Which findings after a fracture suggest complex regional pain syndrome (CRPS)?

Answer

Pain out of proportion to the injury, allodynia, edema, skin colour and temperature change, sweating changes, and stiffness (Budapest criteria). Early recognition and referral improve outcomes.

#136 · Assessment and Diagnosis

Which patient factors commonly contribute to fracture nonunion?

Answer

Smoking, diabetes, poor nutrition, infection, inadequate immobilization or instability, and some medications such as long-term corticosteroids. Persistent pain at the site with no healing progression calls for medical review.

#137 · Assessment and Diagnosis

Which hip movements are restricted after total hip arthroplasty via the posterior approach?

Answer

Avoid flexion beyond 90 degrees, adduction past midline, and internal rotation. These positions risk posterior dislocation. Precautions follow the surgeon's protocol and duration.

#138 · Assessment and Diagnosis

Which combined movement is avoided after total hip arthroplasty via the anterior approach?

Answer

Hip extension combined with external rotation, which risks anterior dislocation. Posterior precautions are generally less strict, but the surgeon's protocol governs.

#139 · Assessment and Diagnosis

What is the general principle for early range of motion after rotator cuff repair?

Answer

Protect the repair: passive range first, with active elevation delayed until the surgeon clears it, often around 6 weeks. Timing varies with tear size and tissue quality.

#140 · Assessment and Diagnosis

Which deficit is typical after ACL reconstruction with a hamstring graft versus a patellar tendon graft?

Answer

Hamstring graft: hamstring weakness at the donor site. Patellar tendon graft: anterior knee pain and kneeling discomfort. Graft type shapes strengthening and monitoring.

#141 · Assessment and Diagnosis

Which postoperative findings after knee arthroplasty should prompt contact with the surgeon?

Answer

Increasing wound drainage, spreading redness, heat, fever, rising pain, or calf swelling. These suggest infection or thrombosis and need prompt medical assessment rather than progression of rehab.

#142 · Assessment and Diagnosis

What are the three stages of the tendinopathy continuum described by Cook and Purdam?

Answer

Reactive tendinopathy, tendon disrepair, and degenerative tendinopathy. Early stages respond to load reduction and gradual re-loading, while degenerative changes are less reversible.

#143 · Assessment and Diagnosis

How does the response to isometric loading help in a painful tendon?

Answer

Sustained isometric holds (for example 45 seconds) can temporarily reduce tendon pain, so they may be used to maintain loading when painful. The effect is short-term and varies.

#144 · Assessment and Diagnosis

How do a complete Achilles tendon rupture and Achilles tendinopathy differ at the bedside?

Answer

Rupture: sudden pop, palpable gap, weak plantarflexion, and positive Thompson test (no plantarflexion on calf squeeze). Tendinopathy: gradual, load-related localized pain and morning stiffness.

#145 · Assessment and Diagnosis

What does the arc sign suggest in midportion Achilles pain?

Answer

A swelling that moves with the tendon through ankle dorsiflexion and plantarflexion points to the tendon itself (midportion tendinopathy) rather than the paratenon or surrounding structures.

#146 · Assessment and Diagnosis

What does the Quebec Task Force classify, and what separates Grade II from Grade III?

Answer

Whiplash-associated disorder severity. Grade II is neck pain with musculoskeletal signs (reduced range, point tenderness). Grade III adds neurological signs. Grade IV is fracture or dislocation.

#147 · Assessment and Diagnosis

Which Canadian C-spine rule factors mandate radiography?

Answer

Age 65 or older, a dangerous mechanism (such as a fall from 1 m (5 stairs) or more, or axial load), or paresthesias in the extremities. Otherwise low-risk factors allow safe range assessment.

#148 · Assessment and Diagnosis

Which features help identify cervicogenic headache?

Answer

Unilateral side-locked pain starting in the neck, provoked by neck movement or sustained posture, reduced cervical range, and a positive flexion-rotation test for upper cervical dysfunction.

#149 · Assessment and Diagnosis

What headache features indicate an emergency referral rather than treatment?

Answer

A sudden thunderclap headache reaching peak intensity within a minute, or headache with fever and neck stiffness, neurological deficits, or after significant head trauma. Suspect hemorrhage or meningitis.

#150 · Assessment and Diagnosis

Why is the cervical spine examined in a client with concussion and persistent headache or dizziness?

Answer

Neck injury often coexists with concussion and can cause cervicogenic headache and dizziness. Identifying it directs treatment, while red flags still need medical clearance.

#151 · Assessment and Diagnosis

What does the FAST stroke recognition tool stand for, and what is the action step?

Answer

Face drooping, Arm weakness, Speech difficulty, Time to call 911. Any one sign means a suspected stroke and an emergency, because clot-busting treatment and thrombectomy are time-dependent.

#152 · Assessment and Diagnosis

What is the scoring range of the NIHSS and what does a higher score indicate?

Answer

The National Institutes of Health Stroke Scale scores 0 to 42. A higher score means a more severe neurological deficit. It is used in acute stroke to grade severity and track change.

#153 · Assessment and Diagnosis

How does the Chedoke-McMaster Stroke Assessment describe motor recovery of the arm, leg, hand, and foot?

Answer

It uses a 7-stage scale per segment. Stage 1 is flaccid paralysis with no voluntary movement, and stage 7 is normal movement. Stages 2 to 3 reflect synergy and spasticity, which guide treatment choices.

#154 · Assessment and Diagnosis

Which line-bisection finding suggests hemineglect, and which lesion side is the classic cause?

Answer

The patient marks the line midpoint shifted toward the unaffected side and ignores the opposite half. Left-sided neglect after a right parietal stroke is most common. Star cancellation and clock drawing also screen for it.

#155 · Assessment and Diagnosis

On the Modified Ashworth Scale, what does grade 1+ describe?

Answer

A catch followed by minimal resistance through less than half of the range of motion. Grade 2 is a more marked increase through most of the range, but the part still moves easily.

#156 · Assessment and Diagnosis

What are the three Glasgow Coma Scale components, total range, and the severe injury cut-off?

Answer

Eye opening (1 to 4), verbal response (1 to 5), motor response (1 to 6). Total is 3 to 15. A score of 8 or less is severe, 9 to 12 moderate, and 13 to 15 mild.

#157 · Assessment and Diagnosis

At which Rancho Los Amigos level is a patient confused and agitated?

Answer

Level IV, confused and agitated. The patient is hyperactive, has a short attention span, and cannot learn new information. Management uses a calm, low-stimulation environment and avoids restraint where possible.

#158 · Assessment and Diagnosis

What defines post-traumatic amnesia, and when is it considered resolved?

Answer

PTA is the period after brain injury when the person cannot form continuous memories and is disoriented. It resolves when they are consistently oriented and recall daily events. Tools include the Westmead PTA Scale and GOAT.

#159 · Assessment and Diagnosis

What is the SCAT6, and what must a clinician remember about its use?

Answer

The Sport Concussion Assessment Tool, 6th edition, is for athletes aged 13 and older (Child SCAT6 is for ages 8 to 12). It supports assessment but cannot alone diagnose or exclude a concussion.

#160 · Assessment and Diagnosis

What is the sequence of return to learn and return to sport after a concussion in Canadian guidance?

Answer

Return to school or learning comes first, with gradual symptom-limited increases. A person should be back to full learning, with no symptoms, before progressing to full-contact sport and medical clearance.

#161 · Assessment and Diagnosis

What does an ISNCSCI AIS grade A (complete) injury mean?

Answer

No sensory or motor function preserved in the lowest sacral segments, S4 to S5. This includes no voluntary anal contraction and no deep anal pressure. Any sacral sparing makes the injury incomplete.

#162 · Assessment and Diagnosis

How does AIS grade C differ from AIS grade D?

Answer

Both are motor incomplete. In C, more than half of the key muscles below the neurological level are graded less than 3. In D, at least half of them are grade 3 or higher.

#163 · Assessment and Diagnosis

How is the neurological level of injury defined in ISNCSCI?

Answer

It is the most caudal segment with intact sensation and antigravity muscle strength (at least 3 of 5), provided the segments above are normal. Both sides are tested, and the single neurological level is the most cephalad (highest) of the sensory and motor levels.

#164 · Assessment and Diagnosis

Which key muscle represents C7 in the ISNCSCI motor examination?

Answer

Elbow extensors (triceps). Memory aid for the key muscles: C5 elbow flexors, C6 wrist extensors, C7 elbow extensors, C8 finger flexors, T1 finger abductors (small finger).

#165 · Assessment and Diagnosis

A patient with T4 spinal cord injury has a pounding headache and high blood pressure. What is the first action?

Answer

Suspect autonomic dysreflexia. Sit the person upright, loosen tight clothing, and look for a trigger such as bladder distension, catheter blockage, or bowel impaction. Call emergency services if it does not resolve.

#166 · Assessment and Diagnosis

What are the three Seddon grades of peripheral nerve injury?

Answer

Neurapraxia (conduction block, no axonal loss, recovers in days to weeks), axonotmesis (axon disrupted, endoneurium intact, regrowth about 1 mm per day), and neurotmesis (complete transection needing surgery).

#167 · Assessment and Diagnosis

How do Sunderland grades relate to Seddon's classification?

Answer

Sunderland has five grades. Grade 1 equals neurapraxia, grades 2 to 4 are progressively worse axonotmesis levels, and grade 5 is complete transection, equal to neurotmesis. Higher grades have poorer spontaneous recovery.

#168 · Assessment and Diagnosis

What arm posture is typical of an upper trunk (Erb) brachial plexus injury?

Answer

The waiter's tip position: shoulder adducted and internally rotated, elbow extended, forearm pronated, wrist flexed. It comes from C5 and C6 damage, often with traction at birth or in a fall.

#169 · Assessment and Diagnosis

What is the classic presentation of Guillain-Barre syndrome and a key physiotherapy precaution?

Answer

Symmetric ascending weakness with areflexia, often after an infection. Monitor respiratory function and autonomic instability, and avoid overfatigue or intense strengthening, which can worsen weakness.

#170 · Assessment and Diagnosis

Which tool screens for loss of protective sensation in diabetic neuropathy of the foot?

Answer

The 10 g Semmes-Weinstein monofilament, applied at several plantar sites until it bends. Inability to feel it at one or more sites signals loss of protective sensation and higher ulcer risk.

#171 · Assessment and Diagnosis

What does Hoehn and Yahr stage 3 describe in Parkinson disease?

Answer

Bilateral disease with mild to moderate disability and postural instability, such as an abnormal pull test, but the person is still physically independent. Stage 2 is bilateral without balance impairment.

#172 · Assessment and Diagnosis

Which feature is required for a clinical diagnosis of parkinsonism, and what other signs accompany it?

Answer

Bradykinesia (slowness with decrement in amplitude or speed of repeated movement) plus rest tremor or rigidity. Postural instability usually appears later and is not required for diagnosis.

#173 · Assessment and Diagnosis

Name cueing strategies that help freezing of gait in Parkinson disease.

Answer

Visual cues such as lines or tape on the floor, auditory cues such as a metronome or counting, and attentional strategies such as stepping over an object or shifting weight before stepping.

#174 · Assessment and Diagnosis

What is festination in Parkinson disease?

Answer

Short, quickening steps with a forward-leaning trunk as the person chases their own centre of mass. It raises fall risk. Training uses larger deliberate steps, cueing, and turning in wide arcs.

#175 · Assessment and Diagnosis

Which tests and signs identify cerebellar ataxia on a physiotherapy exam?

Answer

Finger-to-nose with dysmetria and intention tremor, dysdiadochokinesia on rapid alternating movements, heel-to-shin ataxia, and a wide-based gait. Unlike Parkinson tremor, cerebellar tremor worsens near the target.

#176 · Assessment and Diagnosis

How is a multiple sclerosis relapse defined for clinical purposes?

Answer

New or worsening neurological symptoms lasting at least 24 hours, without fever or infection, and occurring at least 30 days after the start of a previous relapse. Otherwise consider a pseudo-relapse.

#177 · Assessment and Diagnosis

What is Uhthoff phenomenon and how should a physiotherapist respond?

Answer

A temporary worsening of MS symptoms when core body temperature rises with heat, exercise, or fever. It is not a relapse. Cool the person, rest, and plan exercise with cooling strategies.

#178 · Assessment and Diagnosis

What principles guide management of fatigue in multiple sclerosis?

Answer

Fatigue is among the most common and disabling MS symptoms. Use pacing, energy conservation, scheduled rests, cooling, and regular moderate exercise. Avoid exercising to exhaustion, which can worsen next-day function.

#179 · Assessment and Diagnosis

What does an EDSS score of 6.0 mean in multiple sclerosis?

Answer

The Expanded Disability Status Scale runs 0 to 10. At 6.0 the person needs intermittent or unilateral constant assistance, such as a cane, to walk about 100 metres, with or without resting.

#180 · Assessment and Diagnosis

What is Lhermitte sign and which condition is it linked with?

Answer

An electric shock sensation running down the spine, sometimes into the limbs, on neck flexion. It points to dorsal cervical cord irritation and is classically associated with multiple sclerosis.

#181 · Assessment and Diagnosis

Which findings typify amyotrophic lateral sclerosis on examination?

Answer

A mix of upper motor neuron signs (spasticity, hyperreflexia) and lower motor neuron signs (atrophy, fasciculations), with sensation, eye movements, and bladder control typically preserved. Plan submaximal exercise and avoid fatigue.

#182 · Assessment and Diagnosis

What is Gowers sign and in which condition is it classic?

Answer

A child pushes up the thighs with the hands to rise from the floor because of proximal hip weakness. It is classic in Duchenne muscular dystrophy, an X-linked disorder with calf pseudohypertrophy.

#183 · Assessment and Diagnosis

How are GMFCS levels I and V described in cerebral palsy?

Answer

Level I walks without limitations, with reduced speed, balance, and coordination. Level V has very limited self-mobility and is transported in a manual wheelchair. Levels run I to V by age band.

#184 · Assessment and Diagnosis

Which spina bifida type is most severe, and which precaution applies to the care environment?

Answer

Myelomeningocele, where the spinal cord and meninges protrude, often with hydrocephalus and Chiari II malformation. Use latex-free equipment because of the elevated risk of latex allergy.

#185 · Assessment and Diagnosis

When do most children walk independently, and when is referral considered?

Answer

Most walk independently by about 12 to 15 months. Not walking by 18 months, or any loss of previously gained skills, warrants further assessment. Sitting without support is usually achieved by about 6 months.

#186 · Assessment and Diagnosis

What is the MoCA's maximum score, and which result is considered normal?

Answer

The Montreal Cognitive Assessment is scored out of 30, and 26 or higher is considered normal. One point is added if education is 12 years or fewer. It screens, but does not diagnose, cognitive impairment.

#187 · Assessment and Diagnosis

How does delirium differ from dementia in onset and attention?

Answer

Delirium has acute onset over hours to days, fluctuating course, and marked inattention. Dementia develops gradually with relatively preserved attention early on. Delirium is a medical emergency needing urgent referral.

#188 · Assessment and Diagnosis

Which features distinguish dementia with Lewy bodies from Alzheimer disease?

Answer

Lewy body dementia shows early visual hallucinations, fluctuating cognition, and parkinsonism with a falls risk. Alzheimer disease typically presents with early episodic memory loss that progresses gradually.

#189 · Assessment and Diagnosis

What does a score of 10 or more on the PHQ-9 suggest, and what item needs same-day action?

Answer

A score of 10 or higher suggests probable major depression, out of a possible 27. Any positive answer on item 9, thoughts of self-harm, needs same-day follow-up and referral per local policy.

#190 · Assessment and Diagnosis

What four abilities are considered when observing decision-making capacity?

Answer

Understanding relevant information, appreciating how it applies to oneself, reasoning about options, and expressing a choice. Capacity is decision-specific and time-specific, and adults are presumed capable unless shown otherwise.

#191 · Assessment and Diagnosis

When is a seizure treated as status epilepticus?

Answer

When it lasts longer than 5 minutes, or when seizures repeat without recovery of consciousness between them. It is a medical emergency, so call 911 and protect the airway and head.

#192 · Assessment and Diagnosis

What first aid steps apply during a generalized tonic-clonic seizure in the clinic?

Answer

Note the time, protect the head, clear the area, and loosen tight clothing. Do not restrain or put anything in the mouth. After jerking stops, turn the person onto their side.

#193 · Assessment and Diagnosis

How does an absence seizure differ from a focal impaired awareness seizure?

Answer

Absence seizures are brief staring spells of about 5 to 20 seconds with abrupt onset and recovery and no postictal state. Focal impaired awareness seizures last longer, often with automatisms and postictal confusion.

#194 · Assessment and Diagnosis

Which common triggers lower seizure threshold, and what is the exercise guidance?

Answer

Sleep deprivation, missed medication, alcohol, flashing lights, and hyperventilation. Regular exercise is generally safe and encouraged. Avoid hyperventilation and apply safety precautions around water, heights, and heat.

#195 · Assessment and Diagnosis

What is Todd paralysis?

Answer

Temporary focal weakness in a limb or side after a seizure, which usually resolves within about 48 hours. It must be distinguished from stroke, so any new or persistent deficit requires urgent medical assessment.

#196 · Assessment and Diagnosis

What is a positive Dix-Hallpike test for posterior canal BPPV?

Answer

After a few seconds of latency, vertigo with upbeating torsional nystagmus beats toward the undermost ear, lasting under 60 seconds and fatiguing on repeat. Treat with the Epley maneuver.

#197 · Assessment and Diagnosis

What does a corrective saccade on the head impulse test indicate?

Answer

Peripheral vestibular hypofunction on the side the head was turned toward, because the vestibulo-ocular reflex fails to hold gaze. A normal test in acute vertigo with nystagmus raises concern for a central cause.

#198 · Assessment and Diagnosis

What are the three HINTS components and which results suggest a central cause?

Answer

Head impulse, Nystagmus, Test of skew. Central signs are a normal head impulse, direction-changing or vertical nystagmus, and skew deviation. HINTS applies only in acute continuous vertigo with nystagmus.

#199 · Assessment and Diagnosis

How is the Dizziness Handicap Inventory scored and graded?

Answer

It has 25 items scored 0 to 100. Higher scores mean greater perceived handicap: 0 to 30 mild, 31 to 60 moderate, and 61 to 100 severe. It tracks the impact of dizziness over time.

#200 · Assessment and Diagnosis

How many items and what score range does the Berg Balance Scale have, and what score is commonly linked to higher fall risk?

Answer

It has 14 items scored 0 to 4 each, for a total of 56. Scores below 45 are commonly associated with increased fall risk in older adults, and lower scores mean worse balance.

#201 · Assessment and Diagnosis

What does the Timed Up and Go (TUG) test measure, and what time suggests increased fall risk in older adults?

Answer

It times standing from a chair, walking 3 metres, turning, returning and sitting. A time of 12 seconds or more (CDC STEADI cut-off) suggests increased fall risk; other studies cite 13.5 seconds.

#202 · Assessment and Diagnosis

In the 10-metre walk test, how is gait speed calculated and which part of the walkway is timed?

Answer

The client walks 10 metres, but only the middle 6 metres are timed, leaving 2 metres each end for acceleration and deceleration. Speed (m/s) equals 6 divided by the time in seconds.

#203 · Assessment and Diagnosis

What walking speeds separate household, limited community and full community ambulation (Perry classification)?

Answer

Below 0.4 m/s is household ambulation, 0.4 to 0.8 m/s is limited community ambulation, and above 0.8 m/s is community ambulation. Gait speed is a quick functional and prognostic marker.

#204 · Assessment and Diagnosis

What does the Mini-BESTest assess, and how is it scored?

Answer

It assesses dynamic balance across 14 items in four sections: anticipatory postural control, reactive postural control, sensory orientation and dynamic gait. Maximum score is 28, and it has fewer ceiling effects than the Berg.

#205 · Assessment and Diagnosis

What are the classic signs of bacterial meningitis, and how often are all three present together?

Answer

Fever, severe headache and neck stiffness (nuchal rigidity). Only a minority of patients show all three, so photophobia, vomiting or altered mental status with fever should also prompt urgent medical referral.

#206 · Assessment and Diagnosis

How are Kernig and Brudzinski signs elicited, and what do they suggest?

Answer

Kernig: with hip and knee flexed to 90 degrees, passive knee extension causes pain or resistance. Brudzinski: passive neck flexion causes hip and knee flexion. Both suggest meningeal irritation.

#207 · Assessment and Diagnosis

A feverish client has a non-blanching petechial rash and neck stiffness. What is the first action?

Answer

Treat as suspected meningococcal sepsis or meningitis: a medical emergency. Call emergency services (911) immediately and do not continue the physiotherapy session. Delay in antibiotics increases mortality.

#208 · Assessment and Diagnosis

What features separate encephalitis from meningitis?

Answer

Encephalitis involves brain parenchyma, so altered level of consciousness, personality change, seizures or focal deficits dominate, along with fever and headache. Herpes simplex virus is the most common sporadic viral cause.

#209 · Assessment and Diagnosis

What presentation suggests transverse myelitis, and why is it an urgent referral?

Answer

Rapid onset of bilateral weakness, a sensory level, and bladder or bowel dysfunction from spinal cord inflammation, often after infection. It needs urgent medical workup to protect cord function.

#210 · Assessment and Diagnosis

What is the normal resting heart rate range for adults?

Answer

60 to 100 beats per minute. Below 60 is bradycardia and above 100 is tachycardia, though well-trained athletes may normally rest below 60. Always note rhythm regularity and symptoms.

#211 · Assessment and Diagnosis

What does a third heart sound (S3) suggest on cardiac auscultation?

Answer

An early diastolic ventricular gallop. In adults over about 40 it suggests volume overload and heart failure. It occurs from rapid ventricular filling of a dilated, stiff ventricle.

#212 · Assessment and Diagnosis

What are the Borg RPE scale range and the meaning of a rating of 13?

Answer

The original Borg RPE scale runs from 6 (no exertion) to 20 (maximal exertion). A rating of 13 means somewhat hard, roughly moderate intensity. Multiply a rating by 10 to estimate heart rate.

#213 · Assessment and Diagnosis

How does stable angina differ from pain of acute myocardial infarction?

Answer

Angina is brought on by exertion or stress and eases within minutes with rest or nitroglycerin. Infarction pain is usually more severe, lasts longer than 15 to 20 minutes, and is not relieved by rest. Treat as an emergency.

#214 · Assessment and Diagnosis

What do the four NYHA functional classes describe in heart failure?

Answer

Class I: no limitation. Class II: slight limitation, symptoms with ordinary activity. Class III: marked limitation, symptoms with less than ordinary activity. Class IV: symptoms at rest or with any activity.

#215 · Assessment and Diagnosis

What is the value of 1 MET?

Answer

1 MET equals 3.5 mL of oxygen per kilogram per minute, the approximate resting oxygen consumption. Activities are expressed as multiples of this, for example brisk walking at about 3 to 4 METs.

#216 · Assessment and Diagnosis

During exercise testing, what blood pressure response is an absolute indication to stop?

Answer

A drop in systolic blood pressure of more than 10 mm Hg despite an increase in workload, when accompanied by other signs of ischemia. Also stop for worsening angina, severe dyspnea or dizziness.

#217 · Assessment and Diagnosis

How does the Canadian Cardiovascular Society classify heart failure by ejection fraction?

Answer

HFrEF: ejection fraction 40 percent or less. HF with mildly reduced EF: 41 to 49 percent. HFpEF: 50 percent or higher. The type guides medication choices and rehabilitation prescriptions.

#218 · Assessment and Diagnosis

Which clients are typically eligible for referral to cardiac rehabilitation?

Answer

Those after myocardial infarction, percutaneous coronary intervention or bypass surgery, plus stable chronic heart failure and valve surgery. Program entry is by referral, usually soon after the cardiac event.

#219 · Assessment and Diagnosis

How is the ankle-brachial index calculated, and what value suggests peripheral artery disease?

Answer

Highest ankle systolic pressure divided by highest brachial systolic pressure. Normal is 1.0 to 1.4. A value of 0.90 or lower indicates peripheral artery disease; above 1.4 suggests noncompressible vessels.

#220 · Assessment and Diagnosis

How does intermittent claudication typically present?

Answer

Reproducible calf, thigh or buttock cramping pain with walking, relieved within minutes of standing rest. It indicates arterial insufficiency, in contrast with neurogenic claudication, which eases with spinal flexion.

#221 · Assessment and Diagnosis

In the Wells score for DVT, what total indicates a likely DVT in the two-level model?

Answer

A score of 2 or more means DVT is likely; 1 or less means unlikely. Points are added for active cancer, calf swelling, pitting edema and others, and 2 are subtracted if another diagnosis is as likely.

#222 · Assessment and Diagnosis

What calf swelling difference scores a point in the Wells DVT criteria, and where is it measured?

Answer

Calf swelling of more than 3 cm compared with the asymptomatic leg, measured 10 cm below the tibial tuberosity. Pitting edema confined to the symptomatic leg is a separate criterion.

#223 · Assessment and Diagnosis

How do arterial and venous ulcers differ in location and appearance?

Answer

Arterial: painful, punched-out, pale base, on toes or pressure points, with cool, hairless skin. Venous: shallow, irregular, moist, near the medial malleolus (gaiter area), with edema and brown hemosiderin staining.

#224 · Assessment and Diagnosis

What hemoglobin levels define anemia in adults according to the WHO?

Answer

Below 130 g/L in men and below 120 g/L in non-pregnant women. Typical signs include fatigue, pallor, exertional dyspnea and tachycardia, which can limit exercise tolerance.

#225 · Assessment and Diagnosis

What is the normal platelet count, and what exercise precautions apply to a client with a low count?

Answer

Normal is 150 to 400 ×10■/L. With thrombocytopenia, avoid heavy resistance, deep massage and high-impact activity, watch for bruising and petechiae, and check recent counts with the medical team.

#226 · Assessment and Diagnosis

What triggers can precipitate a vaso-occlusive crisis in sickle cell disease?

Answer

Dehydration, cold exposure, hypoxia, infection and overexertion. Encourage hydration, warm environments and pacing; severe pain, chest pain or fever needs urgent medical care.

#227 · Assessment and Diagnosis

What does the INR monitor, and what is the usual therapeutic range for most warfarin indications?

Answer

INR monitors warfarin's effect on clotting. The usual target is 2.0 to 3.0. A higher INR raises bleeding risk, so check the latest value before manual therapy or vigorous exercise.

#228 · Assessment and Diagnosis

A client on an anticoagulant falls and reports a new severe headache. What is the first action?

Answer

Treat as possible intracranial bleeding: stop treatment and arrange emergency medical assessment. Anticoagulated clients can bleed internally after minor trauma, and signs may be delayed.

#229 · Assessment and Diagnosis

What do fine crackles on auscultation typically indicate, and when do they occur?

Answer

Crackles reflect the popping open of small airways or fluid and secretions in the airways. Fine, late-inspiratory crackles suggest interstitial lung disease or pulmonary edema; coarse crackles suggest secretions.

#230 · Assessment and Diagnosis

What does expiratory wheeze indicate, and how does it differ from stridor?

Answer

Wheeze is a high-pitched sound from narrowed intrathoracic airways, as in asthma or COPD. Stridor is a loud inspiratory sound from upper airway obstruction and is an emergency.

#231 · Assessment and Diagnosis

What are normal resting pulse oximetry values, and when can the reading be unreliable?

Answer

Normal SpO2 is about 95 to 100 percent. Readings can be unreliable with poor peripheral perfusion, cold fingers, nail polish, motion, or carbon monoxide poisoning, which falsely raises the value.

#232 · Assessment and Diagnosis

What is the modified Medical Research Council (mMRC) dyspnea scale?

Answer

A 0 to 4 scale of breathlessness with activity. Grade 0 is breathless only with strenuous exercise; grade 4 is too breathless to leave the house or when dressing. It is used in COPD.

#233 · Assessment and Diagnosis

What normal arterial pH and PaCO2 ranges are used, and what pattern is respiratory acidosis?

Answer

Normal pH is 7.35 to 7.45 and PaCO2 is 35 to 45 mm Hg (4.7 to 6.0 kPa). Respiratory acidosis is low pH with high PaCO2, from inadequate ventilation.

#234 · Assessment and Diagnosis

What spirometry finding confirms airflow obstruction in COPD?

Answer

A post-bronchodilator FEV1/FVC ratio below 0.70. This persistent, not fully reversible obstruction distinguishes COPD from restrictive disease, where the ratio is normal or high.

#235 · Assessment and Diagnosis

How does GOLD grade COPD severity by FEV1 percent predicted?

Answer

GOLD 1 (mild) is 80 percent or more, GOLD 2 (moderate) is 50 to 79, GOLD 3 (severe) is 30 to 49, and GOLD 4 (very severe) is below 30, in patients with FEV1/FVC below 0.70.

#236 · Assessment and Diagnosis

What bronchodilator response supports a diagnosis of asthma on spirometry?

Answer

An increase in FEV1 of at least 12 percent and 200 mL after bronchodilator indicates significant reversibility. Reversibility is typical of asthma but is usually limited in COPD.

#237 · Assessment and Diagnosis

What are the three cardinal symptoms of a COPD exacerbation?

Answer

Increased dyspnea, increased sputum volume and increased sputum purulence. Recognizing these in a client prompts medical review, and exercise should be paused until the exacerbation is managed.

#238 · Assessment and Diagnosis

What signs of a severe asthma attack require emergency care?

Answer

Inability to speak full sentences, accessory muscle use, cyanosis, exhaustion, confusion, or a silent chest with little air movement. A silent chest signals near-complete obstruction and is ominous.

#239 · Assessment and Diagnosis

What spirometry pattern defines restrictive lung disease?

Answer

Reduced FVC and total lung capacity with a normal or increased FEV1/FVC ratio, such as 0.80 or higher. Causes include interstitial lung disease, chest wall deformity and neuromuscular weakness.

#240 · Assessment and Diagnosis

How does a tension pneumothorax present, and what is the response?

Answer

Sudden severe dyspnea, tachycardia, hypotension, tracheal deviation away from the affected side, and absent breath sounds. It is a life-threatening emergency requiring immediate 911 activation.

#241 · Assessment and Diagnosis

What chest findings are typical of a large pleural effusion?

Answer

Dullness to percussion, decreased or absent breath sounds, and reduced tactile fremitus over the fluid, sometimes with a pleural rub above it. Chest expansion is reduced on the affected side.

#242 · Assessment and Diagnosis

What findings on assessment suggest lobar pneumonia?

Answer

Fever, productive cough, pleuritic pain, crackles, bronchial breath sounds and dullness to percussion over the consolidated lobe. Increased tactile fremitus and egophony may also be present.

#243 · Assessment and Diagnosis

Which auscultation finding and symptoms suggest interstitial lung disease?

Answer

Fine, dry Velcro-like crackles at the lung bases, with progressive exertional dyspnea and a dry cough. Oxygen saturation often falls during exertion, so monitor SpO2 during exercise.

#244 · Assessment and Diagnosis

What gene and inheritance pattern cause cystic fibrosis?

Answer

Mutations in the CFTR gene on chromosome 7, inherited in an autosomal recessive pattern. Defective chloride transport produces thick mucus, causing lung infection and pancreatic insufficiency.

#245 · Assessment and Diagnosis

What sweat chloride value supports a diagnosis of cystic fibrosis?

Answer

A sweat chloride of 60 mmol/L or higher is diagnostic. Values of 30 to 59 mmol/L are intermediate and need further testing, such as genetic analysis.

#246 · Assessment and Diagnosis

What is bronchiolitis and which virus most commonly causes it?

Answer

A viral lower respiratory infection of infants, usually under 2 years, most often from respiratory syncytial virus (RSV). It causes wheeze, crackles, feeding difficulty and increased work of breathing.

#247 · Assessment and Diagnosis

What are the hallmark signs of croup in a child?

Answer

A barking cough, hoarse voice and inspiratory stridor, usually from parainfluenza virus in children aged 6 months to 3 years. A steeple sign on neck X-ray is classic.

#248 · Assessment and Diagnosis

What signs during exercise should make a child with congenital heart disease stop?

Answer

Worsening cyanosis, dizziness, chest pain, syncope or unusual dyspnea. Children with cyanotic lesions or pulmonary hypertension tolerate exertion poorly, so follow cardiology exercise clearance.

#249 · Assessment and Diagnosis

What causes silicosis, and which workers are most at risk of developing it?

Answer

Inhaling respirable crystalline silica dust causes fibrotic lung nodules, usually in the upper lobes. At risk: miners, quarry and stone workers, sandblasters, tunnellers and foundry workers after years of exposure.

#250 · Assessment and Diagnosis

Which serious infection is people with silicosis at increased risk of developing?

Answer

Tuberculosis. Silica damages alveolar macrophages and weakens lung defences, so new cough, fever, night sweats or weight loss in a client with silicosis should prompt physician review and TB testing.

#251 · Assessment and Diagnosis

What precautions apply when a client has suspected or confirmed active pulmonary tuberculosis?

Answer

Airborne precautions: a negative-pressure single room, door closed, and a fit-tested N95 respirator for staff. The client wears a surgical mask when leaving the room. Defer non-urgent treatment until cleared.

#252 · Assessment and Diagnosis

What classic symptom cluster should make you suspect pulmonary tuberculosis?

Answer

Productive cough lasting more than 3 weeks, often with hemoptysis, plus fever, night sweats, fatigue and unintentional weight loss. Report to the physician and apply airborne precautions while it is investigated.

#253 · Assessment and Diagnosis

How should exercise be paced for a client with persistent post-COVID dyspnea and post-exertional symptom worsening?

Answer

Use symptom-guided pacing: start low, progress slowly, monitor SpO2, heart rate and symptoms, and avoid pushing through fatigue. Worsening 24 to 48 hours later signals the dose was too high.

#254 · Assessment and Diagnosis

How is a stage 1 pressure injury defined?

Answer

Intact skin with a localized area of non-blanchable erythema, which may look different on darker skin tones (purple or maroon discolouration). Check with fingertip pressure and look for warmth, firmness or pain.

#255 · Assessment and Diagnosis

What separates a stage 3 from a stage 4 pressure injury?

Answer

Stage 3 is full-thickness skin loss with visible fat but no exposed fascia, muscle, tendon, ligament, cartilage or bone. Stage 4 exposes fascia, muscle, tendon, ligament, cartilage or bone.

#256 · Assessment and Diagnosis

Using the adult rule of nines, what percentage of total body surface area is each leg (front and back)?

Answer

18 percent per leg. Head 9, each arm 9, anterior trunk 18, posterior trunk 18, perineum 1. Used to estimate burn extent; the pediatric chart gives a larger head and smaller legs.

#257 · Assessment and Diagnosis

What does a healthy granulating wound bed look like?

Answer

Moist, bright red or beefy, bumpy tissue that bleeds easily. Pale or dusky granulation suggests poor perfusion or infection, and yellow slough or black eschar indicates non-viable tissue needing debridement.

#258 · Assessment and Diagnosis

How is stage 2 lymphedema described using the International Society of Lymphology staging?

Answer

Pitting becomes less evident and swelling does not reduce with limb elevation alone. Tissue fibrosis begins, so the limb feels firmer. Stage 1 is pitting edema that reduces with elevation.

#259 · Assessment and Diagnosis

What blood glucose level defines hypoglycemia in a person with diabetes according to Diabetes Canada, and how is it treated?

Answer

Below 4.0 mmol/L. If the client is alert, give 15 g of fast-acting carbohydrate (such as juice or glucose tablets), recheck in 15 minutes, and repeat if it is still below 4.0.

#260 · Assessment and Diagnosis

What presentation suggests an acute Charcot foot in a client with diabetic neuropathy?

Answer

A warm, red, swollen foot, often with little pain, and skin temperature more than about 2 degrees C above the other foot. Offload immediately (usually a total contact cast) and refer urgently.

#261 · Assessment and Diagnosis

Which findings suggest hypothyroidism in a client presenting with fatigue and musculoskeletal complaints?

Answer

Cold intolerance, weight gain, constipation, dry skin, bradycardia, slowed relaxation of deep tendon reflexes, and sometimes carpal tunnel syndrome or myalgia. Refer for thyroid function testing.

#262 · Assessment and Diagnosis

Which findings suggest hyperthyroidism rather than hypothyroidism?

Answer

Heat intolerance, weight loss despite good appetite, tachycardia or palpitations, fine tremor, anxiety and proximal muscle weakness. Check resting heart rate before exercise and refer if uncontrolled.

#263 · Assessment and Diagnosis

How are obesity classes defined by BMI in adults?

Answer

Class I is 30.0 to 34.9 kg/m2, class II is 35.0 to 39.9, and class III is 40 or higher. BMI is a screening measure and does not capture fat distribution or function.

#264 · Assessment and Diagnosis

What is diastasis recti, and how is it commonly assessed?

Answer

Widening of the linea alba between the rectus abdominis bellies. With the client in crook lying, lift the head slightly and palpate the gap above and below the umbilicus. More than about 2 cm is commonly considered wide.

#265 · Assessment and Diagnosis

How is the posterior pelvic pain provocation (P4 or thigh thrust) test performed, and what is positive?

Answer

Client supine with the hip flexed to 90 degrees; apply gentle downward force along the femur. Reproduction of familiar deep pain in the buttock or posterior pelvis is positive for pelvic girdle pain.

#266 · Assessment and Diagnosis

Why is placenta previa after 28 weeks an absolute contraindication to aerobic exercise in pregnancy?

Answer

Exercise and uterine activity raise the risk of bleeding from the low-lying placenta. Other absolute contraindications include ruptured membranes, premature labour, persistent unexplained bleeding, pre-eclampsia and an incompetent cervix.

#267 · Assessment and Diagnosis

Which findings suggest pre-eclampsia in a pregnant client?

Answer

New blood pressure of 140/90 mmHg or higher after 20 weeks with proteinuria or organ dysfunction. Symptoms include severe headache, visual disturbance, upper right abdominal pain and sudden facial or hand swelling. Refer urgently.

#268 · Assessment and Diagnosis

Which symptoms during prenatal exercise mean the client should stop and seek medical advice?

Answer

Vaginal bleeding, regular painful contractions, leaking fluid, dizziness or faintness, chest pain, severe headache, calf pain or swelling, and shortness of breath that does not settle with rest.

#269 · Assessment and Diagnosis

How does cancer-related fatigue differ from ordinary tiredness, and what is the main evidence-based treatment?

Answer

It is distressing, disproportionate to activity, and not relieved by rest. The strongest evidence supports regular aerobic and resistance exercise, with energy conservation and pacing adjusted to treatment cycles.

#270 · Assessment and Diagnosis

What does a Mirels score of 9 or higher indicate for a long bone metastasis?

Answer

High impending fracture risk, for which prophylactic fixation is generally recommended. The score uses site, pain, lesion type (lytic or blastic) and size. Avoid resisted or weight-bearing loading and contact the oncology team.

#271 · Assessment and Diagnosis

What absolute neutrophil count defines severe neutropenia?

Answer

Below 0.5 × 10■/L. Infection risk is very high, so use strict hand hygiene, clean equipment, avoid ill contacts and defer exercise in shared public spaces until the oncology team clears it.

#272 · Assessment and Diagnosis

What should you do if a client who is neutropenic has a temperature of 38.0 degrees C or higher?

Answer

Treat it as a medical emergency (febrile neutropenia). Stop treatment, do not start exercise, and arrange immediate medical assessment, because overwhelming sepsis can develop quickly.

#273 · Assessment and Diagnosis

How should a client with a fresh abdominal incision be taught to cough or sit up after surgery?

Answer

Splint the incision with a folded pillow or hands, and roll to the side to rise instead of sitting straight up. This reduces pain and strain on the wound while keeping airway clearance effective.

#274 · Assessment and Diagnosis

Does a client with HIV need special isolation precautions during physiotherapy?

Answer

No. Routine practices (standard precautions) apply to all clients, with hand hygiene, gloves for blood and body fluids, and sharps safety. HIV is not spread by casual contact or shared equipment.

#275 · Assessment and Diagnosis

Why is hepatitis B a higher occupational concern for physiotherapists than hepatitis C?

Answer

Hepatitis B is more infectious through blood and body fluid exposure, but a vaccine prevents it and health care workers should be immunized. There is no vaccine for hepatitis C.

#276 · Assessment and Diagnosis

Which hand hygiene method is required after contact with a client who has Clostridioides difficile infection?

Answer

Wash with soap and water, because alcohol-based hand rub does not reliably kill C. difficile spores. Use contact precautions with gown and gloves and clean equipment with a sporicidal agent.

#277 · Assessment and Diagnosis

What precautions apply when treating a client with MRSA infection or colonization?

Answer

Contact precautions: gown and gloves for direct care, hand hygiene, a dedicated or cleaned-between-clients set of equipment such as goniometers and plinths, and treating the client last or in a private space when possible.

#278 · Assessment and Diagnosis

Which features suggest systemic lupus erythematosus in a client with joint pain?

Answer

Malar (butterfly) rash, photosensitivity, oral ulcers, fatigue, non-erosive symmetrical arthritis, and serositis. Pace exercise around flares, advise sun protection, and monitor for steroid-related osteoporosis.

#279 · Assessment and Diagnosis

What does a score of 7 on the Clinical Frailty Scale mean?

Answer

Severely frail: completely dependent for personal care from any cause, physical or cognitive, but stable and not at high risk of dying within about 6 months. Scores run 1 (very fit) to 9 (terminally ill).

#280 · Assessment and Diagnosis

What are the three qSOFA criteria, and what does a positive screen mean?

Answer

Respiratory rate of 22 or more, altered mentation, and systolic blood pressure of 100 mmHg or less. Two or more suggests possible sepsis with a higher risk of poor outcome, so escalate urgently.

#281 · Assessment and Diagnosis

What MRC sum score indicates ICU-acquired weakness?

Answer

A sum score below 48 out of 60, from six muscle groups tested bilaterally (grade 0 to 5 each). The weakness is typically symmetrical, proximal and diffuse, in a client who was critically ill.

#282 · Assessment and Diagnosis

Why is polypharmacy important in a falls assessment, and which drug classes raise risk most?

Answer

Taking five or more medications raises fall risk. Fall-risk-increasing drugs include benzodiazepines, other sedatives, antipsychotics, antidepressants, opioids and some antihypertensives. Ask for a pharmacist medication review.

#283 · Assessment and Diagnosis

Which features define delirium on the Confusion Assessment Method?

Answer

Acute onset with a fluctuating course, plus inattention, plus either disorganized thinking or an altered level of consciousness. Delirium is a medical emergency, and hypoactive forms are easy to miss.

#284 · Assessment and Diagnosis

What does a client who answers no to all seven general PAR-Q+ questions do next?

Answer

They can become more physically active without further screening, and sign the declaration. Any yes answer leads to the follow-up questions, which may require review by a qualified exercise professional or physician.

#285 · Assessment and Diagnosis

What do the Canadian 24-Hour Movement Guidelines recommend for adults aged 18 to 64 for physical activity?

Answer

At least 150 minutes of moderate to vigorous aerobic activity per week, muscle-strengthening activities at least 2 days per week, and several hours of light activity, as part of good sleep and less sedentary time.

#286 · Assessment and Diagnosis

What Timed Up and Go time suggests increased fall risk in a community-dwelling older adult?

Answer

12 seconds or longer (the cut-off used in the CDC STEADI algorithm). Other cut-offs exist between studies, so interpret it with strength, balance and history rather than alone.

#287 · Assessment and Diagnosis

By what age is a child expected to walk independently, and when does it become a concern?

Answer

Most children walk independently between about 12 and 15 months. Not walking by 18 months, or any loss of previously gained skills, needs a pediatric developmental assessment or physician referral.

#288 · Assessment and Diagnosis

What rating on the Borg 6 to 20 scale corresponds to moderate exercise intensity?

Answer

About 12 to 14, described as somewhat hard. Each rating is roughly the heart rate divided by 10, so 13 equals about 130 beats per minute in a healthy adult, though medications change this.

#289 · Assessment and Diagnosis

Which non-drug strategies help relieve breathlessness at the end of life?

Answer

A handheld fan directed at the face, pursed-lip breathing, upright forward-leaning positions, a calm environment and pacing. Opioids are the usual drug option, prescribed by the physician.

#290 · Assessment and Diagnosis

What is the main aim of physiotherapy in palliative care?

Answer

To maintain function, comfort, dignity and independence for as long as possible according to the client's goals. Treatment goals shift as the condition advances, and care is shaped by goals-of-care discussions.

#291 · Assessment and Diagnosis

Why is a 30-degree side-lying tilt recommended for a bedbound client at high risk of pressure injury?

Answer

It keeps weight off the sacrum and the greater trochanter, which bear high pressure in a 90-degree side-lying position. Use pillows or wedges, and reposition on the schedule set by skin assessment.

#292 · Assessment and Diagnosis

Which tool do clinicians use to track symptom burden in palliative care, and what does it rate?

Answer

The Edmonton Symptom Assessment System rates pain, tiredness, drowsiness, nausea, lack of appetite, shortness of breath, depression, anxiety and well-being, each from 0 (none) to 10 (worst).

#293 · Assessment and Diagnosis

Which findings are red flags for cauda equina syndrome, and what is the action?

Answer

Saddle anesthesia, new urinary retention or incontinence, bowel dysfunction, sexual dysfunction and bilateral or progressive leg weakness. Send the client to emergency for urgent imaging and surgical review.

#294 · Assessment and Diagnosis

Which history and symptoms in a client with back pain raise suspicion of cancer?

Answer

Previous cancer, unexplained weight loss, age over 50, pain at night or at rest not relieved by position change, and failure to improve after one month. Refer to the physician for investigation.

#295 · Assessment and Diagnosis

Which calf finding in the Wells score for deep vein thrombosis is measured, and how?

Answer

Calf swelling of more than 3 cm compared with the other leg, measured 10 cm below the tibial tuberosity. Other items include unilateral pitting edema, localized tenderness and a recent immobilization or surgery.

#296 · Assessment and Diagnosis

What should you do if a client has chest pressure radiating to the jaw or left arm with sweating?

Answer

Stop activity, have the client rest, call 911 or the local emergency number, and monitor vital signs. Treat it as a possible acute coronary syndrome until a physician says otherwise.

#297 · Assessment and Diagnosis

What is the appropriate action if a client does not improve, or worsens, after a reasonable trial of treatment?

Answer

Reassess and reconsider the working diagnosis, screen again for red flags, and communicate with or refer to the physician or another provider. Continuing the same plan unchanged is not appropriate.

#298 · Care Planning

What does the PEACE part of the PEACE and LOVE acute soft tissue injury framework stand for?

Answer

Protect, Elevate, Avoid anti-inflammatories, Compress, Educate. It covers the first days after injury, when the focus is limiting excess movement and bleeding and teaching the patient about natural healing.

#299 · Care Planning

What does the LOVE part of the PEACE and LOVE framework stand for, and when does it apply?

Answer

Load, Optimism, Vascularization, Exercise. It applies after the first days: gradual pain-guided loading, a positive outlook, pain-free cardiovascular activity, then exercise to restore mobility, strength and proprioception.

#300 · Care Planning

How long should protection by rest typically last after an acute soft tissue injury?

Answer

Only about 1 to 3 days. Prolonged rest weakens tissue and slows recovery, so early pain-guided movement and loading should resume once the initial irritability settles.

#301 · Care Planning

What is the current evidence for ice as a treatment after acute soft tissue injury?

Answer

Evidence is limited. Ice may give short-term analgesia, but there is no strong proof it improves healing or function, and it may impair tissue repair. It should not be relied on as the main treatment.

#302 · Care Planning

Why does the PEACE framework advise avoiding anti-inflammatory medication in the early phase of a soft tissue injury?

Answer

Inflammation is part of normal tissue repair. Anti-inflammatories may blunt it and impair long-term healing of muscle, tendon and ligament, so the framework recommends avoiding them early.

#303 · Care Planning

In the Maitland grading system, what do grades I and II describe?

Answer

Small amplitude (grade I) and large amplitude (grade II) oscillations performed in the early to mid range, before tissue resistance. They are mainly used to reduce pain and modulate irritability.

#304 · Care Planning

In the Maitland grading system, what do grades III and IV describe?

Answer

Grade III is a large amplitude oscillation into resistance, grade IV a small amplitude oscillation at the end of range. They are used to stretch stiffness and restore joint mobility.

#305 · Care Planning

What is a grade V mobilization (manipulation)?

Answer

A small amplitude, high velocity thrust at the end of the available range. An audible cavitation may occur, but it is not required for a clinical effect.

#306 · Care Planning

Name four absolute contraindications to spinal manipulation.

Answer

Fracture, spinal cord compression or cauda equina syndrome, suspected cervical artery dysfunction, and spinal infection or malignancy. A positive finding on screening means no thrust technique, and the patient needs medical referral.

#307 · Care Planning

Before cervical manipulation, which structure's dysfunction must the physiotherapist screen for, and why?

Answer

The cervical arteries (vertebral and carotid). Neck pain and headache can be early signs of arterial dissection, and manipulation could worsen it, so a risk screen following the IFOMPT framework is required.

#308 · Care Planning

What do the letters in FITT stand for in exercise prescription?

Answer

Frequency, Intensity, Time (duration), Type. These four variables define the exercise dose, and each can be adjusted to progress or regress a program.

#309 · Care Planning

What principle states that the body adapts specifically to the demands placed on it?

Answer

Specificity (the SAID principle: specific adaptation to imposed demands). Training should match the task, such as muscle group, movement pattern, contraction type and speed that the patient needs to improve.

#310 · Care Planning

What is progressive overload?

Answer

Gradually increasing training demand, such as load, repetitions, sets or complexity, above what the tissue is used to. Without it, strength and fitness gains plateau.

#311 · Care Planning

Why do beginners gain strength quickly in the first few weeks of resistance training?

Answer

Early gains are mostly neural: better motor unit recruitment and coordination. Visible muscle hypertrophy follows later, usually after about 6 to 8 weeks of consistent training.

#312 · Care Planning

What is the CSEP recommendation for adults aged 18 to 64 for aerobic activity?

Answer

At least 150 minutes per week of moderate to vigorous aerobic activity, accumulated in bouts of any length, plus muscle strengthening activities using major muscle groups at least 2 days per week.

#313 · Care Planning

How does motor control training differ from strength training in dosage?

Answer

Motor control training uses low loads, high attention to movement quality, precise feedback and repetition of correct patterns. Strength training uses high loads near the muscle's capacity to drive tissue adaptation.

#314 · Care Planning

What does the STarT Back screening tool do in low back pain management?

Answer

It stratifies patients into low, medium and high risk of persistent disability using physical and psychosocial items (yellow flags), so treatment intensity can be matched to risk.

#315 · Care Planning

What is the core advice for a patient with acute nonspecific low back pain?

Answer

Reassure them, explain that the prognosis is good, advise them to stay active and return to normal activities as pain allows, and avoid prolonged bed rest. Routine imaging is not needed without red flags.

#316 · Care Planning

What is the usual natural course of acute nonspecific low back pain?

Answer

Most episodes improve substantially within about 6 weeks, though recurrence is common. Pain lasting beyond 12 weeks is classified as chronic.

#317 · Care Planning

What is meant by centralization in directional preference assessment?

Answer

With repeated movements in a direction, referred leg or buttock pain moves proximally toward the spine. It is a favourable prognostic sign and suggests that direction of movement may guide exercise.

#318 · Care Planning

What does a yellow flag indicate in low back pain?

Answer

A psychosocial risk factor for persistent disability, such as fear avoidance beliefs, catastrophizing, depression, or low job satisfaction. Yellow flags call for education, graded activity and sometimes multidisciplinary care.

#319 · Care Planning

What approach to return to work is recommended for a worker recovering from low back pain?

Answer

Early, graded return with modified duties or hours rather than waiting for full pain resolution. Work participation supports recovery and reduces the risk of long-term disability.

#320 · Care Planning

Which exercise type is a core component of guideline-based care for mechanical neck pain?

Answer

Neck and scapulothoracic strengthening plus deep cervical flexor and craniocervical control training. Exercise is supported by stronger evidence than passive modalities for neck pain.

#321 · Care Planning

What advice do guidelines give about cervical collars after whiplash injury?

Answer

Avoid prolonged collar use. Early advice to stay active and perform gentle neck movement leads to better recovery than immobilization, unless a fracture or instability has been identified.

#322 · Care Planning

In the Quebec Task Force classification of whiplash associated disorders, what is grade II?

Answer

Neck complaint with musculoskeletal signs such as decreased range of motion and point tenderness, without neurological signs. Grade III adds neurological signs, and grade IV is fracture or dislocation.

#323 · Care Planning

Which three muscles form the scapular upward rotation force couple?

Answer

Upper trapezius, lower trapezius and serratus anterior. Weakness or poor timing of these muscles, especially serratus anterior, alters scapular position and can contribute to shoulder pain.

#324 · Care Planning

Which exercises are commonly used to strengthen serratus anterior in shoulder rehabilitation?

Answer

Push-up plus and wall slides or punches with scapular protraction. They train protraction and upward rotation with relatively low activation of upper trapezius.

#325 · Care Planning

What is the usual first-line approach for rotator cuff related shoulder pain?

Answer

Education plus a progressive exercise program of rotator cuff and scapular strengthening, with load adjusted to pain. Trials show exercise gives results similar to surgery for many patients with subacromial pain.

#326 · Care Planning

After a rotator cuff repair, what is the typical early rehabilitation phase?

Answer

Sling protection with passive range of motion only, avoiding active contraction of the repaired tendon. Active motion and strengthening begin later, with timing set by the surgeon's protocol and repair size.

#327 · Care Planning

What are the three classic stages of adhesive capsulitis?

Answer

Freezing (painful, progressive stiffness), frozen (stiff, pain easing) and thawing (gradual return of motion). Treatment is matched to irritability: pain control early, mobility work later.

#328 · Care Planning

Which hip movements are restricted by posterolateral approach total hip arthroplasty precautions?

Answer

Hip flexion beyond 90 degrees, adduction past midline, and internal rotation. Combining them puts the new joint at risk of posterior dislocation. Precaution duration depends on the surgeon.

#329 · Care Planning

Which movement combination is the dislocation risk after an anterior approach total hip arthroplasty?

Answer

Extension combined with external rotation. Precautions after an anterior approach therefore avoid these movements, in contrast to the flexion, adduction and internal rotation limits of a posterior approach.

#330 · Care Planning

When should mobilization usually begin after total joint arthroplasty?

Answer

On the day of surgery or the first postoperative day. Early mobilization lowers the risk of blood clots and pneumonia and shortens hospital stay.

#331 · Care Planning

What is the stair technique with crutches or a cane after lower limb surgery?

Answer

Up with the good, down with the bad. The unaffected leg leads when climbing, and the affected leg and the aid lead when descending.

#332 · Care Planning

What does weight bearing as tolerated (WBAT) mean?

Answer

The patient may put as much weight through the limb as pain allows, guided by their symptoms. It differs from partial, touch-down and non-weight bearing, which set specific limits.

#333 · Care Planning

What are common early goals after total knee arthroplasty?

Answer

Full knee extension, flexion of about 90 degrees within the first weeks, quadriceps activation, edema control and independent walking with an aid. Lack of extension predicts a poor gait outcome.

#334 · Care Planning

What is the limb symmetry index (LSI) used in return to sport testing?

Answer

Involved limb performance divided by uninvolved limb performance, multiplied by 100. Many protocols require at least 90 percent on strength and hop tests before an athlete returns to pivoting sport.

#335 · Care Planning

Name the common hop tests used to assess readiness after ACL reconstruction.

Answer

Single hop for distance, triple hop for distance, crossover hop for distance and 6 metre timed hop. Together they assess power, control and symmetry between limbs.

#336 · Care Planning

Why is return to pivoting sport before about 9 months after ACL reconstruction discouraged?

Answer

Research shows reinjury risk falls as return is delayed until at least 9 months, and when strength and hop criteria are met. Early return carries a markedly higher risk of graft rupture.

#337 · Care Planning

Which early goal is the priority after ACL reconstruction?

Answer

Restoring full knee extension and controlling effusion, along with quadriceps activation. Loss of extension early can lead to persistent gait problems and anterior knee pain.

#338 · Care Planning

How does rehabilitation after meniscal repair differ from partial meniscectomy?

Answer

Repair protects the healing tissue, with limited weight bearing and limits on deep flexion early on. Partial meniscectomy allows quicker weight bearing and progression as symptoms settle.

#339 · Care Planning

Which ligament is most commonly injured in a lateral ankle sprain?

Answer

The anterior talofibular ligament (ATFL), usually from an inversion and plantar flexion mechanism. The calcaneofibular ligament is the next most often involved.

#340 · Care Planning

What is the preferred management of an acute grade I or II lateral ankle sprain?

Answer

Functional treatment: a brace or tape for support, early weight bearing and progressive exercise. This gives a faster return to activity than prolonged immobilization.

#341 · Care Planning

What type of training best reduces recurrence of lateral ankle sprains?

Answer

Balance and proprioceptive training, and continued use of a brace or tape for sport. Neuromuscular control exercise lowers the rate of repeat sprains.

#342 · Care Planning

What is the Alfredson protocol for midportion Achilles tendinopathy?

Answer

Eccentric heel drops with straight and bent knee, 3 sets of 15, twice daily, 7 days a week for 12 weeks. Pain during the exercise is allowed within tolerance.

#343 · Care Planning

What is the classic presentation of plantar fasciopathy?

Answer

Heel pain with the first steps in the morning or after rest, tender over the medial calcaneal origin. A positive windlass test supports the diagnosis. High load calf and foot strengthening helps.

#344 · Care Planning

What isometric dose is often used for pain relief in tendinopathy?

Answer

About 5 holds of 45 seconds at roughly 70 percent of maximal voluntary contraction. It can reduce pain short term and allows loading to start in painful, irritable tendons.

#345 · Care Planning

What is heavy slow resistance (HSR) training for tendinopathy?

Answer

Heavy loading performed slowly, about 3 seconds up and 3 seconds down, with load increasing as repetitions drop over roughly 12 weeks. It gives results comparable to eccentric training and is better tolerated.

#346 · Care Planning

In a pain monitoring model for tendinopathy, what pain level is generally acceptable during loading?

Answer

Up to about 5 out of 10, provided it settles by the next morning and does not worsen over 24 hours. Higher pain or lasting morning stiffness means the load should be reduced.

#347 · Care Planning

What are the four stages of secondary (indirect) fracture healing, in order?

Answer

Inflammation (hematoma), soft callus (cartilage), hard callus (woven bone), then remodelling. Loading and activity are progressed to match the stage, because early stress on soft callus can disrupt union.

#348 · Care Planning

Name common client factors that delay fracture healing and need to be addressed in the care plan.

Answer

Smoking, diabetes, poor nutrition, infection, poor blood supply, NSAID or corticosteroid use, and excessive motion at the fracture site. Counselling on smoking cessation and nutrition supports union.

#349 · Care Planning

What does weight bearing as tolerated (WBAT) mean after a lower limb fracture?

Answer

The client may load the limb as far as pain allows, from partial up to full weight, unless the surgeon states a limit. Use a gait aid and monitor pain and swelling.

#350 · Care Planning

Which findings after a distal radius fracture should raise suspicion of complex regional pain syndrome (CRPS)?

Answer

Pain out of proportion to the injury, allodynia, swelling, skin colour and temperature change, sweating change, and stiffness. Diagnosis uses the Budapest criteria. Early movement, edema control, and desensitization help prevention.

#351 · Care Planning

After casting, which pain pattern suggests acute compartment syndrome rather than normal fracture pain?

Answer

Pain out of proportion to the injury, worsened by passive stretch of the muscles in the compartment, with tense swelling. It is a surgical emergency, so arrange urgent medical assessment.

#352 · Care Planning

How is osteoporosis defined by bone mineral density using the WHO T-score?

Answer

A T-score of minus 2.5 or lower at the hip or spine on DXA scan. Osteopenia (low bone mass) is between minus 1.0 and minus 2.5. A fragility fracture can also establish the diagnosis.

#353 · Care Planning

Which spinal movements should a client with osteoporosis and vertebral compression fracture risk avoid?

Answer

Loaded forward flexion and combined flexion with twisting, such as sit-ups or toe touches with weights. Teach hip hinging, neutral spine, and extension-based posture and back strengthening instead.

#354 · Care Planning

What exercise program components are recommended to maintain bone health in adults with osteoporosis?

Answer

Progressive resistance training, weight-bearing impact activity as tolerated, balance training, and posture work. Skeletal loading must be progressive and individualized, with fracture history and pain guiding the intensity.

#355 · Care Planning

Which type of exercise is most effective in preventing falls in older adults living in the community?

Answer

Balance and strength training that is progressively challenging and done regularly, such as the Otago Exercise Programme. Add a home hazard review, footwear check, and medication review.

#356 · Care Planning

What does the FRAX tool estimate, and how is it used in osteoporosis management?

Answer

It estimates the 10-year probability of a major osteoporotic fracture and hip fracture from clinical risk factors, with or without bone density. It helps identify clients who need treatment and referral.

#357 · Care Planning

What are the first-line core treatments for knee and hip osteoarthritis?

Answer

Education, exercise (strengthening and aerobic), and weight management for those who are overweight. These come before medication or surgery and should be offered to every client with symptomatic osteoarthritis.

#358 · Care Planning

What is the GLA:D program, and what does it include?

Answer

Good Life with osteoArthritis in Denmark: patient education plus supervised neuromuscular exercise for hip and knee osteoarthritis, typically two education sessions and twelve exercise sessions over six weeks, with a home program.

#359 · Care Planning

How much does losing 1 kg of body weight reduce knee joint load with each step in walking?

Answer

About 4 kg of compressive load per step. This is why weight loss counselling is a core part of knee osteoarthritis care, along with exercise and activity pacing.

#360 · Care Planning

In which hand should a client with right hip osteoarthritis hold a cane, and why?

Answer

The left hand, opposite the painful hip. The cane and the right leg move together, and the cane's lever arm reduces the hip abductor force and joint load.

#361 · Care Planning

What advice is given for managing a flare-up of osteoarthritis pain?

Answer

Temporarily reduce load and modify activity, keep gentle movement going, use heat or cold for comfort, and return to the usual exercise program gradually. A written flare plan builds self-management confidence.

#362 · Care Planning

List joint protection principles taught to clients with rheumatoid arthritis.

Answer

Use larger, stronger joints, avoid prolonged static positions and tight gripping, respect pain, distribute load over several joints, and use assistive devices and ergonomic tools to lower stress on inflamed joints.

#363 · Care Planning

How does morning stiffness duration help differentiate rheumatoid arthritis from osteoarthritis?

Answer

Rheumatoid arthritis causes morning stiffness lasting longer than about 60 minutes, while osteoarthritis stiffness usually settles within 30 minutes. Prolonged stiffness suggests inflammatory disease and needs medical referral.

#364 · Care Planning

What exercise emphasis is recommended for a client with ankylosing spondylitis?

Answer

Daily spinal extension and mobility exercises, chest expansion and deep breathing, posture training, and aerobic activity. Stiffness improves with movement, not rest, which differs from mechanical back pain.

#365 · Care Planning

What is the usual long-term serum urate target in gout management, and what is the physiotherapist's education role?

Answer

Below 360 micromol/L, achieved with urate-lowering medication prescribed by the physician. The physiotherapist educates on adherence, flare management (rest the joint), weight, alcohol reduction, and gradual return to activity.

#366 · Care Planning

What is activity pacing for a client with inflammatory arthritis?

Answer

Balancing activity and rest through planned breaks, spreading tasks over the day, and reducing activity during flares. It prevents the cycle of overdoing on good days followed by painful days.

#367 · Care Planning

What are the typical parameters for conventional (high-frequency) TENS for pain relief?

Answer

Frequency about 80 to 120 Hz, short pulse duration (about 50 to 200 microseconds), comfortable sensory-level intensity without muscle contraction. It works mainly through the gate control mechanism and can be used for long periods.

#368 · Care Planning

How does acupuncture-like (low-frequency) TENS differ from conventional TENS?

Answer

Frequency is low, about 2 to 4 Hz, with longer pulses and a strong intensity that produces visible muscle twitching. It acts through endogenous opioid release, so onset is slower but relief lasts longer.

#369 · Care Planning

Which situations are contraindications or precautions for applying TENS?

Answer

Demand-type cardiac pacemaker, over the anterior neck or carotid sinus, over the pregnant uterus, broken or infected skin, and areas of impaired sensation. Never place electrodes across the chest in cardiac clients without medical clearance.

#370 · Care Planning

Over which areas should therapeutic ultrasound not be applied?

Answer

Over malignancy, the pregnant uterus, the eyes, and areas of impaired sensation or circulation. Avoid over metal implants with heat-producing modes, and keep the sound head moving to prevent hot spots.

#371 · Care Planning

How does interferential current (IFC) work, and why is it used?

Answer

Two medium-frequency currents (about 4000 Hz) cross to produce a low-frequency beat inside the tissue. Medium frequency lowers skin resistance, so deeper tissue is stimulated with more comfort. It is used for pain and edema.

#372 · Care Planning

Which conditions are contraindications to superficial heat application?

Answer

Acute inflammation or fresh trauma, impaired sensation, poor arterial circulation, active bleeding, malignancy over the area, and deep vein thrombosis. Heat increases blood flow and metabolism, which worsens these conditions.

#373 · Care Planning

Which client conditions are precautions or contraindications for cryotherapy?

Answer

Cold urticaria, Raynaud's phenomenon, cryoglobulinemia, impaired sensation, peripheral vascular disease, and open wounds with poor circulation. Check the client's skin response early in the first application.

#374 · Care Planning

What is the typical duration for applying an ice pack, and how is the skin protected?

Answer

About 10 to 20 minutes with a damp or dry towel barrier between the ice and the skin. Check skin colour and sensation before, during, and after to prevent a cold burn.

#375 · Care Planning

How is a contrast bath usually performed for a swollen hand or foot?

Answer

Alternate warm water (about 3 to 4 minutes) with cold water (about 1 minute), repeating several cycles and finishing with cold when edema is the goal. Avoid in impaired sensation or vascular disease.

#376 · Care Planning

Which properties of water make aquatic exercise useful for clients with painful weight-bearing joints?

Answer

Buoyancy unloads joints and allows movement with less pain, hydrostatic pressure helps reduce edema, and water resistance strengthens muscles. Open wounds, uncontrolled incontinence, and infection are contraindications.

#377 · Care Planning

How is the correct height of a cane or walker set for an adult?

Answer

The handle sits at the level of the greater trochanter or wrist crease when standing, giving about 20 to 30 degrees of elbow flexion when gripped. Too high or low causes poor posture and shoulder strain.

#378 · Care Planning

How are crutches fitted to avoid nerve injury under the arm?

Answer

Crutch pads sit about two to three finger widths (about 5 cm) below the axilla. Weight goes through the hands and not the armpit, because pressure on the axilla can cause radial nerve (crutch) palsy.

#379 · Care Planning

Which gait pattern with crutches is used for a client who must stay non-weight-bearing on one leg?

Answer

Three-point gait: both crutches and the unaffected leg bear weight, while the affected leg is held off the ground. Swing-to and swing-through patterns are options with good balance and upper body strength.

#380 · Care Planning

What is the rule for climbing and descending stairs with crutches or a cane?

Answer

Up with the good leg first, down with the affected leg and the aid first. The memory hook is: up with the good, down with the bad. Always guard from behind going up and in front going down.

#381 · Care Planning

When is a four-point gait pattern with two crutches chosen?

Answer

For clients with poor balance, weakness in both legs, or ataxia, who need a slow, stable pattern that keeps three points of contact on the ground at all times. It is slower than three-point or swing gait.

#382 · Care Planning

What is the main indication for an ankle-foot orthosis (AFO) in gait rehabilitation?

Answer

Foot drop from dorsiflexor weakness, such as after stroke or peroneal nerve injury. The AFO holds the ankle near neutral so the toes clear the floor in swing phase and heel strike is safer.

#383 · Care Planning

How is a new orthosis introduced into daily use?

Answer

Use a graded wearing schedule, adding time gradually, and check skin after each period. Redness that does not fade within about 20 to 30 minutes of removal signals excess pressure and needs adjustment.

#384 · Care Planning

What are the usual phases of rehabilitation for a person after lower limb amputation?

Answer

Preoperative, acute postoperative, pre-prosthetic, prosthetic fitting and training, and community reintegration with long-term follow-up. Each phase has goals for wound healing, limb shaping, strength, and function.

#385 · Care Planning

How is a residual limb wrapped with an elastic bandage to control edema and shape it?

Answer

Use diagonal or figure-of-eight turns from distal to proximal, with the most pressure distally and decreasing proximally, and cover the joint above. Avoid circular wraps that create a tourniquet effect.

#386 · Care Planning

Which positions should a person with a transfemoral amputation avoid to prevent hip contracture?

Answer

Prolonged hip flexion, abduction, and external rotation, such as a pillow under the residual limb or long sitting. Teach lying prone and hip extension exercises to maintain range.

#387 · Care Planning

What is the core message of pain neuroscience education for a client with chronic pain?

Answer

Pain is a protective output of the nervous system and does not always equal tissue damage. In persistent pain the system becomes sensitized, so hurt does not mean harm, which reduces fear and supports movement.

#388 · Care Planning

What is graded exposure in the management of chronic pain with fear-avoidance?

Answer

Building a hierarchy of feared or avoided activities and gradually doing them, beginning with the least threatening, to reduce fear and improve function. Progress is time-based and does not depend on pain levels.

#389 · Care Planning

What is the difference between activity pacing and boom-bust behaviour in chronic pain?

Answer

Pacing sets activity quotas by time and gradually increases them regardless of pain on that day. Boom-bust is overdoing on good days, then resting for days after a flare, which maintains deconditioning and pain.

#390 · Care Planning

What are yellow flags in the assessment of a client with persistent pain?

Answer

Psychosocial risk factors for chronicity, such as catastrophizing, fear-avoidance beliefs, low mood, poor coping, and job dissatisfaction. They guide the care plan toward education, behaviour change, and interdisciplinary referral.

#391 · Care Planning

By the IASP definition, after how long is pain considered chronic?

Answer

When it persists or recurs for longer than three months. Chronic primary pain is a condition in its own right, while chronic secondary pain arises from another disease or injury.

#392 · Care Planning

How do stress urinary incontinence and urge urinary incontinence differ?

Answer

Stress incontinence is leakage with effort such as coughing, sneezing, or lifting, due to weak sphincter and pelvic floor support. Urge incontinence is leakage with a sudden strong need to void, linked to bladder overactivity.

#393 · Care Planning

What is the first-line physiotherapy treatment for stress urinary incontinence in women?

Answer

Supervised pelvic floor muscle training, with proper contraction technique confirmed (squeeze and lift), then progressive strength and endurance holds plus a trained 'knack' contraction before coughing or lifting.

#394 · Care Planning

What does bladder training involve for overactive bladder?

Answer

Scheduled voiding with gradually lengthened intervals toward a goal of about 3 to 4 hours, plus urge suppression using pelvic floor contractions and relaxation. A bladder diary tracks fluid intake, voiding, and leaks.

#395 · Care Planning

What is the role of biofeedback in pelvic floor rehabilitation?

Answer

It gives visual or auditory feedback of pelvic floor muscle activity from EMG or ultrasound, helping the client learn correct contraction and relaxation, especially when she cannot feel or isolate the muscles.

#396 · Care Planning

What conservative measures are used for symptomatic mild to moderate pelvic organ prolapse?

Answer

Pelvic floor muscle training, constipation management, avoiding heavy lifting and straining, weight management, and a vaginal pessary when appropriate. Surgical referral is considered when conservative care fails.

#397 · Care Planning

Which muscle is shortened in congenital muscular torticollis, and what head posture results?

Answer

The sternocleidomastoid is shortened or fibrotic. The head tilts toward the affected side while the chin rotates toward the opposite side. Early stretching and positioning give the best outcomes.

#398 · Care Planning

What is the purpose of a Pavlik harness, and what position error must be avoided?

Answer

It holds the infant hip flexed and abducted in a safe zone to keep the femoral head centred in developmental hip dysplasia. Forced or excessive abduction risks avascular necrosis of the femoral head.

#399 · Care Planning

What Cobb angle defines scoliosis on a standing radiograph?

Answer

A lateral curve with a Cobb angle of 10 degrees or more, with vertebral rotation. Smaller curves are called asymmetry. Cobb angle is the standard measure for tracking progression.

#400 · Care Planning

In which adolescent scoliosis curves is bracing typically considered?

Answer

Skeletally immature adolescents with curves of about 25 to 40 degrees and remaining growth. The goal is to prevent progression, not to correct the curve. Larger curves are referred for surgical opinion.

#401 · Care Planning

A child has tenderness over a growth plate and a normal radiograph. How should it be managed?

Answer

Treat as a Salter-Harris type I physeal fracture until proven otherwise: protect, immobilize, and refer. Missed growth plate injury can cause growth arrest or angular deformity.

#402 · Care Planning

What does the acronym SMART stand for in goal setting?

Answer

Specific, measurable, achievable, relevant, and time-bound. A goal such as walking 500 metres without a cane in four weeks meets all five criteria and can be checked at reassessment.

#403 · Care Planning

Name the components of the ICF framework used to organize physiotherapy goals.

Answer

Body functions and structures, activities, and participation, shaped by environmental and personal contextual factors. Goals at activity and participation levels reflect what matters most to the patient.

#404 · Care Planning

How is goal attainment scaling scored?

Answer

Each goal is rated from minus 2 to plus 2. Zero is the expected outcome, minus values are less than expected, and plus values are better than expected. It allows individualized goals to be compared.

#405 · Care Planning

Which factors guide how often and how long a patient needs physiotherapy?

Answer

Tissue healing stage, severity and irritability, patient function and goals, comorbidities, self-management ability, and access or funding. Dosing is reassessed as the patient responds.

#406 · Care Planning

Why write goals at the activity and participation level rather than as impairments only?

Answer

They describe what the patient wants to do, such as climbing stairs at home, and can be measured by performance. Impairment gains like range of motion do not guarantee functional change.

#407 · Care Planning

How is a Grade II hamstring strain defined?

Answer

A partial tear of muscle fibres with noticeable loss of strength, bruising, and pain with activity. Grade I is minor fibre damage, and Grade III is a complete rupture.

#408 · Care Planning

Which exercise type is emphasized in hamstring strain rehabilitation and prevention?

Answer

Eccentric hamstring strengthening, such as the Nordic hamstring exercise, progressed gradually. It improves load tolerance of the muscle-tendon unit and lowers recurrence risk in sport.

#409 · Care Planning

What is myositis ossificans and what early treatment should be avoided?

Answer

Heterotopic bone forming within muscle after a contusion, commonly the quadriceps. Avoid early deep massage, heat, and aggressive passive stretching, which can provoke further bleeding and ossification.

#410 · Care Planning

What does criteria-based return to play mean?

Answer

The athlete progresses by meeting objective criteria such as pain-free movement, strength symmetry, and sport-specific function tests, not by elapsed time alone. It lowers the risk of premature return.

#411 · Care Planning

What does the POLICE principle for acute soft tissue injury stand for?

Answer

POLICE (protection, optimal loading, ice, compression, elevation) is an older framework that has been superseded by PEACE and LOVE, because evidence for ice and routine compression is weak. Its useful idea was optimal loading in place of complete rest, since controlled early movement supports tissue healing.

#412 · Care Planning

What splint position is used for conservative management of carpal tunnel syndrome?

Answer

A night wrist splint holding the wrist in neutral. Flexed or extended wrist positions raise carpal tunnel pressure, so neutral keeps pressure on the median nerve lowest during sleep.

#413 · Care Planning

Which skin areas are affected in carpal tunnel syndrome, and which are spared?

Answer

Palmar thumb, index, middle, and radial half of the ring finger are affected. The thenar eminence skin is spared because the palmar cutaneous branch leaves the median nerve before the tunnel.

#414 · Care Planning

What is the purpose of tendon gliding exercises after hand surgery or injury?

Answer

They promote independent gliding of the flexor digitorum superficialis and profundus tendons to limit adhesions. The sequence moves through straight, hook, full fist, tabletop, and straight fist positions.

#415 · Care Planning

What is the first-line approach to hand edema control?

Answer

Elevate the hand above heart level with active finger and wrist movement, and add compression or retrograde massage as appropriate. Persistent edema leads to stiffness and adhesions.

#416 · Care Planning

What is the intrinsic-plus (safe) hand position used in splinting?

Answer

Wrist slightly extended, MCP joints flexed about 70 to 90 degrees, and IP joints extended. It keeps collateral ligaments lengthened and prevents contracture of the MCP and IP joints.

#417 · Care Planning

What is the purpose of a functional capacity evaluation?

Answer

To objectively measure what a person can safely do physically, compared with job demands, to guide return-to-work decisions. Results depend on consistent effort and use standardized, reliable tests.

#418 · Care Planning

What does a job demands analysis document?

Answer

The physical, cognitive, and environmental requirements of a job, such as lifting loads, postures, repetition, and pace. It gives a target for rehabilitation and for matching worker to tasks.

#419 · Care Planning

What is the principle of graded return to work?

Answer

Hours and duties are increased in steps, often with modified tasks, coordinated with the employer and worker. It builds tolerance, supports confidence, and lowers the risk of re-injury.

#420 · Care Planning

In the hierarchy of controls, which workplace ergonomic measures come first?

Answer

Eliminate the hazard or use engineering controls, such as an adjustable workstation or lift aid, before administrative controls and personal protective equipment. Higher controls protect all workers, not just one.

#421 · Care Planning

What does the duty to accommodate require of Canadian employers?

Answer

Under human rights legislation, employers must adapt work for an injured worker's needs up to the point of undue hardship. Physiotherapists provide restrictions and abilities, not just a diagnosis.

#422 · Care Planning

What do neuromuscular injury prevention programs like FIFA 11+ include, and what do they reduce?

Answer

Running warm-up, strength, balance, and plyometric exercises with landing technique. Used consistently as a warm-up, they reduce lower-limb injuries such as ACL and ankle sprains in team sport athletes.

#423 · Care Planning

Why does load management matter in injury prevention?

Answer

Sudden spikes in training load relative to what the athlete is accustomed to raise injury risk. Gradual progression of volume and intensity lets tissues adapt and keeps chronic load high.

#424 · Care Planning

What is the immediate action for an athlete with a suspected concussion?

Answer

Remove from play immediately, assess, and do not return to sport that day. Red flags such as worsening headache, repeated vomiting, or confusion need urgent medical assessment.

#425 · Care Planning

How does stepwise return to sport after concussion progress?

Answer

Each step lasts at least 24 hours, moving from symptom-limited activity to aerobic exercise, sport drills, non-contact training, then full practice. If symptoms return, go back to the previous step.

#426 · Care Planning

In the Canadian Guideline on Concussion in Sport, what order is followed for school and sport?

Answer

Return to school comes before return to sport. A student should manage a full school day without symptom worsening before starting contact sport, and medical clearance is needed for full contact.

#427 · Care Planning

What is a practical strategy to improve home exercise program adherence?

Answer

Prescribe a few key exercises, give written or video instructions, tie them to an existing daily routine, and agree on a plan with the patient. Long, complex programs reduce adherence.

#428 · Care Planning

What does OARS stand for in motivational interviewing?

Answer

Open questions, affirmations, reflective listening, and summarizing. These skills draw out the patient's own reasons for change and support autonomy rather than telling them what to do.

#429 · Care Planning

What is self-efficacy and how can a physiotherapist build it?

Answer

Confidence in one's ability to carry out a behaviour. It grows through mastery experiences: start with achievable exercises, progress gradually, and give feedback on success.

#430 · Care Planning

Name the stages of the transtheoretical model of behaviour change.

Answer

Precontemplation, contemplation, preparation, action, and maintenance. Matching advice to the stage works better than giving action plans to someone who is not yet ready to change.

#431 · Care Planning

What should a self-management plan for a chronic musculoskeletal condition include?

Answer

Core exercises, activity pacing, a flare-up plan, and clear signs of when to seek help. It lets the patient manage symptoms independently and lowers reliance on repeated treatment visits.

#432 · Care Planning

What must be obtained and documented before dry needling?

Answer

Informed consent covering the procedure, benefits, and risks such as bruising, pain, and rare pneumothorax. The patient must be able to decline or stop at any time, and the discussion is documented.

#433 · Care Planning

How is dry needling regulated in Canadian physiotherapy practice?

Answer

Authorization and training requirements vary by province and regulatory college. A physiotherapist may perform it only if their college permits it and they hold the required competence.

#434 · Care Planning

Name a contraindication to dry needling at a site.

Answer

Local skin infection, or patient refusal. Bleeding disorders, anticoagulant use, and lymphedema in the limb need extra caution or avoidance. Needle phobia also limits use.

#435 · Care Planning

What does the evidence say about kinesiology taping for musculoskeletal pain?

Answer

Trials show small or no clinically important benefit over sham taping. Tape may still be used as an adjunct for proprioceptive cueing, with skin checked for adhesive allergy or irritation.

#436 · Care Planning

When is soft tissue mobilization contraindicated over an area?

Answer

Over a suspected deep vein thrombosis, open wounds, skin infection, or known malignancy at the site. These carry risk of embolism, spread of infection, or tumour disruption.

#437 · Care Planning

How does water immersion level affect weight bearing in aquatic therapy?

Answer

Buoyancy unloads the joints. Standing at chest level, the body bears only about 25 to 35 percent of its weight (about 50 percent at waist level, about 10 percent at neck level). Shallower water gives more loading, so depth is chosen for the weight-bearing goal.

#438 · Care Planning

Why is caution needed with aquatic therapy in patients with heart failure?

Answer

Hydrostatic pressure pushes fluid toward the chest and increases venous return and cardiac preload. This can overload a failing heart, so such patients need medical clearance and monitoring.

#439 · Care Planning

Name situations in which pool therapy should be avoided.

Answer

Open wounds, active infection, uncontrolled seizures, and fecal incontinence. These risk infection spread or safety problems in the water, so they are standard precautions to screen.

#440 · Care Planning

Why is perceived exertion preferred for monitoring aerobic exercise in water?

Answer

Heart rate responses are lower during immersion than on land at the same effort. Rating of perceived exertion gives a more reliable measure of intensity than target heart rate alone.

#441 · Care Planning

What do CSEP guidelines recommend for adult physical activity weekly?

Answer

At least 150 minutes of moderate to vigorous aerobic activity per week, plus muscle strengthening on at least two days. Patients with musculoskeletal conditions work toward this gradually.

#442 · Care Planning

What are common criteria for discharging a patient from musculoskeletal physiotherapy?

Answer

Goals are met, progress has plateaued, the patient can self-manage, or the patient chooses to stop. Reasons and outcomes are documented, and referral is considered if goals remain unmet.

#443 · Care Planning

What should a maintenance program at discharge include?

Answer

A written exercise program with a plan for progression, guidance on activity levels, and advice on how often to continue. It protects gains and supports long-term self-management.

#444 · Care Planning

What is a relapse plan in discharge planning?

Answer

A set of steps the patient follows if symptoms flare, including self-care actions, warning signs, and when to book a return visit or seek medical care. It reduces fear and delays in seeking help.

#445 · Care Planning

Give an example of a community program for transition after discharge for knee or hip osteoarthritis.

Answer

GLA:D Canada, an education and exercise program delivered by trained physiotherapists. Community programs support continued activity, social support, and long-term management after formal care ends.

#446 · Care Planning

What belongs in a discharge summary for musculoskeletal care?

Answer

Outcome measures compared with baseline, goals achieved, remaining problems, and recommendations. It is shared with the referral source or other providers with the patient's consent.

#447 · Care Planning

What are the key features of task-specific training after stroke, and why is it favoured over general exercise?

Answer

Practise the actual functional task (reaching, sit to stand, walking) with many repetitions and progressive challenge. Neuroplasticity is specific, so gains transfer best to the task trained.

#448 · Care Planning

What motor criterion is usually required before offering constraint-induced movement therapy to a client after stroke?

Answer

Some residual voluntary movement in the affected hand, typically about 20 degrees of active wrist extension and 10 degrees of finger extension. The unaffected hand is restrained during much of the waking day while the affected arm trains intensively.

#449 · Care Planning

How should a physiotherapist handle a hemiplegic shoulder to reduce the risk of subluxation and pain?

Answer

Support the arm during transfers and positioning, handle the scapula and trunk rather than pulling on the arm, and avoid overhead pulleys. Traction on a flaccid shoulder stretches the capsule and promotes pain.

#450 · Care Planning

What is the role of body weight supported treadmill training in stroke gait rehabilitation?

Answer

It permits high repetition stepping practice with safe harness support for clients who cannot yet walk well overground. Support is reduced and speed is increased as the client improves, to build walking speed and endurance.

#451 · Care Planning

What is a sound positioning principle in bed for a client with hemiplegia after stroke?

Answer

Alternate positions, including side lying on the affected side with the arm forward and scapula protracted. Avoid prolonged supine lying, which promotes extensor tone and shoulder retraction, and give regular position changes to protect skin.

#452 · Care Planning

In motor learning, what is the difference between knowledge of results and knowledge of performance?

Answer

Knowledge of results is feedback about the outcome of the movement (did the cup reach the target). Knowledge of performance is feedback about movement pattern or quality (trunk leaned forward during the reach).

#453 · Care Planning

Which practice schedule favours long-term retention and transfer: blocked or random practice?

Answer

Random (variable) practice. Blocked practice (one task repeated) gives better performance during early acquisition, but random practice produces better retention and transfer, so progress from blocked to random as the client improves.

#454 · Care Planning

Why is giving feedback after every single attempt not ideal when teaching a motor skill?

Answer

Constant feedback can create dependence and weaken retention when it is withdrawn. Reduced or faded feedback frequency, with summary feedback after several trials, encourages the learner to use their own error detection.

#455 · Care Planning

When is whole-task practice preferred over part-task practice in motor learning?

Answer

Whole practice suits simple or highly integrated tasks whose parts depend on each other, such as gait. Part practice suits complex, serial tasks with separable components, and should be reintegrated into the whole task soon after.

#456 · Care Planning

Name the principles of experience-dependent neuroplasticity that guide intensity and dosage in neurological rehabilitation.

Answer

Use it or lose it, use it and improve it, specificity, repetition matters, intensity matters, salience (task must be meaningful), and timing matters. High repetition and challenge drive cortical reorganization.

#457 · Care Planning

What is the usual sequence of return to activity after a sport concussion in Canadian guidelines?

Answer

Relative rest, symptom-limited activity, light aerobic exercise, sport-specific exercise, non-contact drills, then full practice after medical clearance, then game play. Progress only if symptoms have not worsened at the current stage, usually for at least 24 hours before moving to the next stage.

#458 · Care Planning

How should a physiotherapist manage a client with traumatic brain injury who is agitated and confused (Rancho Los Amigos level IV)?

Answer

Reduce stimulation: quiet room, short sessions, familiar staff, one simple instruction at a time, and no restraint of movement where avoidable. Keep the environment safe rather than correcting or arguing.

#459 · Care Planning

What does the Buffalo Concussion Treadmill Test determine, and how is it used in rehabilitation?

Answer

It finds the heart rate at which symptoms are first provoked. Subthreshold aerobic exercise, typically about 80 percent of that heart rate, is then prescribed to promote recovery without worsening symptoms.

#460 · Care Planning

Which cognitive strategies help a client with memory impairment after traumatic brain injury during therapy?

Answer

Use errorless learning, chunking of tasks, consistent routines, repetition, and external aids such as notebooks, calendars and phone alarms. Giving one-step commands reduces cognitive load and error.

#461 · Care Planning

What Glasgow Coma Scale scores define mild, moderate and severe traumatic brain injury?

Answer

Mild is 13 to 15, moderate is 9 to 12, and severe is 8 or less. The scale scores eye opening, verbal response and motor response, with a total range of 3 to 15.

#462 · Care Planning

What is the first action for a client with spinal cord injury at T6 or above who develops autonomic dysreflexia?

Answer

Sit the client upright, lower the legs, loosen tight clothing or binders, and check the bladder and bowel for the trigger, such as a kinked catheter. Monitor blood pressure and call emergency help if it persists.

#463 · Care Planning

How often should a wheelchair user with spinal cord injury perform pressure relief, and why?

Answer

Typically every 15 to 30 minutes, held for 1 to 2 minutes, by push-up, forward lean or side lean. This restores blood flow over the ischial tuberosities and prevents pressure injuries.

#464 · Care Planning

Which muscle is key for independent level transfers in a client with a C7 spinal cord injury?

Answer

The triceps, which stabilizes the elbow in extension so the arms can depress the body. Sliding board transfers are used when full lifting is not possible, and shoulder preservation must be taught.

#465 · Care Planning

Why do clients with cervical spinal cord injury need an assisted cough, and how is it performed?

Answer

Paralyzed abdominal muscles cannot generate cough force. The clinician applies a timed upward abdominal thrust as the client exhales, below the rib cage, to help clear secretions and prevent pneumonia.

#466 · Care Planning

How does moving the rear axle of a manual wheelchair forward affect the chair?

Answer

It makes propulsion easier and the chair more manoeuvrable, but it reduces rearward stability and increases tipping risk. Anti-tip devices are often used while the client learns the balance point.

#467 · Care Planning

Why are external cues effective for freezing of gait in Parkinson disease?

Answer

The basal ganglia are impaired in generating automatic, internally timed movement. Visual cues (floor lines) or auditory cues (metronome, rhythmic counting) shift control to intact, attention-based cortical pathways.

#468 · Care Planning

What is the standard dosage of LSVT BIG for Parkinson disease?

Answer

Four consecutive weeks of one-hour sessions, four days per week, with daily home practice. It trains large-amplitude whole-body movement to counter bradykinesia and hypokinesia.

#469 · Care Planning

When should a physiotherapist schedule a session for a client with Parkinson disease relative to levodopa dosing?

Answer

During the medication peak, the "on" period, typically about one hour after a dose. Mobility is best and the client can engage in more intense training with less risk.

#470 · Care Planning

At which Hoehn and Yahr stage does postural instability first appear in Parkinson disease?

Answer

Stage 3, with impaired righting reflexes and bilateral involvement, while the client is still physically independent. Balance training and fall prevention become a priority at this point.

#471 · Care Planning

What aerobic exercise target is recommended for people with Parkinson disease?

Answer

Moderate to vigorous aerobic exercise, working toward 150 minutes per week, plus strength and balance training. Exercise improves mobility, and treadmill walking can be progressed in speed and incline.

#472 · Care Planning

What is Uhthoff phenomenon in multiple sclerosis, and how is it managed in rehabilitation?

Answer

A temporary worsening of symptoms with rising body temperature that resolves with cooling. Use cool environments, cooling vests, cold drinks and pre-cooling before exercise, and avoid hot baths or overheating.

#473 · Care Planning

What are the four Ps of energy conservation taught for MS fatigue?

Answer

Pace, plan, prioritize and position. Spread tasks through the day, schedule demanding activity when energy is highest, drop low-value tasks, and use efficient posture and seated work.

#474 · Care Planning

What exercise guidelines exist for adults with mild to moderate MS disability?

Answer

Aerobic exercise at least twice weekly for 30 minutes at moderate intensity, plus strength training of major muscle groups at least twice weekly. These Canadian MS guidelines improve fitness and function.

#475 · Care Planning

How should exercise be structured to avoid triggering excessive fatigue in a client with MS?

Answer

Use short bouts with rest breaks, moderate intensity, a cool setting, and the time of day with least fatigue, often morning. Avoid exercising to exhaustion and monitor symptoms over the next day.

#476 · Care Planning

How does a pseudo-exacerbation differ from a true relapse in MS?

Answer

A pseudo-exacerbation is a temporary worsening of symptoms from heat, infection or stress that resolves within 24 hours. A true relapse involves new or worsening neurological symptoms lasting longer than 24 hours.

#477 · Care Planning

What respiratory measures (the 20, 30, 40 rule) suggest impending respiratory failure in Guillain-Barre syndrome?

Answer

Vital capacity below 20 mL/kg, maximal inspiratory pressure weaker than minus 30 cmH2O, and maximal expiratory pressure below 40 cmH2O. Such values prompt consideration of ventilatory support.

#478 · Care Planning

What exercise approach is recommended for a client in the recovery phase of Guillain-Barre syndrome?

Answer

Gentle, low-intensity, submaximal exercise with frequent rests. Overwork fatigue can set back recovery, so avoid fatiguing resistance training and watch for autonomic instability such as blood pressure swings.

#479 · Care Planning

What exercise precaution applies to a client with ALS?

Answer

Use submaximal, low-resistance exercise and avoid overwork weakness. Stop or reduce the program if fatigue or weakness persists into the next day. Range of motion and stretching help limit contractures.

#480 · Care Planning

When is non-invasive ventilation usually considered in ALS?

Answer

When forced vital capacity falls to about 50 percent of predicted or sooner, or with symptoms of nocturnal hypoventilation, such as morning headache. Mechanical cough assist is added when cough strength declines.

#481 · Care Planning

What foot care advice should a physiotherapist give to a client with diabetic peripheral neuropathy?

Answer

Inspect feet daily, wear well-fitting protective footwear, never walk barefoot, and check bath water with a thermometer or the elbow. Loss of sensation allows unnoticed injury and ulceration.

#482 · Care Planning

Which orthosis and equipment are typically planned for foot drop and progressive weakness in neuromuscular disease?

Answer

An ankle-foot orthosis for foot drop, a collar for head drop in ALS, and early power wheelchair assessment as the disease progresses. Anticipating equipment avoids delay when function declines.

#483 · Care Planning

What does a GMFCS level III child with cerebral palsy typically use for mobility?

Answer

A hand-held mobility device such as a walker indoors, and wheeled mobility for longer distances. Level I walks without limitation, level II walks with limitations on uneven surfaces, IV needs powered mobility, V is transported.

#484 · Care Planning

Why is the GMFCS used when setting goals for children with cerebral palsy?

Answer

It classifies self-initiated movement, sitting and mobility by age band into five levels, and predicts likely motor development. Goals and equipment are matched to the child's level rather than to typical development.

#485 · Care Planning

Which children with cerebral palsy have the highest risk of hip displacement, and what measure is used to monitor it?

Answer

Children at GMFCS levels IV and V. Hip surveillance with radiographs measures the Reimers migration percentage, where a value above 30 percent is concerning and prompts orthopedic referral.

#486 · Care Planning

How does a solid ankle-foot orthosis differ from a hinged one in a child with cerebral palsy?

Answer

A solid AFO blocks ankle plantar flexion and dorsiflexion to control equinus and stabilize stance. A hinged AFO allows dorsiflexion for sit to stand and stairs while still limiting plantar flexion.

#487 · Care Planning

What is family-centred care in pediatric neurological rehabilitation?

Answer

Parents and the child are partners who set goals and share decisions. Tools such as the Canadian Occupational Performance Measure and Goal Attainment Scaling capture the family's own priorities.

#488 · Care Planning

Which canal and finding does the Epley maneuver treat?

Answer

Posterior canal benign paroxysmal positional vertigo, confirmed by a positive Dix-Hallpike test with upbeating torsional nystagmus. The head is moved through positions to move otoconia out of the canal.

#489 · Care Planning

What are the Brandt-Daroff exercises and who uses them?

Answer

A home habituation exercise for positional vertigo: the client sits, lies quickly on one side with the head turned about 45 degrees up, waits until vertigo settles, returns upright, and then alternates sides.

#490 · Care Planning

How is the VOR x1 viewing gaze stabilization exercise performed?

Answer

The client fixes the eyes on a stationary target while turning the head horizontally or vertically at a speed that keeps the target clear. It is used for unilateral vestibular hypofunction and progressed with speed and background.

#491 · Care Planning

What is the principle of habituation exercises in vestibular rehabilitation?

Answer

Repeated, controlled exposure to a movement that provokes dizziness reduces the response over time. It suits motion-provoked dizziness and is graded to symptom tolerance.

#492 · Care Planning

Which nystagmus findings suggest a central rather than peripheral cause and should prompt referral?

Answer

Purely vertical or downbeating nystagmus, nystagmus without latency that does not fatigue, or new neurological signs. Peripheral BPPV nystagmus has a short latency, fatigues, and is torsional.

#493 · Care Planning

What are the main features of the Otago Exercise Programme for falls prevention?

Answer

An individualized, progressive program of leg strengthening and balance exercises delivered by a physiotherapist, usually at home, three times weekly, plus walking. It targets adults aged 65 and older, especially those over 80.

#494 · Care Planning

Why is tai chi recommended in falls prevention programs for older adults?

Answer

Slow controlled weight shifting and postural control practice challenge balance, and trials show it reduces falls. It is suitable as a group program in the community.

#495 · Care Planning

What dose of balance exercise is linked with greater fall reduction in older adults?

Answer

At least 3 hours per week of challenging balance exercise, ideally combined with strength training. Multicomponent programs that progress difficulty are more effective than walking alone.

#496 · Care Planning

Who benefits most from home hazard modification, and what changes are made?

Answer

Older adults at high risk, particularly those with prior falls, with an occupational therapist assessing. Typical changes are removing loose rugs, adding grab bars and lighting, and securing nonslip surfaces.

#497 · Care Planning

What does the Modified Ashworth Scale measure, and what does a grade of 3 indicate?

Answer

It grades muscle tone (resistance to passive movement) from 0 to 4. Grade 3 means considerable increase in tone, with passive movement difficult throughout the range of motion.

#498 · Care Planning

Why is a prolonged low-load stretch preferred over a quick, forceful stretch for a spastic muscle?

Answer

A quick stretch triggers the stretch reflex and increases tone. Slow, sustained, low-load stretching reduces reflex activation and gives a better chance of lengthening the muscle and maintaining joint range.

#499 · Care Planning

What is the purpose of serial casting in a client with a developing plantarflexion contracture?

Answer

Serial casting applies a sustained, low-load stretch. The cast is changed repeatedly, each time positioning the joint nearer the end of range, to gradually lengthen soft tissue and regain dorsiflexion.

#500 · Care Planning

Which skin and circulation checks are essential when applying a resting hand or ankle splint to a spastic limb?

Answer

Check for redness that does not fade within about 20 to 30 minutes, pressure over bony prominences, swelling, and circulation or sensation changes. Establish a wearing schedule and teach the client or caregiver to inspect skin.

#501 · Care Planning

A client with spasticity is started on botulinum toxin injections. What is the physiotherapist's role?

Answer

Collaborate with the prescribing physician: identify target muscles, then time stretching, strengthening, splinting and task practice during the weeks of effect. Reassess tone and function to judge outcome.

#502 · Care Planning

When is an intermittent pneumatic compression pump contraindicated for a swollen leg?

Answer

Avoid it with suspected or confirmed acute deep vein thrombosis, uncontrolled heart failure, active infection such as cellulitis, or severe arterial insufficiency, since it may dislodge a clot or overload circulation.

#503 · Care Planning

What communication approach helps when a client with moderate dementia becomes agitated during a session?

Answer

Stay calm, use a low tone, short simple one-step cues, and approach from the front at eye level. Do not argue or correct. Validate feelings and redirect to a familiar task.

#504 · Care Planning

What is one benefit of regular exercise for older adults with dementia?

Answer

Exercise can improve physical function, mobility and balance, and may improve mood and sleep. It reduces fall risk when it includes balance and strength components and is practised consistently.

#505 · Care Planning

How should a physiotherapist teach a new exercise to a client with dementia who has impaired short-term memory?

Answer

Rely on procedural memory: use demonstration, repetition and consistent routine, same time and place. Give single-step cues and avoid depending on written or verbal recall of instructions.

#506 · Care Planning

Why is caregiver involvement important in a home program for a client with dementia?

Answer

The client may not remember or start exercises independently. Trained caregivers can cue, supervise safety, keep routine consistent, and report changes. Include caregiver education and respite needs in the plan.

#507 · Care Planning

Which types of exercise can help clients with mild to moderate depression, and what should the plan include?

Answer

Aerobic exercise, and resistance training, both show benefit for mild to moderate depression. Plan regular sessions, graded progression and social contact, and coordinate with the client's mental health team.

#508 · Care Planning

What are the typical phases of cardiac rehabilitation?

Answer

Phase I is inpatient, early mobilization. Phase II is a supervised outpatient program with monitoring. Phase III is long-term maintenance with independent, community or home-based exercise and risk factor management.

#509 · Care Planning

How is the target heart rate calculated using the heart rate reserve (Karvonen) method?

Answer

Target HR = resting HR + (percentage intensity x (maximal HR minus resting HR)). It individualizes intensity using the reserve, and is common when no stress test with a measured maximum is available.

#510 · Care Planning

On the Borg 6 to 20 rating of perceived exertion scale, what range is usually targeted for moderate cardiac exercise?

Answer

About 11 to 14, corresponding to light to somewhat hard effort. RPE is useful when heart rate response is blunted, for example in clients taking beta-blockers.

#511 · Care Planning

Why is heart rate an unreliable intensity guide for a client taking beta-blockers?

Answer

Beta-blockers blunt the heart rate response to exercise, so target heart rates from age-based formulas overestimate the rate. Use RPE, the talk test, and a heart rate target set from the client's own stress test.

#512 · Care Planning

Name two signs during cardiac rehab exercise that should prompt stopping the session.

Answer

Chest pain or angina, severe shortness of breath, dizziness, a drop in systolic blood pressure with increasing workload, pallor, or new arrhythmia. Stop, rest and follow the facility emergency plan.

#513 · Care Planning

What energy conservation strategies help a client with heart failure manage daily activities?

Answer

Pace tasks with planned rest breaks, sit for activities such as grooming, prioritize essential tasks, spread effort across the day, and avoid tasks right after meals or in extreme heat or cold.

#514 · Care Planning

Why should a client with heart failure be taught to weigh themselves daily?

Answer

Rapid weight gain (commonly more than 2 kg in 2 to 3 days) signals fluid retention. The client should report it early so treatment can be adjusted before hospitalization.

#515 · Care Planning

Which findings suggest worsening fluid overload in a client with heart failure?

Answer

Increasing edema, orthopnea, paroxysmal nocturnal dyspnea, rapid weight gain, crackles on auscultation and increased fatigue or breathlessness at lower activity levels. Report them to the medical team.

#516 · Care Planning

What shoulder precaution is commonly given soon after pacemaker implantation?

Answer

Avoid raising the arm on the implant side above shoulder height and avoid heavy lifting, vigorous or forceful arm movements for the period advised by the surgeon, to protect lead position while it settles.

#517 · Care Planning

How should exercise intensity be set for a client with an implantable cardioverter-defibrillator (ICD)?

Answer

Keep peak heart rate at least 10 to 20 beats per minute below the programmed ICD shock threshold, and confirm the threshold with the cardiology team before progressing the exercise program.

#518 · Care Planning

What is the main aim of supervised walking therapy for intermittent claudication?

Answer

Walk to moderate claudication pain, rest until it settles, then repeat, for sessions of 30 to 60 minutes at least three times a week. It improves pain-free and maximal walking distance.

#519 · Care Planning

A client has a calf-pain presentation with warmth, swelling and a high Wells score. What is the first action?

Answer

Suspect deep vein thrombosis: do not massage or vigorously exercise the leg, avoid further mobilization until it is assessed, and refer urgently for diagnostic imaging and medical management.

#520 · Care Planning

What is the major contraindication to compression therapy for leg edema or venous ulcers?

Answer

Significant arterial disease, usually judged by an ankle-brachial index below about 0.5, with only reduced compression under specialist guidance between 0.5 and 0.8. Compression may reduce already limited arterial blood flow and cause tissue damage.

#521 · Care Planning

What essential foot care advice should be given to a client with diabetes and peripheral arterial disease?

Answer

Inspect feet daily, wear well-fitting protective footwear, never walk barefoot, keep skin moisturized but dry between toes, and report any wound, blister or colour change promptly.

#522 · Care Planning

What are the three phases of the active cycle of breathing technique (ACBT)?

Answer

Breathing control, thoracic expansion exercises (deep breaths with an optional hold), and the forced expiration technique (huffs). Cycles repeat, with breathing control between to prevent bronchospasm and fatigue.

#523 · Care Planning

Which positions are used for postural drainage of the right middle lobe?

Answer

Lie on the left side, rotated about a quarter turn backward, with the foot of the bed raised about 15 degrees (Trendelenburg). Modify or avoid head-down positions with reflux, cardiac instability or raised intracranial pressure.

#524 · Care Planning

Name two situations where head-down postural drainage positions should be avoided or modified.

Answer

Uncontrolled hypertension, heart failure with orthopnea, gastroesophageal reflux, raised intracranial pressure, recent eye or neurosurgery, or hemoptysis. Use modified or flat positions and monitor tolerance.

#525 · Care Planning

How does a positive expiratory pressure (PEP) device assist secretion clearance?

Answer

Breathing out against resistance creates back pressure that splints airways open, preventing early collapse and moving secretions toward larger airways, where huffing or coughing can clear them.

#526 · Care Planning

What should the physiotherapist do before using high-frequency chest wall oscillation on a client?

Answer

Screen for contraindications such as unstable head or neck injury, recent chest trauma or surgery, untreated pneumothorax, hemoptysis or rib fractures, then fit the vest correctly and monitor comfort and oxygen saturation.

#527 · Care Planning

Why is a huff preferred over a forceful cough for clients with airway collapse, such as COPD?

Answer

A huff keeps the glottis open and generates lower pleural pressure, so airways collapse less. It moves secretions with less fatigue and less bronchospasm than repeated forceful coughing.

#528 · Care Planning

How does pursed-lip breathing help clients with COPD?

Answer

Exhaling against pursed lips creates back pressure that keeps small airways open longer, reducing air trapping and respiratory rate. It lowers breathlessness during exertion and recovery.

#529 · Care Planning

What is the typical oxygen saturation goal during exercise for clients with COPD who need supplemental oxygen?

Answer

Titrate oxygen to keep SpO2 at or above about 88 to 92 percent during exertion, per the prescribing guideline, and stop and rest or adjust flow if saturation falls below the target.

#530 · Care Planning

What exercise intensity is recommended for aerobic training in pulmonary rehabilitation?

Answer

Start with moderate intensity, often 60 to 80 percent of peak work rate or a Borg dyspnea rating of 3 to 4 out of 10, and progress as tolerated. Include both lower and upper limb training.

#531 · Care Planning

Which energy conservation strategy applies when a client with COPD climbs stairs?

Answer

Pace the climb by coordinating breathing with steps, for example inhaling at rest and exhaling through pursed lips while climbing, and pause on landings. Plan rest instead of rushing.

#532 · Care Planning

Why does early mobilization after major abdominal surgery reduce pulmonary complications?

Answer

Upright posture and walking increase lung volumes, improve ventilation and secretion clearance, and prevent atelectasis and pneumonia. Pain control and supported coughing, such as splinting the incision, help.

#533 · Care Planning

How should a client use an incentive spirometer correctly?

Answer

Exhale fully, seal lips around the mouthpiece, inhale slowly and deeply to raise the indicator, hold the breath for about 3 to 5 seconds, then exhale. Repeat about 10 times each hour while awake.

#534 · Care Planning

What is atelectasis, and what is a key sign on examination?

Answer

Collapse of alveoli causing loss of lung volume. Expect diminished breath sounds, possible bronchial breathing, reduced chest expansion over the area and increased work of breathing or low oxygen saturation.

#535 · Care Planning

A mechanically ventilated ICU client is being considered for mobilization. What is one safety criterion to check?

Answer

Check hemodynamic and respiratory stability: for example, stable blood pressure and heart rate, adequate oxygenation, no uncontrolled arrhythmia, no raised intracranial pressure, and an alert enough client who can follow commands.

#536 · Care Planning

Which physiological change from prolonged bed rest in the ICU prompts early progressive mobility?

Answer

Rapid loss of muscle mass and strength (ICU-acquired weakness), along with deconditioning and reduced lung volumes. Early mobility improves function and may shorten delirium and hospital stay.

#537 · Care Planning

What is exercise-induced bronchoconstriction (EIB)?

Answer

Transient narrowing of the airways during or soon after vigorous exercise, causing cough, wheeze, chest tightness or breathlessness. It is triggered by dry, cool air and is relieved or prevented by bronchodilators.

#538 · Care Planning

When should a client with asthma use a short-acting bronchodilator before exercise?

Answer

Typically 10 to 15 minutes before exercise, if prescribed. A gradual warm-up can also reduce symptoms. Keep the reliever inhaler available throughout the session.

#539 · Care Planning

What should the physiotherapist do if a client with asthma develops wheeze and tightness during exercise?

Answer

Stop activity, have the client sit upright and use the reliever inhaler as prescribed, and monitor. If symptoms do not settle or worsen, follow the emergency plan and seek urgent medical help.

#540 · Care Planning

Why is airway clearance a daily therapy in cystic fibrosis?

Answer

Thick, sticky mucus causes airway obstruction, chronic infection and inflammation. Daily clearance with techniques such as ACBT, PEP, oscillating devices or vest therapy slows lung damage.

#541 · Care Planning

What should the physiotherapist remind clients with cystic fibrosis about hydration and salt during exercise?

Answer

They lose extra sodium in sweat, so they need adequate fluids and added salt to avoid dehydration and heat illness, especially in hot weather or long sessions.

#542 · Care Planning

What blood pressure level defines hypertension in the current Hypertension Canada guideline?

Answer

The 2025 Hypertension Canada guideline defines hypertension as BP of 130/80 mmHg or higher measured with a validated device under optimal conditions, confirmed with out-of-office measurement. The treatment target is systolic below 130 mmHg if tolerated. Drug therapy is recommended at 140/90 or higher, or at systolic 130-139 with high cardiovascular risk.

#543 · Care Planning

Above what resting blood pressure should a physiotherapist generally defer vigorous exercise testing or training?

Answer

A resting systolic above about 180 mmHg or diastolic above about 110 mmHg is a common reason to delay or cancel exercise. Refer for medical review and follow the facility policy for the client.

#544 · Care Planning

What breathing precaution applies when a client with hypertension performs resistance exercises?

Answer

Avoid the Valsalva maneuver. Breathing out during the effort and in during the return lowers the spike in blood pressure and reduces intrathoracic pressure surges.

#545 · Care Planning

What exercise recommendation do Canadian guidelines give to adults with hypertension for aerobic training?

Answer

Accumulate 150 minutes per week of moderate intensity aerobic activity, in sessions of at least 10 minutes. Regular exercise lowers blood pressure and cardiovascular risk.

#546 · Care Planning

Which blood pressure response during exercise should prompt stopping?

Answer

A fall in systolic pressure of more than 10 mmHg with increasing workload, or a systolic above about 250 mmHg or diastolic above 115 mmHg. Stop, rest and monitor, and report to the physician.

#547 · Care Planning

What is the usual therapeutic INR target for most warfarin indications, and why does it matter before exercise planning?

Answer

The usual target is 2.0 to 3.0. A supratherapeutic INR raises bleeding risk, so confirm the recent value with the care team before resistance or balance training and hold high-risk activity if it is elevated.

#548 · Care Planning

A client on a DOAC or warfarin has fall risk. Name three safety planning steps for physiotherapy sessions.

Answer

Clear the environment, use gait aids and supervision during balance work, avoid high-impact or contact activity, and teach the client to report any fall, head strike, or unusual bruising promptly, since bleeding can be delayed.

#549 · Care Planning

Which bleeding signs should a client on anticoagulants be taught to report after exercise?

Answer

Large or spreading bruises, blood in urine, black or bloody stools, nosebleeds that will not stop, a new severe headache, or a joint that becomes hot and swollen. Any head injury needs urgent medical assessment.

#550 · Care Planning

In hemophilia, what is the first-line plan for a suspected acute joint bleed (hemarthrosis) during therapy?

Answer

Stop activity, apply rest, ice, compression and elevation, and contact the hemophilia team for factor replacement as soon as possible. Do not stretch, massage, or resist the joint. Restart gently only after medical clearance.

#551 · Care Planning

Which symptoms during exercise in a client with anemia mean you should stop and reassess?

Answer

Disproportionate shortness of breath, dizziness, chest pain, palpitations, or a heart rate far above expected for the effort. Low oxygen-carrying capacity limits tolerance, so use RPE and the talk test and progress slowly.

#552 · Care Planning

What is the usual minimum turning interval for a bed-bound client at risk of pressure injury on a standard mattress?

Answer

At least every 2 hours, individualized to skin condition, comfort, and the support surface used. Document the schedule and check bony prominences each turn for non-blanchable redness, which signals early injury.

#553 · Care Planning

Why is 30-degree side-lying preferred over 90-degree side-lying for pressure injury prevention?

Answer

The 30-degree tilt keeps weight off the greater trochanter and sacrum, spreading pressure over the buttock and thigh muscle. A full 90-degree side-lying position loads the trochanter directly and raises injury risk.

#554 · Care Planning

Why should the head of the bed be kept at or below 30 degrees when clinically allowed?

Answer

Higher angles cause the body to slide down, producing shear and high sacral pressure. Exceptions include meals, respiratory needs, or reflux precautions, in which case keep the time upright as short as practical.

#555 · Care Planning

How often should a wheelchair user perform a pressure relief or weight shift to protect the ischial region?

Answer

Commonly every 15 to 30 minutes, held for about 1 to 2 minutes, by forward lean, side lean, or push-up. Recommended timing varies by guideline and individual risk, so tailor it and teach it as a habit.

#556 · Care Planning

What ankle-brachial pressure index (ABPI) finding contraindicates compression therapy for a venous leg ulcer?

Answer

An ABPI below 0.5 suggests severe arterial disease, so full compression is contraindicated. Between 0.5 and 0.8, use reduced compression under specialist guidance. An ABPI above 0.8 generally allows full compression.

#557 · Care Planning

How should an anterior neck burn be positioned to prevent contracture?

Answer

Keep the neck in extension, usually with no pillow, and a towel roll under the shoulders if tolerated. Flexion, the position of comfort, drives scar contracture, so encourage extension and active range of motion.

#558 · Care Planning

What shoulder position helps prevent axillary contracture after an axilla burn?

Answer

Abduction of about 90 to 100 degrees with slight external rotation, using a splint, airplane position, or overhead support. The adducted, internally rotated posture is comfortable but leads to a tight scarred axilla.

#559 · Care Planning

Describe the safe (intrinsic-plus) splint position for a burned hand.

Answer

Wrist in 20 to 30 degrees of extension, MCP joints flexed 70 to 90 degrees, IP joints straight, and the thumb abducted. This keeps collateral ligaments lengthened and prevents claw deformity.

#560 · Care Planning

What is the usual wear schedule and purpose of pressure garments in hypertrophic burn scar management?

Answer

Worn about 23 hours daily, commonly for 12 to 18 months until scar maturation. Garments apply sustained pressure, often cited near 24 mmHg, to flatten and soften scar. Reassess fit as the tissue changes.

#561 · Care Planning

After a split-thickness skin graft, when does range of motion over the graft site usually resume?

Answer

Typically after about 3 to 5 days of immobilization, once the surgeon confirms graft take. Early movement can shear the graft, so follow the surgical team's written protocol and then progress gradually.

#562 · Care Planning

What blood glucose threshold defines hypoglycemia in Diabetes Canada guidance, and what is the first response?

Answer

Below 4.0 mmol/L. If the client can swallow, give 15 g of fast-acting carbohydrate, recheck in 15 minutes, and repeat if still below 4.0. Stop exercise until glucose recovers and symptoms resolve.

#563 · Care Planning

A person with type 1 diabetes has pre-exercise ketones above 1.5 mmol/L. What should you advise?

Answer

Avoid exercise and contact the diabetes care team, since high ketones with insufficient insulin risk ketoacidosis. Vigorous activity can worsen hyperglycemia and ketosis when insulin is inadequate.

#564 · Care Planning

Which exercise targets does Diabetes Canada recommend weekly for adults with type 2 diabetes?

Answer

At least 150 minutes of moderate to vigorous aerobic activity per week spread over 3 days or more, plus resistance training 2 to 3 times weekly on non-consecutive days. Combined training gives the best glycemic benefit.

#565 · Care Planning

What is the foot protection plan for a client with diabetes and peripheral neuropathy?

Answer

Inspect feet daily, wear well-fitting protective footwear and clean socks, never exercise barefoot, and check the 10 g monofilament for loss of protective sensation. Report blisters or redness promptly.

#566 · Care Planning

Why can hypoglycemia occur hours after exercise in someone using insulin, and how do you plan for it?

Answer

Muscles keep replenishing glycogen and insulin sensitivity stays high, so glucose can drop for up to 24 hours. Monitor later, avoid late-evening intense sessions, and have carbohydrate available.

#567 · Care Planning

How do you monitor intensity when diabetic autonomic neuropathy blunts the heart rate response?

Answer

Use the RPE scale or the talk test rather than target heart rate. Autonomic neuropathy also causes orthostatic hypotension, so change positions slowly and screen for silent cardiac ischemia before vigorous exercise.

#568 · Care Planning

What general exercise targets does evidence-based oncology guidance suggest during cancer treatment when the client is stable?

Answer

Roughly 150 minutes of moderate aerobic activity weekly plus resistance training twice weekly, adjusted for fatigue, blood counts, and treatment cycle. Start low, progress slowly, and check with the oncology team.

#569 · Care Planning

Name the four core components of complete decongestive therapy (CDT) for lymphedema.

Answer

Manual lymphatic drainage, compression bandaging or garments, remedial exercise, and skin care with education. The intensive phase reduces volume, and the maintenance phase uses garments and self-management to keep it reduced.

#570 · Care Planning

Which conditions are contraindications to decongestive therapy or compression for lymphedema?

Answer

Acute infection such as cellulitis, decompensated heart failure, acute deep vein thrombosis, and severe arterial insufficiency. Any red, hot, painful limb with fever needs urgent medical assessment, not therapy.

#571 · Care Planning

What does a Mirels score of 9 or more indicate for a long-bone metastasis?

Answer

A high fracture risk where prophylactic fixation is generally recommended. Until the surgeon clears it, avoid resisted loading, torsion, and impact on that bone, and provide protected weight bearing with a gait aid.

#572 · Care Planning

What weekly activity target does the Canadian guideline give for healthy pregnant women without contraindications?

Answer

At least 150 minutes of moderate-intensity activity per week, spread over at least 3 days and ideally daily, plus gentle stretching and pelvic floor muscle training. Add resistance exercise and keep activities safe from falls.

#573 · Care Planning

List warning signs that mean a pregnant client should stop exercising and seek medical advice.

Answer

Vaginal bleeding, regular painful contractions, leakage of fluid, persistent dizziness or faintness, chest pain, severe breathlessness before exertion, and calf pain or swelling. Do not resume until a clinician clears her.

#574 · Care Planning

What measurement is commonly used to identify diastasis recti, and what exercise feature should be avoided?

Answer

An inter-rectus distance greater than about 2 cm is commonly used. Avoid movements that cause visible abdominal doming or coning along the linea alba, and teach deep abdominal engagement with good breathing.

#575 · Care Planning

Where should a pelvic girdle support belt be positioned for pregnancy-related pelvic girdle pain?

Answer

Low across the pelvis, below the anterior superior iliac spines and over the greater trochanters, snug but comfortable. It improves pelvic stability and pain with walking, and works best alongside exercise and education.

#576 · Care Planning

What symptoms suggest a postpartum runner is not yet ready to return to impact activity?

Answer

Urinary leakage, a sense of pelvic heaviness or bulging, pelvic or back pain, or doming of the abdomen. Consensus guidance suggests no running before about 12 weeks, with graded loading and pelvic floor assessment first.

#577 · Care Planning

Using BMI, how is obesity class II defined in adults?

Answer

A BMI of 35.0 to 39.9 kg/m2. Class I is 30.0 to 34.9, and class III is 40 or higher. BMI is a screening tool, so also consider waist circumference, function, and health risks.

#578 · Care Planning

How does weight loss relate to knee joint loading in people with obesity and knee osteoarthritis?

Answer

Each 1 kg lost reduces knee load per step by roughly 4 kg during walking. This supports pairing exercise and weight management in knee osteoarthritis plans, with low-impact options like cycling and water exercise.

#579 · Care Planning

What equipment check should you complete before treating a client with severe obesity?

Answer

Confirm the weight capacity of plinths, chairs, commodes, walkers, and lifts, and arrange bariatric equipment and enough staff in advance. Using equipment beyond its rating risks injury to the client and clinician.

#580 · Care Planning

How should a physiotherapist address weight with a client in a respectful, non-stigmatizing way?

Answer

Use person-first language, ask permission before discussing weight, and avoid attributing unrelated complaints to body size. Set goals around function and health behaviours, and ensure equipment and spaces fit the client.

#581 · Care Planning

Why is early ambulation emphasized after bariatric surgery?

Answer

It lowers the risk of venous thromboembolism and pulmonary complications, which are higher in this population. Begin walking soon after surgery as cleared, with surgeon-specified limits on lifting and abdominal strain.

#582 · Care Planning

What commonly cited physiological criteria support starting early mobilization in a ventilated ICU patient?

Answer

Hemodynamic stability without escalating vasopressors, FiO2 of 0.6 or less, PEEP of 10 cm H2O or less, and an adequate level of arousal. Criteria vary by unit, so follow local protocol and team agreement.

#583 · Care Planning

What should you check about lines and tubes before mobilizing an ICU patient?

Answer

Confirm each line, drain, catheter, and endotracheal tube is secure and has slack, and ask the nurse about restrictions, such as hip flexion limits with a femoral catheter. Assign someone to manage the airway and lines.

#584 · Care Planning

Can an intubated, mechanically ventilated patient be mobilized, and who should be present?

Answer

Yes, if stable, because early mobility improves outcomes. Coordinate with the nurse and respiratory therapist, who manages the airway and ventilator, plus enough staff to handle lines safely while the patient moves.

#585 · Care Planning

Which tool screens for delirium in ICU patients, and name a non-drug prevention strategy.

Answer

The CAM-ICU. Prevention includes early mobilization, orientation cues, glasses and hearing aids, daytime light, sleep protection, and minimizing restraints and sedation, all coordinated with the team.

#586 · Care Planning

How is ICU-acquired weakness diagnosed using the Medical Research Council sum score?

Answer

An MRC sum score below 48 out of 60, from six muscle groups per side tested on a 0 to 5 scale, in an awake, cooperative patient. It indicates clinically significant weakness and a need for tailored rehabilitation.

#587 · Care Planning

What are the primary goals of physiotherapy in palliative care?

Answer

Comfort, maintaining the best possible function, safety, and dignity, guided by the client's own goals. Interventions are adjusted to prognosis and energy and may include positioning, mobility, breathlessness strategies, and caregiver teaching.

#588 · Care Planning

Which positions help relieve breathlessness in a palliative client?

Answer

Forward-lean sitting with arms supported on a table, or high side-lying with pillows. These positions improve diaphragm mechanics and accessory muscle use, reducing the sense of breathlessness and anxiety.

#589 · Care Planning

Which simple non-drug tools can reduce breathlessness in advanced illness?

Answer

A handheld fan directed at the face, pursed-lip breathing, paced breathing, and a calm environment with open air flow. The cool air on facial trigeminal receptors can lessen the perception of breathlessness.

#590 · Care Planning

What are the four Ps of energy conservation taught to palliative clients and families?

Answer

Pacing, planning, prioritizing, and positioning. Spread tasks through the day, plan rest breaks, choose what matters most, and use positions that reduce effort. These preserve strength for meaningful activities.

#591 · Care Planning

How can a physiotherapist support family caregivers of a client at end of life?

Answer

Teach safe transfers, positioning, skin protection, and equipment use, offer clear written instructions, and encourage questions. Respect cultural and personal wishes, and coordinate with the palliative team on timing of care.

#592 · Care Planning

What are the CSEP 24-hour movement guideline targets for adults aged 18 to 64?

Answer

At least 150 minutes of moderate to vigorous activity weekly, muscle strengthening twice weekly, several hours of light activity, 7 to 9 hours of sleep, and sedentary time limited to 8 hours or less.

#593 · Care Planning

What are the CSEP 24-hour guidelines for children and youth aged 5 to 17?

Answer

An average of at least 60 minutes of moderate to vigorous activity daily, several hours of light activity, no more than 2 hours of recreational screen time, and 9 to 11 hours of sleep for ages 5 to 13.

#594 · Care Planning

What do the CSEP guidelines add specifically for adults aged 65 and older?

Answer

The same 150 minutes of moderate to vigorous activity weekly and muscle strengthening twice weekly, plus physical activities that challenge balance, several hours of light activity, and 7 to 8 hours of sleep.

#595 · Care Planning

Which exercise program has strong evidence for reducing falls in community-dwelling older adults?

Answer

The Otago Exercise Programme: individualized strength and balance exercises plus walking, delivered at home with physiotherapist support. Aim for about three sessions weekly, progressing difficulty as ability improves.

#596 · Care Planning

What is the PAR-Q+ used for before starting an exercise program?

Answer

It is a pre-participation screening questionnaire identifying who needs medical clearance before becoming more active. A yes response leads to follow-up questions and possible referral, while all no responses allow gradual progression.

#597 · Client Safety and Client-Centred Care

What three elements must be present for consent to assessment or treatment to be valid?

Answer

Consent must be voluntary (free of pressure), informed (nature, benefits, risks, alternatives, and consequences of declining were explained), and given by a client with capacity to decide.

#598 · Client Safety and Client-Centred Care

How is capacity to consent judged for a specific treatment decision?

Answer

The client must understand the relevant information and appreciate the reasonably foreseeable consequences of deciding or not deciding. Capacity is decision-specific and presumed present unless there is reason to doubt it.

#599 · Client Safety and Client-Centred Care

What is the difference between implied and expressed consent in physiotherapy practice?

Answer

Implied consent is inferred from actions, such as a client positioning an arm for examination. Expressed consent is stated verbally or in writing. Use expressed consent for new, higher-risk, or intimate procedures.

#600 · Client Safety and Client-Centred Care

A client says midway through a treatment that they want to stop. What is the physiotherapist's response?

Answer

Stop immediately. Consent can be withdrawn at any time without penalty. Make the client safe, explore their concerns, explain options, and document the withdrawal and the discussion.

#601 · Client Safety and Client-Centred Care

A client lacks capacity to consent to treatment. Who gives consent, and what is documented?

Answer

The substitute decision-maker, as set out by provincial or territorial legislation, gives consent. Document the capacity concern, who decided, the information given, and the consent obtained.

#602 · Client Safety and Client-Centred Care

Which two client identifiers should be checked before treatment, and why not use the room number?

Answer

Use two person-specific identifiers, such as full name plus date of birth or health card number, ideally stated by the client. Room or bed numbers can change and cause identification errors.

#603 · Client Safety and Client-Centred Care

What must be confirmed before treating a limb or one side of the body?

Answer

Confirm the correct side with the client and against the referral or chart before starting. Laterality errors cause wrong-site treatment, so verify again if the client changes position.

#604 · Client Safety and Client-Centred Care

What is a time-out in a clinical setting?

Answer

A deliberate pause before a procedure where the team confirms the right client, the right procedure, and the right site. It catches errors before they reach the client.

#605 · Client Safety and Client-Centred Care

What should you verify before applying an electrical modality to a client?

Answer

Verify the client, the order or plan of care, the treatment area and side, the parameters and dosage, and the absence of contraindications or precautions for that modality.

#606 · Client Safety and Client-Centred Care

Two clients on your schedule have nearly identical names. What safety step reduces the risk of a mix-up?

Answer

Flag the charts as a name alert, keep them separate, and use an extra identifier such as date of birth every time. Never rely on the name alone.

#607 · Client Safety and Client-Centred Care

What are the four moments for hand hygiene in Canadian health care guidance?

Answer

Before initial contact with the client or their environment, before an aseptic procedure, after risk of body fluid exposure, and after contact with the client or their environment.

#608 · Client Safety and Client-Centred Care

When is alcohol-based hand rub acceptable, and when must you use soap and water?

Answer

Use alcohol-based hand rub (60 to 90 percent alcohol) when hands are not visibly soiled. Wash with soap and running water when hands are visibly soiled or after contact with C. difficile.

#609 · Client Safety and Client-Centred Care

What is the usual order for putting on and taking off personal protective equipment?

Answer

Put on: gown, mask, eye protection, gloves. Take off: gloves first, then eye protection and gown, then mask, according to local policy. Perform hand hygiene after removal.

#610 · Client Safety and Client-Centred Care

What are routine practices, and who do they apply to?

Answer

Routine practices are infection control measures applied to all clients at all times, guided by a point-of-care risk assessment. They include hand hygiene, PPE as needed, and environmental cleaning.

#611 · Client Safety and Client-Centred Care

What PPE and room setup does airborne precaution require compared with droplet precaution?

Answer

Airborne precautions need a fit-tested N95 respirator and a negative pressure room. Droplet precautions need a surgical mask within about two metres of the client, usually in a single room.

#612 · Client Safety and Client-Centred Care

What do the Spaulding categories require for noncritical, semicritical, and critical items?

Answer

Noncritical items touch intact skin: cleaning and low-level disinfection. Semicritical items touch mucous membranes or non-intact skin: high-level disinfection. Critical items enter sterile tissue: sterilization.

#613 · Client Safety and Client-Centred Care

How should treatment plinths be managed between clients?

Answer

Plinths contact intact skin, so they are noncritical. Clean and disinfect with a low-level disinfectant between clients, and use a clean linen or disposable cover for each person.

#614 · Client Safety and Client-Centred Care

What safety checks apply to electrotherapy equipment before use?

Answer

Inspect cords, leads, plugs, and electrodes for damage, confirm the device passed its scheduled biomedical inspection, and remove any faulty unit from service and label it.

#615 · Client Safety and Client-Centred Care

How should a therapeutic ultrasound transducer be handled between clients?

Answer

Wipe off the gel, then clean and disinfect the transducer head with a facility-approved low-level disinfectant after every client, because it contacts skin and gel.

#616 · Client Safety and Client-Centred Care

How should soiled linen and towels be handled?

Answer

Handle with minimal agitation, do not shake them, keep them away from your clothing, and place them in a designated bag or bin for laundering. Wash hands afterward.

#617 · Client Safety and Client-Centred Care

How is a gait belt applied for a transfer?

Answer

Place it snugly around the client's waist over clothing, check it is secure and not over tubes or incisions, and hold the belt to guide the transfer, never to lift the client.

#618 · Client Safety and Client-Centred Care

When is a mechanical lift appropriate for a transfer?

Answer

When the client cannot bear weight or help safely, or when manual handling would risk injury to client or staff. Check the sling, the lift's weight rating, and facility staffing policy first.

#619 · Client Safety and Client-Centred Care

What are the key body mechanics principles for a physiotherapist during handling tasks?

Answer

Keep a wide base of support, hold the load close to your body, bend at the hips and knees, keep the spine neutral, and avoid twisting by moving your feet.

#620 · Client Safety and Client-Centred Care

What equipment checks are needed when treating a bariatric client?

Answer

Confirm that plinths, chairs, commodes, and lifts are rated for the client's weight. Plan for extra staff, wider spaces, and suitable bariatric equipment before starting.

#621 · Client Safety and Client-Centred Care

What steps come before starting any client transfer?

Answer

Assess the client's ability and cognition, explain the plan, clear the path, lock brakes on wheelchairs and beds, set heights, and arrange enough help. Then give clear, timed instructions.

#622 · Client Safety and Client-Centred Care

What is a near miss, and why should it be reported?

Answer

A near miss is an event that could have harmed a client but did not reach them or caused no harm. Reporting reveals system weaknesses so they can be fixed before harm occurs.

#623 · Client Safety and Client-Centred Care

What defines an adverse event in client safety terms?

Answer

An adverse event is an unintended event during care that results in harm to the client, such as a fall during gait training, rather than harm caused by the underlying condition.

#624 · Client Safety and Client-Centred Care

What should disclosure of an adverse event to a client include?

Answer

Disclose promptly and honestly: what happened, the known consequences, an expression of regret or apology, and the steps being taken to manage harm and prevent recurrence. Offer ongoing support.

#625 · Client Safety and Client-Centred Care

How should an adverse event be documented in the client record?

Answer

Record objective facts: what happened, the client's condition, actions taken, who was notified, and what was disclosed. Avoid blame or speculation, and file the incident report according to facility policy.

#626 · Client Safety and Client-Centred Care

What does a just culture or learning culture mean for incident reporting?

Answer

Errors are treated mainly as system problems to learn from, not occasions for blame. Staff are encouraged to report events and near misses, while reckless behaviour is still held to account.

#627 · Client Safety and Client-Centred Care

On the Borg RPE scale from 6 to 20, which range is moderate intensity?

Answer

Ratings of about 12 to 14, described as somewhat hard, correspond to moderate intensity. Because it reflects effort felt, RPE is useful when heart rate is unreliable.

#628 · Client Safety and Client-Centred Care

How does the talk test indicate exercise intensity?

Answer

At moderate intensity a client can talk but not sing. If they can sing it is light intensity. If they can only say a few words, intensity is vigorous.

#629 · Client Safety and Client-Centred Care

Which symptoms during exercise mean you should stop the session and reassess?

Answer

Chest pain, severe shortness of breath, dizziness or light-headedness, pallor, confusion, unusual fatigue, or a drop in systolic blood pressure despite increasing workload. Monitor and escalate as needed.

#630 · Client Safety and Client-Centred Care

What blood pressure change defines orthostatic hypotension?

Answer

A fall in systolic pressure of at least 20 mmHg or in diastolic pressure of at least 10 mmHg within three minutes of standing from lying or sitting, often with dizziness.

#631 · Client Safety and Client-Centred Care

What is the normal resting respiratory rate for an adult?

Answer

About 12 to 20 breaths per minute. A resting rate above this range suggests respiratory distress, pain, fever, anxiety, or another problem needing assessment.

#632 · Client Safety and Client-Centred Care

Which electrotherapy modalities are contraindicated for clients with a cardiac pacemaker?

Answer

Shortwave diathermy is contraindicated because it can interfere with the device. Avoid electrical stimulation such as TENS near the chest or device, unless cleared by the cardiologist or manufacturer.

#633 · Client Safety and Client-Centred Care

Which modalities should be avoided over the abdomen or low back of a pregnant client?

Answer

Avoid shortwave diathermy and therapeutic ultrasound over the uterus and adjacent area, and avoid heat to the abdomen, because of fetal risk. Choose safer alternatives.

#634 · Client Safety and Client-Centred Care

Why are heat, ultrasound, and electrical stimulation avoided over a known malignancy?

Answer

They may increase blood flow and cell activity, potentially promoting tumour growth or spread. Treat as contraindicated over the area and coordinate with the oncology team.

#635 · Client Safety and Client-Centred Care

Why is impaired sensation a precaution for thermal modalities?

Answer

The client cannot reliably report excessive heat or cold, so burns or frostbite can occur. Test sensation first, use low intensity, add insulation, and check the skin often.

#636 · Client Safety and Client-Centred Care

Which modality is contraindicated with metal implants in the treatment field, and which is generally acceptable?

Answer

Shortwave diathermy is contraindicated because metal heats unpredictably. Therapeutic ultrasound over metal implants is generally considered acceptable, though avoid over cemented components if your guideline says so.

#637 · Client Safety and Client-Centred Care

How do beta blockers affect exercise monitoring?

Answer

They lower resting and exercise heart rate and blunt the heart rate response, so heart rate targets are unreliable. Use RPE and the talk test, and monitor blood pressure.

#638 · Client Safety and Client-Centred Care

What precautions apply to clients taking anticoagulants such as warfarin or a DOAC?

Answer

They have increased bleeding and bruising risk. Avoid vigorous soft tissue techniques and deep friction, prevent falls, watch for unexplained bruising, and report suspected bleeding promptly.

#639 · Client Safety and Client-Centred Care

What is the main exercise risk for a client taking insulin, and how do you prepare?

Answer

Hypoglycemia. Check glucose before and, if needed, during activity, know the symptoms, and keep a fast-acting carbohydrate available. Know the timing of the last insulin dose and meal.

#640 · Client Safety and Client-Centred Care

What safety concerns arise when a client is taking opioid analgesics?

Answer

Drowsiness, dizziness, slowed reaction, and reduced breathing can raise fall risk and impair judgment. Pain may be masked, so treatment loads must be set cautiously.

#641 · Client Safety and Client-Centred Care

Why do long-term oral corticosteroids matter for physiotherapy treatment?

Answer

They can cause osteoporosis, tendon weakening, muscle wasting, and fragile skin. Use caution with high-load exercise, joint mobilization force, and manual techniques.

#642 · Client Safety and Client-Centred Care

What does the FAST stroke warning stand for, and what is the action?

Answer

Face drooping, Arm weakness, Speech difficulty, Time to call 911. Note the time symptoms started and do not give food, drink, or further treatment.

#643 · Client Safety and Client-Centred Care

What are the typical signs of a myocardial infarction, and what is your first action?

Answer

Chest pressure or pain, possibly radiating to the arm or jaw, with sweating, nausea, or shortness of breath. Stop activity, seat the client, call 911, and stay with them.

#644 · Client Safety and Client-Centred Care

What presentation should raise suspicion of pulmonary embolism, and what is the response?

Answer

Sudden shortness of breath, pleuritic chest pain, rapid heart rate, or cough, especially with leg swelling or recent surgery or immobility. Stop activity and arrange emergency care at once.

#645 · Client Safety and Client-Centred Care

What red flags suggest cauda equina syndrome?

Answer

Saddle numbness, new bladder retention or bowel incontinence, sexual dysfunction, and bilateral or progressive leg weakness. This is a surgical emergency requiring same-day emergency referral.

#646 · Client Safety and Client-Centred Care

How do you respond to suspected anaphylaxis during a session?

Answer

Call 911, administer epinephrine by auto-injector into the outer mid-thigh if available, lay the client flat with legs raised unless breathing is difficult, and monitor airway and breathing.

#647 · Client Safety and Client-Centred Care

You find an adult client unresponsive in the gym with no normal breathing. What are your first actions?

Answer

Check the scene is safe, call for help and activate emergency services (911), send someone for the AED, then start chest compressions right away. Early compressions and early defibrillation give the best chance of survival.

#648 · Client Safety and Client-Centred Care

What are the recommended compression depth and rate for adult CPR?

Answer

Compress at least 5 cm but no more than 6 cm, at a rate of 100 to 120 per minute. Allow full chest recoil between compressions and minimize interruptions.

#649 · Client Safety and Client-Centred Care

After turning on an AED and attaching the pads, what must everyone do before a shock is delivered?

Answer

Stand clear and make sure no one is touching the client while the AED analyzes the rhythm and delivers the shock. Then resume compressions immediately after the shock, following the device prompts.

#650 · Client Safety and Client-Centred Care

A client has a generalized tonic-clonic seizure during treatment. What is the correct first aid?

Answer

Protect the head, clear nearby hazards, and time the seizure. Do not restrain the client or put anything in the mouth. Roll the client onto their side afterward and call 911 if it lasts over 5 minutes.

#651 · Client Safety and Client-Centred Care

A conscious adult has a severe airway obstruction and cannot cough, speak, or breathe. What do you do?

Answer

Give five back blows alternating with five abdominal thrusts until the object is expelled or the client becomes unresponsive. If they become unresponsive, call 911 and start CPR, checking the mouth for a visible object.

#652 · Client Safety and Client-Centred Care

What does cultural safety mean, and who determines whether care was culturally safe?

Answer

Cultural safety is care that addresses power imbalances and leaves clients feeling respected and free from discrimination. The client, not the physiotherapist, decides whether an experience was culturally safe.

#653 · Client Safety and Client-Centred Care

How does cultural humility differ from cultural competence?

Answer

Cultural humility is an ongoing process of self-reflection, learning from clients, and correcting power imbalances. Cultural competence implies a fixed endpoint of knowledge about a group, which can lead to stereotyping.

#654 · Client Safety and Client-Centred Care

Which Truth and Reconciliation Commission Calls to Action section addresses health, and what does it call for?

Answer

The Health section (Calls 18 to 24) calls for closing health gaps between Indigenous and non-Indigenous people, recognizing traditional healing, and cultural competency training for health professionals, including anti-racism and the history of residential schools.

#655 · Client Safety and Client-Centred Care

What is Indigenous-specific racism in health care, and why is it a patient safety issue?

Answer

It is stereotyping and discrimination directed at Indigenous clients, which can lead to delayed or denied care. The BC In Plain Sight report documented it, and it harms outcomes and trust.

#656 · Client Safety and Client-Centred Care

A client from a cultural background asks for a same-gender physiotherapist and a family member present. What is the best approach?

Answer

Respect the request where reasonably possible, explore the client's preferences without assumptions, and explain options. Accommodating cultural preferences supports trust, provided safety and professional standards are maintained.

#657 · Client Safety and Client-Centred Care

What does the equity, diversity and inclusion approach ask a physiotherapist to do about barriers to care?

Answer

Identify and reduce barriers such as language, income, geography, and discrimination, and adapt care to individual needs. Treating everyone identically does not always produce equitable outcomes.

#658 · Client Safety and Client-Centred Care

What are the core principles of trauma-informed care?

Answer

Safety, trustworthiness, choice, collaboration, and empowerment, with sensitivity to cultural and gender issues. Care assumes any client may have a trauma history, whether or not disclosed.

#659 · Client Safety and Client-Centred Care

A client suddenly freezes and becomes distant when you approach to palpate. What should you do first?

Answer

Stop the touch, pause, and check in verbally. Reorient the client to the present, offer choice about continuing, and adapt. A trauma response is not non-compliance and should not be pushed through.

#660 · Client Safety and Client-Centred Care

How can a physiotherapist give a client choice and control during an assessment?

Answer

Explain each step beforehand, ask permission before touch, offer options such as position, pace, and stopping, and agree on a stop signal. Predictability and control reduce the risk of retraumatization.

#661 · Client Safety and Client-Centred Care

Why is consent for touch an ongoing process rather than a single signature?

Answer

Consent can be withdrawn at any time, and a client who agreed earlier may feel unsafe later. Check in repeatedly, especially before sensitive areas, and respect any refusal or hesitation immediately.

#662 · Client Safety and Client-Centred Care

Which practice commonly risks retraumatizing a client during physiotherapy?

Answer

Unannounced touch, closed doors with no explanation, pressuring a client to remove clothing, or ignoring signs of distress. Avoid surprises, explain procedures, and let the client set the pace.

#663 · Client Safety and Client-Centred Care

A wheelchair user must access your clinic. What minimum environmental features support accessibility?

Answer

A step-free entrance or ramp, doors wide enough and with automatic or easy openers, accessible washroom, adequate turning space, and height-adjustable plinths. Requirements come from provincial accessibility legislation and building codes.

#664 · Client Safety and Client-Centred Care

What is a physiotherapist's duty regarding accommodating a client with a disability?

Answer

Under human rights legislation there is a duty to accommodate to the point of undue hardship. Providers should modify the environment, communication, or process rather than refuse or limit service.

#665 · Client Safety and Client-Centred Care

A client who is Deaf attends for assessment. What communication supports should you consider?

Answer

Book a qualified sign language interpreter, face the client when speaking, use written notes or text, and confirm understanding. Do not rely on family members to interpret clinical information.

#666 · Client Safety and Client-Centred Care

How should you adapt written home exercise instructions for a client with low vision?

Answer

Provide large print with high contrast, audio or video versions, or tactile cues, and review the exercises verbally. Check that the client can actually read and follow the format given.

#667 · Client Safety and Client-Centred Care

What is assistive technology in rehabilitation, and who should be involved in selecting it?

Answer

Devices that improve function, such as walkers, communication boards, or voice-activated controls. Select them with the client, matching their goals, environment, and abilities, and review fit and use after issuing.

#668 · Client Safety and Client-Centred Care

A competent adult refuses your recommended treatment. What is your obligation?

Answer

Respect the refusal, since competent adults have the right to decline care. Make sure the decision is informed by explaining risks and alternatives, document the discussion, and keep offering appropriate options.

#669 · Client Safety and Client-Centred Care

What is the legal presumption about decision-making capacity in adults?

Answer

Adults are presumed capable of making their own decisions unless shown otherwise. Capacity is specific to the decision and time, and can fluctuate, so assess it for each decision.

#670 · Client Safety and Client-Centred Care

A client lacks capacity to consent. Who normally makes the treatment decision?

Answer

A substitute decision maker, determined by provincial legislation, often a hierarchy beginning with a court-appointed guardian or attorney for personal care, then spouse or partner, adult children, and so on. Follow the client's prior known wishes.

#671 · Client Safety and Client-Centred Care

What are the key elements for assessing whether a client has decision-making capacity?

Answer

Whether the client understands relevant information, appreciates how it applies to their own situation, can reason about options, and can communicate a choice. Capacity concerns the specific decision, not a diagnosis.

#672 · Client Safety and Client-Centred Care

What is advance care planning, and how does it affect your treatment decisions?

Answer

A process in which a person records their wishes and names a substitute decision maker for future incapacity. Clinicians must follow clearly applicable prior wishes, and unclear ones should be discussed with the substitute.

#673 · Client Safety and Client-Centred Care

What is the purpose of draping during a physiotherapy examination?

Answer

To protect the client's privacy and dignity by exposing only the area being assessed or treated. Explain the drape plan beforehand, and uncover one region at a time.

#674 · Client Safety and Client-Centred Care

A client is uncomfortable undressing for a lumbar assessment. What is the best response?

Answer

Offer alternatives such as a gown, shorts, or a loose drape, explain why exposure is needed, and let the client choose. Never pressure a client, and accept refusal while adapting the assessment.

#675 · Client Safety and Client-Centred Care

When should you offer a chaperone, and who may request one?

Answer

Offer one whenever sensitive areas will be examined or when the client or you feel it would help. The client may request a chaperone at any time, and the request must be respected and documented.

#676 · Client Safety and Client-Centred Care

What should you do before treating the pelvic floor or another sensitive area?

Answer

Explain the purpose and procedure, obtain specific informed consent, offer a chaperone, ensure privacy and draping, and confirm the client can stop at any time. Document the consent discussion.

#677 · Client Safety and Client-Centred Care

How can the treatment room setup protect client privacy and dignity?

Answer

Use a closed door or full curtain, knock before entering, avoid sharing the client's information within hearing of others, and keep draping in place. Environment is part of respectful care.

#678 · Client Safety and Client-Centred Care

Which exercise intensity guide is commonly used to keep pregnant clients safe?

Answer

Moderate intensity, where the client can still talk during exercise (talk test). The Canadian 2019 Guideline for Physical Activity throughout Pregnancy recommends at least 150 minutes of moderate activity weekly for clients without contraindications.

#679 · Client Safety and Client-Centred Care

Name an absolute reason to stop exercise during pregnancy and seek medical advice.

Answer

Vaginal bleeding, persistent loss of fluid from the vagina, regular painful uterine contractions, severe chest pain, persistent excessive shortness of breath that does not resolve with rest, or persistent dizziness or faintness that does not resolve with rest (2019 Canadian pregnancy guideline). Stop activity and refer promptly.

#680 · Client Safety and Client-Centred Care

After the first trimester, what positioning precaution applies to pregnant clients?

Answer

Avoid prolonged supine positioning, which may compress the inferior vena cava and reduce venous return. Use side-lying or a wedge, and watch for lightheadedness, nausea, or dizziness.

#681 · Client Safety and Client-Centred Care

Which exercise type is a priority for older adults according to Canadian 24-hour movement guidelines for adults 65 and older?

Answer

Muscle and bone strengthening activities at least twice a week, balance training, plus at least 150 minutes of moderate to vigorous aerobic activity weekly. Adapt for frailty and chronic conditions.

#682 · Client Safety and Client-Centred Care

What is the main exercise safety concern when an older client takes antihypertensive drugs and moves from lying to standing?

Answer

Orthostatic hypotension causing dizziness and falls. Change positions slowly, check blood pressure when indicated, hydrate, and stay close during transfers and exercise.

#683 · Client Safety and Client-Centred Care

What should be avoided in resistance exercise for growing children?

Answer

Maximal lifts with poor technique and unsupervised heavy loads. Supervised resistance training with proper technique and appropriate loads is safe and beneficial, and growth plates are protected by good supervision.

#684 · Client Safety and Client-Centred Care

What infection control step is most important when treating an immunocompromised client?

Answer

Strict hand hygiene, cleaning equipment between clients, and avoiding treatment when the physiotherapist is ill. Consider scheduling the client early in the day and using appropriate personal protective equipment.

#685 · Client Safety and Client-Centred Care

A client undergoing chemotherapy has low platelets. What exercise precaution applies?

Answer

Avoid high-risk activities and vigorous resistance that could cause bruising or bleeding, and check current lab values with the oncology team. Follow the team's thresholds for exercise, as limits vary.

#686 · Client Safety and Client-Centred Care

Why are latex allergies a concern in physiotherapy, and what is a safe alternative?

Answer

Latex can trigger reactions from mild contact dermatitis to anaphylaxis. Ask about allergies at intake and use nitrile or vinyl gloves and latex-free bands and tubing for sensitive clients.

#687 · Client Safety and Client-Centred Care

What should you do before applying a topical gel such as ultrasound gel or a counterirritant to a new client?

Answer

Ask about allergies and skin sensitivity, inspect the skin, and consider a small patch test. Stop immediately and wash the area if redness, itching, or burning develops.

#688 · Client Safety and Client-Centred Care

A client develops a rash under electrode pads after electrical stimulation. What is the likely cause and your action?

Answer

Likely contact dermatitis from adhesive, gel, or a chemical burn from current density. Stop the treatment, inspect and document the skin, and use hypoallergenic electrodes or change technique at the next visit.

#689 · Client Safety and Client-Centred Care

Why should skin be checked before and after applying heat or cold modalities?

Answer

To detect sensation changes, erythema, blistering, or burns, particularly in clients with impaired sensation or circulation. Compare with the initial skin check and document changes.

#690 · Client Safety and Client-Centred Care

What is the safest approach to taping fragile or older skin?

Answer

Use hypoallergenic tape, avoid pulling tight, apply an underlay, and remove gently in the direction of hair growth. Check the skin after removal and stop use if irritation occurs.

#691 · Client Safety and Client-Centred Care

Which anatomical areas are the most common sites for pressure injuries?

Answer

Bony prominences such as the sacrum, heels, ischial tuberosities, greater trochanters, and occiput. Pressure plus shear and moisture over these sites over time causes tissue damage.

#692 · Client Safety and Client-Centred Care

A client with limited mobility is positioned on a plinth for 45 minutes. How do you reduce pressure injury risk?

Answer

Use padding over bony prominences, avoid wrinkled linen, reposition regularly, and inspect the skin afterward. Limit time in one position, especially for clients with impaired sensation.

#693 · Client Safety and Client-Centred Care

What is the difference between pressure and shear when moving a client in bed?

Answer

Pressure is force perpendicular to the skin, while shear is sliding force parallel to it. Lift rather than drag the client, using slide sheets, to avoid shear injury.

#694 · Client Safety and Client-Centred Care

What should you do before mobilizing a client who has an indwelling urinary catheter?

Answer

Secure the tubing, keep the drainage bag below bladder level, avoid kinks, and never lift the bag above the bladder. Empty the bag if full to prevent traction or backflow.

#695 · Client Safety and Client-Centred Care

What is the safe handling of chest tubes and drains during mobilization?

Answer

Keep the drainage system upright and below chest level, avoid kinking or pulling, and never clamp unless ordered. Report sudden changes in drainage or breathing difficulty immediately.

#696 · Client Safety and Client-Centred Care

What are key oxygen safety rules in a clinic where a client uses supplemental oxygen?

Answer

No open flames, smoking, or sparks nearby, because oxygen supports combustion. Secure cylinders upright, avoid oil-based products near equipment, and keep tubing free of trip hazards.

#697 · Client Safety and Client-Centred Care

Where should the physiotherapist position themselves when guarding a client who is walking with a high risk of falling?

Answer

Stand slightly behind and to the weaker side, close enough to catch the client, with a hand on the gait belt. This position lets you control the trunk and the fall direction without blocking the client's stepping.

#698 · Client Safety and Client-Centred Care

Why should a gait belt be placed over clothing at the waist and never be used as a lifting handle?

Answer

A belt at the waist sits near the centre of mass, giving control of balance loss. Lifting by the belt can injure the client's ribs or abdomen. Its purpose is guarding and controlled lowering, not lifting.

#699 · Client Safety and Client-Centred Care

Which common medication group is strongly linked to falls in older adults and should prompt a referral for medication review?

Answer

Sedatives, hypnotics, antidepressants and antipsychotics, along with having four or more medications (polypharmacy). These cause dizziness, slowed reaction and orthostatic hypotension. Flag the finding to the prescriber or pharmacist for review.

#700 · Client Safety and Client-Centred Care

A client reports a fall in the past year. What is the single strongest predictor of another fall that this history reveals?

Answer

A previous fall is one of the strongest predictors of future falls. It should trigger a multifactorial falls risk assessment covering balance, strength, vision, medications, footwear, cognition and home hazards.

#701 · Client Safety and Client-Centred Care

Name three environmental hazards in a clinic gym that you should check and fix before treating a client with poor balance.

Answer

Wet or cluttered floors, trailing cords or equipment in walkways, and poor lighting. Also check the bed or plinth height and locked brakes on chairs and wheelchairs before any transfer or standing task.

#702 · Client Safety and Client-Centred Care

What does equitable access to physiotherapy mean, and what is one barrier a physiotherapist can help reduce?

Answer

Clients receive care based on need, not on income, location or background. Barriers include cost, rural distance, language and mobility. Physiotherapists can advocate for sliding fees, virtual options, interpreters and accessible clinic design.

#703 · Client Safety and Client-Centred Care

How does the physiotherapist act as an advocate at the system level, beyond advocating for a single client?

Answer

By supporting changes in policy, funding or program design that improve access and outcomes for groups of clients, for example through professional associations, community committees or submissions to government, in addition to individual client advocacy.

#704 · Client Safety and Client-Centred Care

What is resource stewardship in physiotherapy practice?

Answer

Using time, equipment and public funds responsibly by choosing evidence-based, necessary interventions, avoiding low-value care, and discharging when goals are met, so resources remain available for other clients who need them.

#705 · Client Safety and Client-Centred Care

Why do physiotherapists partner with community organizations when planning services for underserved groups?

Answer

Community partners know local needs, build trust and reach clients the clinic cannot. Partnerships such as with seniors centres or Indigenous health organizations support culturally safe, accessible programs and shared use of resources.

#706 · Client Safety and Client-Centred Care

What should you do when a client's insurer funding runs out before the client's goals have been met?

Answer

Discuss the situation openly, set priorities for remaining visits, teach a home program, and explore other funding or community programs. Do not abandon the client or discharge without a plan for continued care.

#707 · Client Safety and Client-Centred Care

Why is a family member generally not the preferred interpreter for a client who does not speak the clinic language?

Answer

Family may filter, change or omit information, and the client may not speak freely about sensitive topics. A trained, qualified interpreter protects accuracy, confidentiality and the client's informed participation.

#708 · Client Safety and Client-Centred Care

What is the teach-back method and what does it confirm?

Answer

You ask the client to explain or demonstrate the instructions in their own words. It checks that the message was understood, not just heard, and puts responsibility for clear explanation on the clinician, not the client.

#709 · Client Safety and Client-Centred Care

What written material approach best supports clients with low health literacy when giving a home exercise program?

Answer

Use plain language, short sentences, large clear pictures and only the few most important points. Avoid jargon, and confirm understanding with teach-back. Aim for a reading level of about grade six.

#710 · Client Safety and Client-Centred Care

When working with a qualified interpreter, where should the physiotherapist look and speak during the session?

Answer

Speak directly to the client, not the interpreter, using short sentences and the first person. Pause for interpretation, and brief the interpreter beforehand. This keeps the therapeutic relationship between you and the client.

#711 · Client Safety and Client-Centred Care

Why is a signature on a consent form not enough if the client cannot understand the language it was written in?

Answer

Valid informed consent requires that the client understands the nature, risks, benefits and alternatives. Without an interpreter or translated information, understanding cannot be shown, so the consent is not informed and may not be valid.

#712 · Client Safety and Client-Centred Care

What is the recommended first step when you suspect a client is experiencing intimate partner violence?

Answer

Ask privately, with the client alone and without the partner or family present, using non-judgmental open questions. Respond with support, believe the client, and respect their pace and decisions about next steps.

#713 · Client Safety and Client-Centred Care

What is a safety plan for a client who discloses ongoing abuse at home?

Answer

A personalized plan for staying safer, covering safe contacts, a packed emergency bag, important documents, a safe place to go and code words. Offer it and local crisis line or shelter information, with the client directing choices.

#714 · Client Safety and Client-Centred Care

A client discloses abuse but does not want you to tell anyone. What generally guides your decision about reporting an adult?

Answer

Competent adults generally control disclosure, so respect confidentiality unless there is a legal duty or imminent serious risk. Child protection reporting duties are different and mandatory. Know your provincial legislation and college guidance.

#715 · Client Safety and Client-Centred Care

What documentation approach is appropriate when a client's injuries suggest possible abuse?

Answer

Record objective findings, the client's own words in quotation marks, and body location and size of injuries, factually and without speculation. Store the record securely, as it may later be used for legal purposes.

#716 · Client Safety and Client-Centred Care

A client with signs of acute distress and thoughts of self-harm states a plan. What is the first action?

Answer

Do not leave the client alone. Assess immediate risk directly, remove access to means if possible, and arrange urgent help through emergency services, a crisis line or the nearest emergency department, as clinic policy directs.

#717 · Client Safety and Client-Centred Care

What should be confirmed before choosing a telehealth physiotherapy visit for a client?

Answer

Suitability: the condition is safe to assess and treat remotely, the client has adequate technology and a private space, and has no red flags requiring in-person examination. Also obtain consent for virtual care.

#718 · Client Safety and Client-Centred Care

Before a telehealth session begins, what safety information must you record about the client's location?

Answer

The client's exact physical address and a phone number, plus a local emergency contact. This lets you direct emergency services to the correct place if the client becomes unwell or the connection fails.

#719 · Client Safety and Client-Centred Care

What privacy steps apply when delivering physiotherapy by video?

Answer

Use a platform meeting provincial health privacy requirements with encryption, work in a private space, verify the client's identity, confirm who else is present, and obtain consent for any recording. Avoid public Wi-Fi.

#720 · Client Safety and Client-Centred Care

What is a key limitation of remote monitoring when assessing a client's balance or neurological status over video?

Answer

You cannot hands-on test tone, sensation, strength or give physical protection during balance tasks. Limit tasks to what is safe without guarding, use a stable support, and move to in-person care when findings are unclear.

#721 · Professional Responsibilities

What is the primary duty of a provincial physiotherapy regulatory college?

Answer

To protect the public interest by regulating the profession: setting registration standards and practice standards, and handling complaints and discipline. It is not an advocate for physiotherapists, which is the role of professional associations.

#722 · Professional Responsibilities

What distinguishes a regulatory college from a professional association such as the Canadian Physiotherapy Association?

Answer

The college is created by provincial law to protect the public and licenses members. The association advocates for the profession, offers education and member services, and has no legal authority to license or discipline.

#723 · Professional Responsibilities

What generally happens first when a member of the public files a complaint about a physiotherapist with the college?

Answer

The college's complaints process begins with screening and investigation, and the physiotherapist is notified and given a chance to respond. A committee then decides to dismiss, resolve, refer to discipline or require remedial steps.

#724 · Professional Responsibilities

What outcomes can a discipline committee order if it finds a physiotherapist guilty of professional misconduct?

Answer

Possible orders include a reprimand, conditions on practice, suspension, revocation of registration, fines and costs. Findings are often published, because the focus is public protection and maintaining trust.

#725 · Professional Responsibilities

Why must a physiotherapist keep their registration current and use only the protected title they hold?

Answer

Practising or using the protected title physiotherapist without current registration is prohibited by provincial legislation and may lead to penalties. Registration confirms the person meets the entry standards and is accountable to the college.

#726 · Professional Responsibilities

Where does a physiotherapist's scope of practice come from?

Answer

From provincial health professions legislation and the college's standards, shaped by the individual's own education, training and competence. Being within the legal scope does not mean every activity is within your personal competence.

#727 · Professional Responsibilities

What is a controlled act in Canadian health regulation, and why does it matter to physiotherapy?

Answer

A controlled act is a higher-risk procedure that only authorized professions may perform, defined in provincial law (for example in Ontario under the Regulated Health Professions Act). Know which acts your province authorizes you to perform.

#728 · Professional Responsibilities

A physiotherapist plans to perform spinal manipulation. What must be confirmed first?

Answer

That your provincial legislation and college standards authorize it, that you have the competence through education and training, and that informed consent and screening for contraindications were completed beforehand.

#729 · Professional Responsibilities

When a physiotherapist delegates a task to a physiotherapist assistant, who remains accountable for the client's care?

Answer

The physiotherapist. Delegation requires assessing the assistant's competence, giving clear instructions, providing appropriate supervision and monitoring the outcome. You may not delegate your clinical judgment or the initial assessment.

#730 · Professional Responsibilities

What should you do when asked to perform a technique you were not trained in?

Answer

Decline to perform it until you have the education and supervised practice to be competent, or refer the client to someone who is. Practising outside your competence puts the client at risk and may be misconduct.

#731 · Professional Responsibilities

Can a client withdraw consent partway through a treatment, and what must the physiotherapist do?

Answer

Yes. Consent can be withdrawn at any time, with or without a reason. Stop the treatment immediately, check that the client is safe and comfortable, discuss concerns, and document the withdrawal.

#732 · Professional Responsibilities

A physiotherapy student wishes to treat a client in your clinic. What does the client need to be told?

Answer

That a student will be involved, the student's role and level of supervision, and that the client may refuse student involvement without any effect on the quality of their care. Obtain and document the client's agreement.

#733 · Professional Responsibilities

Is a general consent to treatment sufficient for taking and using photographs or video of a client?

Answer

No. Separate, specific consent is needed that states the purpose, who will see the images and how they are stored. Consent for teaching or publication is distinct again, and the client may withdraw consent.

#734 · Professional Responsibilities

Who gives consent for treatment of a child, and what is the key principle about a mature minor?

Answer

A capable child may consent for themselves; capacity is based on understanding, not a fixed age, under most provincial approaches. If the child lacks capacity, a parent or legal substitute decision-maker gives consent. Know your provincial rules.

#735 · Professional Responsibilities

Which federal privacy law applies to private-sector commercial activity where no provincial privacy law is substantially similar?

Answer

PIPEDA, the Personal Information Protection and Electronic Documents Act. Many provinces have their own health privacy acts governing health information, and those usually apply to physiotherapists as custodians or health information custodians.

#736 · Professional Responsibilities

What is the circle of care and how does it affect sharing client information?

Answer

It describes health providers directly involved in a client's care who can rely on implied consent to share information needed for that care. Sharing outside the circle, such as to an employer or lawyer, generally requires consent.

#737 · Professional Responsibilities

A client asks for a copy of their physiotherapy record. What is the general rule?

Answer

Clients have a right of access to their own health record, with limited exceptions set in legislation, such as risk of serious harm to others. The clinic must respond within the legislated timeframe and may charge only a reasonable fee.

#738 · Professional Responsibilities

You discover a client's record was sent to the wrong person by email. What is the appropriate response?

Answer

Act immediately to contain the breach, retrieve or secure the information, notify the client and the privacy officer, document the event, and report to the privacy commissioner or college where legislation requires. Then review safeguards.

#739 · Professional Responsibilities

Why is looking at the record of a friend or co-worker who is not your client a privacy breach?

Answer

Access is allowed only when needed for care or another authorized purpose. Snooping without a legitimate need violates privacy legislation and professional standards, even if no information is shared further, and can lead to discipline.

#740 · Professional Responsibilities

What is the duty to warn, and when can confidentiality be broken for it?

Answer

When there is a clear, serious and imminent risk of harm to an identifiable person or group, a health professional may disclose the minimum necessary information to prevent it. Provincial law and college guidance define the threshold.

#741 · Professional Responsibilities

A physiotherapist suspects that a child is being abused or neglected. What is the legal obligation?

Answer

Report the concern promptly to the child protection agency. Provincial child welfare legislation imposes a mandatory duty on everyone, including health professionals, and the duty cannot be set aside by confidentiality or by the family's wishes.

#742 · Professional Responsibilities

You receive a subpoena requesting a client's physiotherapy records. How should you respond?

Answer

Do not ignore it. Verify that it is valid, tell the client, and seek advice from the college, your professional liability insurer or a lawyer. Disclose only what is legally required and document what was released.

#743 · Professional Responsibilities

An insurer asks for a client's full file to process a claim. What must be in place before releasing it?

Answer

The client's valid consent, usually written, that identifies the information and the purpose. Release only what the insurer needs for the claim, not the whole record, and document the disclosure.

#744 · Professional Responsibilities

What is a conflict of interest for a physiotherapist, and whose interest must take priority?

Answer

A situation where a personal or financial interest could influence, or appear to influence, professional judgment. The client's interest always takes priority. Disclose the conflict and manage it, or step away from the decision.

#745 · Professional Responsibilities

A client offers a gift of significant value after discharge. What is the safest professional response?

Answer

Decline politely and explain the reason, because a large gift can create obligation or influence future care. Small tokens of thanks are usually acceptable under college guidance. Document the offer and check your college's standard.

#746 · Professional Responsibilities

You sell braces or orthotics in your clinic. What must you do to avoid a conflict of interest?

Answer

Recommend products only if clinically indicated, disclose your financial benefit, and tell the client they may buy elsewhere. Never pressure the client. The recommendation must be based on need, not profit.

#747 · Professional Responsibilities

You want to refer a client to an imaging or exercise facility you partly own. What is required?

Answer

Disclose your financial interest before referring, and tell the client they are free to choose another provider without any effect on their care. Self-referral without disclosure is a conflict of interest.

#748 · Professional Responsibilities

How do college standards generally treat payments or incentives received for referring clients to another provider?

Answer

Accepting payment or incentives for referrals is generally treated as a conflict of interest and professional misconduct. Referrals must be based on client need alone. Check your provincial college's standard for the exact wording.

#749 · Professional Responsibilities

What is a dual relationship, and why is it a risk in physiotherapy?

Answer

A dual relationship means holding a second role with a client, such as friend, employer, or business partner. It blurs objectivity, can exploit the power imbalance, and may harm the therapeutic relationship.

#750 · Professional Responsibilities

Does a client's consent make a sexual relationship with a current client acceptable?

Answer

No. Because of the power imbalance, client consent is not a defence. Sexual abuse of a client is serious professional misconduct and, in some provinces, carries mandatory penalties up to loss of registration.

#751 · Professional Responsibilities

What is the guidance on social media contact and posts involving clients?

Answer

Do not accept personal friend requests from current clients. Never post identifiable client information or photos without written consent. Even without a name, details can identify a client, so avoid case stories.

#752 · Professional Responsibilities

You have reasonable grounds to believe another regulated health professional sexually abused a client. What is your duty?

Answer

Report to the professional's regulatory college as provincial law requires. In Ontario, the Regulated Health Professions Act sets a written report within 30 days. Client safety outweighs collegial loyalty.

#753 · Professional Responsibilities

What is the difference between a boundary crossing and a boundary violation?

Answer

A crossing is a brief, usually harmless departure from usual practice, such as a short personal chat, that serves the client. A violation exploits the client or serves the therapist's needs and causes harm.

#754 · Professional Responsibilities

What does sensitivity tell you, and what is the memory hook?

Answer

Sensitivity is the proportion of people with the condition who test positive. A highly sensitive test with a negative result helps rule the condition out: SnNout.

#755 · Professional Responsibilities

What does specificity tell you, and what is the memory hook?

Answer

Specificity is the proportion of people without the condition who test negative. A highly specific test with a positive result helps rule the condition in: SpPin.

#756 · Professional Responsibilities

How is the positive likelihood ratio calculated, and what value gives a large shift in probability?

Answer

LR+ equals sensitivity divided by (1 minus specificity). An LR+ above 10 produces a large, often conclusive increase in post-test probability. Values near 1 change almost nothing.

#757 · Professional Responsibilities

How do you calculate number needed to treat (NNT) from a trial result?

Answer

NNT equals 1 divided by the absolute risk reduction. It is the number of patients who must be treated to prevent one additional bad outcome. A smaller NNT means a more effective intervention.

#758 · Professional Responsibilities

A 95% confidence interval for a risk ratio is 0.8 to 1.4. How do you interpret it?

Answer

The interval includes 1, the null value for ratios, so the result is not statistically significant at the 0.05 level. For mean differences the null value is 0.

#759 · Professional Responsibilities

For a question about treatment effectiveness, which study design sits at the top of the evidence hierarchy?

Answer

A systematic review (with meta-analysis) of randomized controlled trials. Below it come single RCTs, cohort studies, case-control studies, case series, and finally expert opinion at the bottom.

#760 · Professional Responsibilities

What is the minimal clinically important difference (MCID)?

Answer

The smallest change in an outcome measure that a patient would perceive as meaningful or beneficial. A statistically significant change smaller than the MCID may not matter to the client.

#761 · Professional Responsibilities

What are the three components of evidence-informed practice?

Answer

Best available research evidence, the clinician's expertise, and the client's values and preferences. All three guide the decision, so research alone never dictates the treatment.

#762 · Professional Responsibilities

What does PICO stand for when forming a clinical question?

Answer

Population, Intervention, Comparison, Outcome. A well built PICO question focuses a literature search, for example: in adults with knee osteoarthritis, does exercise versus no exercise reduce pain?

#763 · Professional Responsibilities

How should a physiotherapist use a clinical practice guideline with an individual client?

Answer

Use it as a systematically developed support for decisions, then adapt it to the client's presentation, comorbidities, and preferences. Guidelines inform judgment but do not replace it.

#764 · Professional Responsibilities

What is the first step in building a continuing competence plan?

Answer

Reflect on and identify your learning needs, using self-assessment against the national Competency Profile for Physiotherapists in Canada and practice data. Then set goals and activities to address the gaps.

#765 · Professional Responsibilities

What is the purpose of a college's quality assurance program?

Answer

To confirm that registrants maintain and improve competence throughout their careers. Typical elements are self-assessment, a learning plan, activity records, and possible practice review or portfolio audit.

#766 · Professional Responsibilities

How should you respond to critical feedback from a colleague or client?

Answer

Listen without defensiveness, clarify the concern, reflect on whether it reveals a learning need, and act on it. Seeking feedback regularly is a core part of reflective practice.

#767 · Professional Responsibilities

What makes a learning goal SMART?

Answer

Specific, Measurable, Achievable, Relevant, and Time-bound. For example: complete a vestibular rehabilitation course and apply the technique with five clients within six months.

#768 · Professional Responsibilities

Why should you keep records of your continuing competence activities?

Answer

The college can request or audit them to confirm participation in its quality assurance program. Records should show the learning need, the activity, and how it changed your practice.

#769 · Professional Responsibilities

When should a physiotherapist refer a client to another professional?

Answer

When the client's needs exceed your scope or competence, when red flags appear, or when there is no meaningful progress as expected. Explain the reason to the client and communicate with the receiving provider.

#770 · Professional Responsibilities

What is the difference between scope of practice and personal competence?

Answer

Scope is what the law and your college allow the profession to do. Competence is what you personally are trained and able to do safely. Practise only where both apply.

#771 · Professional Responsibilities

You are asked to treat a condition you have never managed. What is the appropriate action?

Answer

Recognize the limit of your competence, seek mentorship or consultation, complete training before taking on the case, or refer the client. Never learn unsupervised on a client.

#772 · Professional Responsibilities

How can a physiotherapist end a therapeutic relationship without abandoning the client?

Answer

Give reasonable notice, explain the reason, arrange or recommend appropriate alternative care, provide needed records or a summary, and document the process. The client's care must not be left unsafe.

#773 · Professional Responsibilities

An employer pressures you to treat beyond your competence. What should you do?

Answer

Your duty to the client and your college standards come first. Decline the task, explain your concern, seek supervision or training, and document the discussion. Do not compromise client safety.

#774 · Professional Responsibilities

What are the three dimensions of burnout?

Answer

Emotional exhaustion, depersonalization (detachment or cynicism toward clients), and reduced sense of personal accomplishment. It results from chronic, unmanaged workplace stress.

#775 · Professional Responsibilities

How does compassion fatigue differ from burnout?

Answer

Compassion fatigue comes from repeated exposure to clients' suffering and trauma, while burnout comes from chronic workplace stress and overload. Both reduce well-being and can affect care quality.

#776 · Professional Responsibilities

A physiotherapist's substance use is starting to affect their judgment. What is their duty?

Answer

Stop practising until safe, seek help, and follow the college's requirements. Practising while impaired puts clients at risk and is a professional conduct matter. Early treatment protects both client and clinician.

#777 · Professional Responsibilities

You suspect a colleague is impaired at work. What is the first priority?

Answer

Protect client safety. Raise your concern with the colleague or supervisor when appropriate, and report to the regulatory college if there is risk of harm. Concern for a colleague does not override this duty.

#778 · Professional Responsibilities

What is an Employee Assistance Program, and why does it matter for practitioner well-being?

Answer

A confidential, employer-funded service offering counselling and support for personal or work stress. Physiotherapists should use such supports early rather than waiting until stress affects their practice.

#779 · Professional Responsibilities

What are the social determinants of health?

Answer

The conditions in which people are born, live, work, and age, such as income, housing, education, food security, employment, and discrimination. They strongly shape health outcomes and access to rehabilitation.

#780 · Professional Responsibilities

A client cannot afford private physiotherapy visits. How should you respond?

Answer

Discuss options openly: a home program, fewer supervised visits, publicly funded services, or community resources. Adapt the plan to the client's circumstances and advocate for access without lowering safe care.

#781 · Professional Responsibilities

What does cultural safety mean in physiotherapy, especially with Indigenous clients?

Answer

Care in which the client decides whether the experience feels respectful and safe. It requires reflecting on your own bias, power, and the legacy of colonialism and racism, beyond simply learning about cultures.

#782 · Professional Responsibilities

What key issues must be addressed before delivering physiotherapy by virtual care?

Answer

Informed consent for virtual care, a secure and private platform, the limits of remote assessment, safety planning, and meeting registration requirements for the province where the client is located.

#783 · Professional Responsibilities

A physiotherapist uses an artificial intelligence tool to help draft a plan. What responsibilities remain?

Answer

The physiotherapist remains fully accountable for clinical decisions and must verify the output. Do not enter identifiable client information into tools that lack approved privacy safeguards, and obtain consent where required.

#784 · Professional Responsibilities

What is the standard for truthful advertising by a physiotherapist?

Answer

Advertising must be accurate and verifiable, and must not be false, misleading, or exaggerated. Do not guarantee outcomes or imply superiority without evidence, and use protected titles only if registered.

#785 · Professional Responsibilities

How should the length of a treatment course be determined?

Answer

By ongoing clinical reassessment and the client's needs and goals, not by revenue or insurance maximums. Continuing treatment without expected benefit is an ethical and billing concern.

#786 · Professional Responsibilities

When is billing considered fraud?

Answer

When you bill for services not provided, for longer or different services than delivered, or under an inaccurate code to gain payment. Bills to clients and third-party payers must match what was actually done.

#787 · Professional Responsibilities

What should a client be told about fees?

Answer

Fees and any cancellation or other charges must be explained clearly before treatment begins, and the client should be told what an insurer will and will not cover. Transparent communication prevents later disputes.

#788 · Professional Responsibilities

A client discloses information that makes you suspect child abuse. What is your duty?

Answer

Child protection laws require anyone, including health professionals, who has reasonable grounds to suspect a child needs protection to report promptly to the child welfare agency. The duty cannot be delegated to a colleague.

#789 · Professional Responsibilities

Is reporting suspected elder abuse mandatory for all physiotherapists across Canada?

Answer

No. Mandatory reporting of elder abuse varies by province and setting, such as long-term care. Know your own province's law, and where reporting is not required, still protect the client and consider consulting.

#790 · Professional Responsibilities

A client with a medical condition continues to drive unsafely. What should you check?

Answer

Whether your province requires reporting medical unfitness to drive to the licensing authority, and which professionals the duty applies to. Otherwise counsel the client, involve their physician, and document.

#791 · Professional Responsibilities

What should you do if you see another professional practising in an unsafe way?

Answer

Act to protect clients: address it directly if appropriate, and report to the person's regulatory college when you have reasonable grounds to believe clients are at risk. Safety outweighs loyalty to a colleague.

#792 · Professional Responsibilities

When may you disclose personal health information without the client's consent?

Answer

When the law requires it, such as mandatory reports or a court order, or when needed to prevent serious imminent harm. Disclose only the minimum necessary information, and document the reason.

#793 · Collaboration

A client has diabetes, a recent stroke, and depression, and progress has stalled. What approach to care does this complexity call for?

Answer

An interprofessional, collaborative approach. Multiple interacting conditions exceed one profession's expertise, so shared goals and coordinated input from several providers give safer, more effective care than working alone.

#794 · Collaboration

Your client reports a new, unexplained weight loss and night sweats during rehabilitation. What is the appropriate collaborative action?

Answer

Refer promptly to the physician or nurse practitioner for medical assessment. These are red flags outside physiotherapy scope, so the physiotherapist communicates findings and holds off treatment that could mask the problem.

#795 · Collaboration

When should a physiotherapist refer a client to another profession rather than continue treating alone?

Answer

When client needs fall outside physiotherapy scope or your competence, such as swallowing or cognitive-communication problems. Refer when another provider's expertise will improve outcomes, and keep the client informed.

#796 · Collaboration

Why is collaboration especially valuable for a client with chronic pain and complex psychosocial barriers?

Answer

Pain has biological, psychological, and social contributors. A team (physiotherapy, psychology, physician, social work) can address each factor, which a single discipline cannot, improving function and reducing duplication.

#797 · Collaboration

Physiotherapy goals conflict with the client's other providers' plans. What is the first step toward resolving the issue?

Answer

Communicate directly with those providers to clarify goals and share findings. Aim for a unified, client-centred plan with agreed priorities rather than working in parallel with contradictory instructions.

#798 · Collaboration

Which team member addresses activities of daily living, home adaptations, and adaptive equipment, often overlapping with physiotherapy mobility goals?

Answer

The occupational therapist. They focus on meaningful daily activities, cognition-related function, and environmental modification, while the physiotherapist focuses on movement, strength, and mobility. Goals are coordinated.

#799 · Collaboration

Which professional assesses and manages swallowing difficulty (dysphagia) and communication disorders after a stroke?

Answer

The speech-language pathologist. Refer clients with coughing during meals, aspiration concerns, or speech and language deficits. The physiotherapist may flag signs and support positioning during eating.

#800 · Collaboration

What contribution does a registered dietitian make to a client's rehabilitation plan?

Answer

Assesses nutritional status and designs diet plans for malnutrition, diabetes, weight management, or wound healing. Adequate protein and energy intake supports tissue repair and exercise tolerance in therapy.

#801 · Collaboration

When would a physiotherapist involve a social worker in a client's care?

Answer

When barriers such as housing, financial strain, caregiver burnout, abuse concerns, or access to community services limit recovery. The social worker connects clients to resources and supports coping and discharge planning.

#802 · Collaboration

Which team member typically provides 24-hour bedside monitoring and medication administration, and is often first to notice changes in a hospitalized client?

Answer

The nurse. Communicate activity tolerance, mobility precautions, and transfer plans to nursing, and ask about overnight observations such as pain, dizziness, or skin changes that affect therapy.

#803 · Collaboration

What does the SBAR communication tool stand for?

Answer

Situation, Background, Assessment, Recommendation. It structures concise, clear handover, for example reporting a client's deteriorating status, so the receiver understands the concern and the requested action quickly.

#804 · Collaboration

In an SBAR report about a client who became dizzy on standing, what belongs in the Recommendation step?

Answer

The specific action requested, such as medical review, orthostatic vital sign checks, or a change in mobility orders. State what you need and by when, rather than only describing the problem.

#805 · Collaboration

Can a physiotherapist share client health information with other team members involved in the client's care without explicit new consent?

Answer

Generally yes within the circle of care, under implied consent in many provinces, but share only what is necessary. Obtain express consent for sharing outside the circle of care, and follow provincial privacy legislation.

#806 · Collaboration

What is the main purpose of an interprofessional case conference?

Answer

To bring the team and often the client together to share assessments, agree on shared goals, assign responsibilities, and plan discharge. It reduces fragmented, duplicated, or conflicting care.

#807 · Collaboration

What should a physiotherapist's written report to a referring physician include to support collaborative care?

Answer

Reason for referral, relevant findings, clinical impression, treatment plan, expected progress, and any concerns needing medical follow-up. Write clearly, avoid unexplained jargon, and keep it timely and confidential.

#808 · Collaboration

Two professionals on your team both perform balance training with the same client. How should this overlap be handled?

Answer

Discuss it openly to clarify who does what, agree on progression and documentation, and avoid duplication or conflicting instructions. Overlapping scopes are acceptable when roles are negotiated.

#809 · Collaboration

What does role clarity mean on an interprofessional team?

Answer

Each member understands their own responsibilities, scope, and limits, and those of others. Clear roles reduce gaps and duplication, build trust, and let the client receive coordinated care.

#810 · Collaboration

A physiotherapist's scope overlaps with an occupational therapist's in upper-limb rehabilitation. What does a good team agreement define?

Answer

Which discipline leads which intervention, how goals and progress are shared, and who communicates with the client. The agreement is documented so all members apply it consistently.

#811 · Collaboration

Who holds accountability for a physiotherapy decision when responsibilities are shared across a team?

Answer

The physiotherapist remains accountable for their own professional judgment and actions under their regulatory college, regardless of team structure. Shared responsibility does not remove individual accountability.

#812 · Collaboration

How should a physiotherapist respond when another profession's practice seems to exceed that profession's scope?

Answer

Raise the concern respectfully with the individual first, using facts and client safety as the focus. If unresolved or unsafe, escalate through the supervisor or the relevant regulatory body.

#813 · Collaboration

Name early warning signs of team conflict that you should address before they affect client care.

Answer

Withdrawal from meetings, side conversations, blaming language, missed handovers, and inconsistent messages to clients. Naming the problem early and calmly prevents escalation and protects care quality.

#814 · Collaboration

What is a constructive first response when you disagree with a colleague about a client's care plan?

Answer

Discuss it privately and respectfully, focus on the client's goals and the evidence, listen to their reasoning, and seek common ground. Avoid criticizing in front of the client or the team.

#815 · Collaboration

A disagreement over a care plan persists after discussion. What is the appropriate next step?

Answer

Escalate to a team lead, manager, or ethics resource, or call a case conference. Keep the client's safety and wishes central and document the steps taken without assigning blame.

#816 · Collaboration

What conflict-resolution approach seeks a solution that meets the needs of all parties, rather than one side winning?

Answer

Collaboration, or win-win problem solving. Parties explore interests behind positions and generate shared solutions. It suits important issues and preserves working relationships better than competing or avoiding.

#817 · Collaboration

A team member repeatedly raises safety concerns about your technique. How should you respond professionally?

Answer

Listen without defensiveness, clarify the concern, review the evidence or protocol, and adjust practice if warranted. Treat feedback as a chance to improve client safety and maintain mutual respect.

#818 · Collaboration

Who is responsible for deciding which tasks to delegate to a physiotherapist assistant?

Answer

The physiotherapist. Delegation is a professional judgment based on the client's condition, task complexity, and the assistant's competence. Accountability for the client's care stays with the physiotherapist.

#819 · Collaboration

Which client factors make delegation to support personnel more appropriate?

Answer

A stable, predictable condition, a well-established plan, low risk of change, and clear parameters. Unstable, complex, or rapidly changing clients need the physiotherapist's own assessment and judgment.

#820 · Collaboration

Can a physiotherapist delegate the initial assessment and clinical decision making to a support worker?

Answer

No. Assessment, diagnosis or clinical impression, treatment planning, and reassessment require physiotherapist judgment and cannot be delegated. Support personnel carry out assigned tasks within the plan.

#821 · Collaboration

What determines the level of supervision a physiotherapist assistant needs?

Answer

Task complexity, client stability and risk, the assistant's competence and experience, and the practice setting. Higher risk or less experience requires closer, more direct supervision, per college standards.

#822 · Collaboration

Why must the physiotherapist be available for consultation when a support person is carrying out delegated care?

Answer

So the assistant can report changes, ask questions, and get immediate direction if the client's response is unexpected. Availability allows timely intervention and keeps the client safe.

#823 · Collaboration

What must be clearly communicated to a support person when assigning a task?

Answer

The specific task, the client's goals, parameters and precautions, expected responses, what to report, and when to stop and seek help. Clear, documented instructions prevent errors and unsafe practice.

#824 · Collaboration

Before assigning a task to support personnel, how can a physiotherapist verify the person's competence?

Answer

Review their training and credentials, confirm employer competency records, and observe them performing the task. Do not assume competence from job title alone.

#825 · Collaboration

A new support worker has not performed a specific modality before. What should the physiotherapist do before delegating it?

Answer

Provide or confirm training, demonstrate the task, observe the worker performing it safely, and confirm competence before assigning it independently. Until verified, do not delegate that task.

#826 · Collaboration

How does task complexity influence the decision to delegate?

Answer

The more judgment, adaptation, and risk a task involves, the less suitable it is for delegation. Simple, routine, well-defined tasks with predictable outcomes are better matched to support personnel.

#827 · Collaboration

What is the most reliable way to confirm that a support person can perform a delegated technique correctly?

Answer

Direct observation of the person performing the technique, with feedback and return demonstration. Observation shows actual performance, which a certificate or verbal assurance cannot.

#828 · Collaboration

After a support person delivers part of a treatment, how should the physiotherapist monitor the client?

Answer

Reassess the client regularly in person, review the assistant's reports and the client's response, and adjust the plan as needed. Ongoing monitoring remains the physiotherapist's responsibility.

#829 · Collaboration

A support person reports that the client had increased pain after an exercise. What should the physiotherapist do?

Answer

Assess the client promptly, review the exercise and its dose, modify or stop the task as needed, and update the plan. Thank the assistant for reporting and document the event.

#830 · Collaboration

What is a constructive way to give feedback to a support person who made an error in delivering care?

Answer

Give timely, private, specific feedback focused on the behaviour and client safety, explain the correct method, check understanding, and arrange retraining if needed. Document significant events.

#831 · Collaboration

If a support person repeatedly works outside their delegated tasks, what is the physiotherapist's appropriate action?

Answer

Address it directly, restate the limits of delegation, retrain or reduce the delegated duties, and report to the employer if unresolved. Client safety and professional accountability come first.

#832 · Collaboration

Who is accountable if a delegated task harms a client because it was assigned inappropriately?

Answer

The physiotherapist who delegated it shares accountability, along with the employer and the support person for their own roles. Poor delegation decisions can lead to college complaints.

#833 · Collaboration

What is a learning plan for a physiotherapy student placement?

Answer

A document stating the student's goals, learning needs, and the activities and supports to meet them. Created with the student early in placement and reviewed regularly, it guides teaching and evaluation.

#834 · Collaboration

What qualities make student feedback effective?

Answer

It is timely, specific, behaviour-focused, balanced between strengths and areas to improve, and linked to goals, with a plan for improvement. Regular formative feedback helps more than only a final evaluation.

#835 · Collaboration

How should a clinical educator keep clients safe when a student treats them?

Answer

Match supervision to the student's competence and the client's risk, obtain client consent for student involvement, and stay available to intervene. The educator remains responsible for client safety.

#836 · Collaboration

A client declines treatment by a student. How should the clinical educator respond?

Answer

Respect the refusal without penalty and provide care by the physiotherapist. Client consent for student participation must be informed and voluntary, and declining must not affect the quality of care.

#837 · Collaboration

What is the difference between formative and summative evaluation of a student?

Answer

Formative evaluation is ongoing feedback during placement to support learning. Summative evaluation is the final judgment of whether the student met the required competencies, such as pass or fail.

#838 · Collaboration

What is a good first step when planning an orientation for a new colleague or support staff member?

Answer

Identify the learner's existing knowledge and skills, then set clear objectives, a schedule, key policies, safety and privacy training, and a named contact person. Plan matches their needs and role.

#839 · Collaboration

Which teaching approach best builds a practical skill, such as safe patient transfers, in a new staff member?

Answer

Demonstrate the skill, explain the reasoning, then have the learner practise with guidance and return demonstration with feedback. Active, hands-on practice builds skill better than lecture alone.

#840 · Collaboration

How can you check that a newly trained staff member is competent after training?

Answer

Observe them performing the task independently, use a checklist or competency assessment, ask questions to probe understanding, and give feedback. Document results and provide further training if gaps remain.

#841 · Collaboration

Why should orientation include privacy and safety training for new staff?

Answer

New staff handle confidential information and risks such as infection, falls, and equipment hazards. Early training in privacy legislation, WHMIS 2015, and emergency procedures protects clients and staff.

#842 · Collaboration

What teaching principle helps adult learners such as new colleagues engage with training?

Answer

Link learning to their practical needs and experience, involve them actively, and give relevant, goal-oriented content. Adults learn best when they see why the material matters to their work.

#843 · Collaboration

Who should be involved in a transition of care meeting before a client is discharged from hospital to home?

Answer

The client, family or caregivers, and the interprofessional team (physiotherapist, nurse, occupational therapist, social worker, physician, home care coordinator). The client is a partner in the plan, not a spectator.

#844 · Collaboration

What discharge information must the physiotherapist pass on to the home care provider so rehabilitation continues safely?

Answer

Current function, mobility aids, precautions, home exercise program, falls risk, goals, and who to call with concerns. Clear, written handover prevents gaps and repeated assessments.

#845 · Collaboration

In most provinces, who coordinates publicly funded home care services such as in-home physiotherapy after discharge?

Answer

A provincial or regional health authority home care coordinator or case manager, often via referral from the hospital team. The physiotherapist recommends needs; the coordinator allocates services based on eligibility.

#846 · Collaboration

A client cannot safely return home alone. Which discharge options should the team discuss with the client and family?

Answer

Home with added supports, retirement or assisted living, supportive housing, or long-term care. The team presents options, risks and costs; the capable client decides, and the physiotherapist contributes function-based evidence.

#847 · Collaboration

Why should a physiotherapist recommend a home visit or home safety assessment before discharge for a client with mobility limits?

Answer

It identifies real barriers (stairs, bathroom, thresholds, clutter, lighting) and equipment needs the hospital cannot show. Usually done with the occupational therapist, it lowers falls and readmission risk.

#848 · Collaboration

When a capable client wants to go home but the family wants long-term care, whose decision prevails?

Answer

The capable client's. Capacity is presumed; the team can share risks and support the family's concerns, but cannot override a capable adult's informed choice about where to live.

#849 · Collaboration

What are the key elements of effective caregiver training in physiotherapy?

Answer

Demonstration, return demonstration by the caregiver, clear written or illustrated instructions, safe handling and body mechanics, and checking understanding. Teaching without watching the caregiver perform leaves safety gaps.

#850 · Collaboration

Who makes treatment decisions for an incapable client who has no written directive?

Answer

A substitute decision-maker, identified by the provincial hierarchy (for example guardian, attorney for personal care, spouse, adult child, parent). They decide based on the client's known wishes, then best interests.

#851 · Collaboration

A daughter asks you to hide a poor prognosis from her capable father. What do you do?

Answer

Do not withhold information from the capable client. Explain that he has the right to know and decide, acknowledge her concern, and offer to support both of them during the conversation.

#852 · Collaboration

Can a physiotherapist share a client's information with the family without consent?

Answer

Generally no. Capable clients decide who receives information. Exceptions are implied consent within the circle of care for the client's care, a substitute decision-maker, or legal exceptions under provincial privacy law.

#853 · Collaboration

How should you respond when family and client goals for rehabilitation conflict?

Answer

Return to the client's own goals, listen to the family's concerns, clarify the reasons for each view, and negotiate a shared plan. The capable client's goals lead the plan.

#854 · Collaboration

When does a physiotherapist need a physician referral to treat a client in most Canadian provinces?

Answer

Most provinces allow direct access with no referral needed. Exceptions include some insurers, public programs, hospital orders or workplace plans that require one. Check your provincial college and the funder's rules.

#855 · Collaboration

What belongs in a physiotherapist's letter back to a referring physician?

Answer

Key assessment findings, clinical impression, treatment plan and expected duration, progress, and any concerns or requests. Keep it concise and specific so the physician can act on it.

#856 · Collaboration

A client reports new chest pain and breathlessness during assessment. What is the first action and who do you collaborate with?

Answer

Stop activity, ensure safety, and activate emergency services if signs are unstable. Afterward, inform the physician and document. Do not wait for a routine referral reply.

#857 · Collaboration

When should a physiotherapist seek medical clearance before starting exercise?

Answer

When screening shows red flags or unstable conditions, such as unstable cardiac symptoms, uncontrolled hypertension, or suspected fracture. Use the PAR-Q+ or similar screen to decide.

#858 · Collaboration

A client asks whether to stop a prescribed medication because it causes dizziness. How should you respond?

Answer

Do not advise stopping or changing it, which is outside physiotherapy scope. Document the symptom, advise the client to speak with the prescriber or pharmacist, and communicate your observations to the physician.

#859 · Collaboration

What communication format helps convey urgent clinical concerns to a physician concisely?

Answer

SBAR: Situation, Background, Assessment, Recommendation. It organizes the message so the physician quickly knows what is happening, what you think it means, and what you need.

#860 · Collaboration

What is the purpose of shared goal setting in a rehabilitation team?

Answer

It aligns all providers and the client on priorities so interventions reinforce each other. Goals should be client-centred, measurable and time-bound, and documented where the whole team can see them.

#861 · Collaboration

Why might a physiotherapist and occupational therapist run a joint session with a client?

Answer

To address overlapping tasks such as transfers or bathing, give consistent instructions, and reduce client fatigue and duplicated assessments. Combined sessions improve safety and coordination.

#862 · Collaboration

What should shared documentation in an interprofessional team avoid?

Answer

Contradicting instructions, unexplained jargon, duplicate entries, and information outside the circle of care. Entries should be clear, dated, signed and relevant to the shared plan.

#863 · Collaboration

What makes a rehabilitation team meeting effective?

Answer

A clear purpose, the client's goals up front, defined roles, each discipline reporting briefly, agreed actions with owners, and documentation of decisions. Respectful listening to every member is essential.

#864 · Collaboration

A teammate's recommendation for a mobility aid conflicts with yours. What is your first step?

Answer

Discuss it directly and respectfully, share your assessment data, listen to their reasoning, and reach a consistent plan for the client. Avoid giving the client conflicting messages.

#865 · Collaboration

Which funder commonly requires a physiotherapist to submit progress reports and treatment plans for a motor vehicle accident claim?

Answer

The auto insurer, under its provincial accident benefits scheme. Reports must be factual and objective, state the client's function and expected recovery, and respect the client's consent for release.

#866 · Collaboration

In a workplace return-to-work plan, what should the physiotherapist share with the employer?

Answer

Functional abilities, restrictions and recommended modified duties, with the client's written consent. Do not disclose diagnosis or clinical details beyond what is needed for the plan.

#867 · Collaboration

A school asks for guidance on a student's mobility limitations. What are your obligations?

Answer

Obtain consent from the parent or guardian (and the student when capable), then provide functional recommendations about participation, safety and accommodations. Share only what the school needs to implement the plan.

#868 · Collaboration

What is the best way to present an in-service to colleagues so they retain and apply it?

Answer

State clear learning objectives, keep it brief and practical, use cases and active participation, give a handout or summary, and ask for feedback. Match content to the audience's existing knowledge.

#869 · Collaboration

When teaching a student or new colleague a technique, what teaching sequence supports safe skill learning?

Answer

Explain, demonstrate, observe them performing it with feedback, then supervise gradually less. Check understanding and safety at each stage before allowing independent practice.

#870 · Collaboration

How does physiotherapy support the learning of other health professionals in a team?

Answer

By sharing expertise on mobility, movement and safe handling through brief teaching, joint sessions and clear written guidance, while respecting each profession's own scope and role.

#871 · Collaboration

What features make a patient education handout effective?

Answer

Plain language, short sentences, large clear print, relevant images, key messages first, and an action plan with contact details. Test it with a client and adapt to language and literacy needs.

#872 · Collaboration

What reading level is commonly recommended for patient education materials?

Answer

About a Grade 6 to 8 reading level. Using plain language, avoiding jargon and confirming understanding with teach-back help clients with limited health literacy.

#873 · Collaboration

What is the teach-back method?

Answer

Asking the client to explain or demonstrate instructions in their own words after teaching. It checks the clinician's explanation worked, not the client's ability, and allows immediate correction.

#874 · Collaboration

What is shared care in chronic disease management?

Answer

A coordinated approach in which the client, primary care provider and other professionals, including the physiotherapist, divide responsibilities under one agreed plan with clear communication between them.

#875 · Collaboration

What is the role of self-management programs in chronic disease care?

Answer

They build the client's knowledge, skills and confidence to manage symptoms, exercise and daily routines. Physiotherapists support them by tailoring activity, setting goals and reinforcing self-efficacy.

#876 · Collaboration

What is a care pathway?

Answer

A standardized, evidence-informed plan of care for a specific condition (for example total knee replacement) that outlines expected steps, timing and team roles, while allowing individual adjustment.

#877 · Collaboration

Why is exercise a core intervention in many chronic conditions managed across teams?

Answer

It improves function, pain, cardiovascular health and mood in conditions such as osteoarthritis, COPD, diabetes and heart disease. CSEP guidelines inform dose, with modification for individual limits and safety.

#878 · Collaboration

Which behaviour change approach do physiotherapists often use to support long-term chronic disease self-management?

Answer

Motivational interviewing: open questions, affirmation, reflective listening and summaries that draw out the client's own reasons for change. It supports autonomy and does not lecture.

#879 · Collaboration

What should a team do first when two professionals disagree about assessment findings?

Answer

Discuss them directly and respectfully, compare the methods and data each used, and consider re-assessing together. Focus on the client's best interests rather than on who is right.

#880 · Collaboration

Why might two clinicians get different findings on the same client?

Answer

Differences in timing, technique, positioning, client symptom fluctuation, tool reliability or interpretation. Standardized measures and reassessment help reconcile the findings.

#881 · Collaboration

If a disagreement about the plan cannot be resolved between peers, what is the next step?

Answer

Escalate through the agreed channel, such as a team lead, manager or case conference, while keeping the client informed and safe. Document the issue and outcome objectively.

#882 · Collaboration

A physician orders an activity you believe is unsafe for the client. What should you do?

Answer

Do not carry it out until clarified. Contact the physician, share your findings and concerns, request clarification or alternatives, and document the conversation. Client safety takes priority.

#883 · Collaboration

How should you disagree with a colleague in front of a client?

Answer

Avoid doing so. Raise differences privately afterward, then give the client a consistent message. Visible conflict can erode trust and confuse the client.

#884 · Collaboration

What are the privacy requirements when using telehealth for physiotherapy?

Answer

Use a secure, privacy-compliant platform, verify client identity, obtain consent for virtual care, confirm location and emergency contact, and follow provincial privacy law and college standards. Avoid public or unsecured apps.

#885 · Collaboration

What is a virtual case conference?

Answer

A scheduled online meeting where team members and often the client discuss a case and agree on a plan. It needs a clear agenda, consent, a secure platform and documented outcomes.

#886 · Collaboration

Which communication practices are safe for sending client information to another provider electronically?

Answer

Use encrypted or approved secure messaging or the shared electronic record, send the minimum necessary information, confirm the recipient, and avoid regular email or text unless the client has consented to the risks.

#887 · Collaboration

What is a limitation of telehealth assessment that physiotherapists must plan for?

Answer

Limited hands-on examination and objective testing. Plan safe functional tests, use a carer's help when needed, and refer for in-person assessment when findings or red flags require it.

#888 · Collaboration

What makes feedback to a colleague constructive?

Answer

It is timely, specific, based on observed behaviour, delivered privately and respectfully, focused on impact, and paired with a plan or suggestion. Ask for their perspective too.

#889 · Collaboration

How should a team member respond when receiving critical feedback?

Answer

Listen without defensiveness, ask clarifying questions, thank the person, reflect, and decide on actions. Treat feedback as a chance to improve practice.

#890 · Collaboration

What should you do if you have a performance concern about a colleague that may affect client safety?

Answer

Raise it directly with the colleague if safe to do so, and if unresolved or serious, report to the manager or the regulator. Client safety outweighs collegial loyalty. Document the facts.

#891 · Collaboration

Which language style is best when giving feedback in a team?

Answer

Use neutral, behaviour-focused "I" statements such as "I noticed the notes were missing the plan," instead of labels or blame. This lowers defensiveness and keeps the focus on solutions.

#892 · Collaboration

When should a physiotherapist escalate a team concern to management?

Answer

When a safety risk, repeated unprofessional behaviour or a system problem persists after direct discussion, or immediately for serious harm or possible misconduct. Follow organizational policy and keep records.

#893 · Communication

Which type of question invites a client to describe their experience in their own words, and when do closed questions fit better?

Answer

Open-ended questions (for example, "How has this injury affected your work?") draw out detail and concerns. Closed yes/no questions suit confirming specific facts, such as dates or medication use.

#894 · Communication

Beyond hearing the words, what does active listening involve during a client interview?

Answer

Giving full attention, using appropriate eye contact and nods, paraphrasing what you heard, and checking your understanding. It shows the client that their story matters and prevents assumptions.

#895 · Communication

Why should a physiotherapist use a deliberate pause after a client answers a difficult question?

Answer

Silence gives the client time to think and add detail, and it signals patience. Rushing to fill the gap can cut off an important disclosure about fears, pain, or home circumstances.

#896 · Communication

What is a reflection of feeling, and what is an example?

Answer

Naming the emotion you hear, such as "You sound frustrated that recovery feels slow." It shows empathy, lets the client correct you, and often opens deeper conversation.

#897 · Communication

At the start of a first visit, which steps help build trust with a new client?

Answer

Introduce yourself and your role, confirm the name the client prefers, explain what will happen in the session, and ask what the client hopes to achieve. This sets shared expectations early.

#898 · Communication

A client says "I'm fine" while guarding a limb, grimacing, and holding their breath. How do you respond?

Answer

Treat the nonverbal cues as important information. Gently name what you observe and ask an open question, because pain or worry may be underreported verbally.

#899 · Communication

Where should you position yourself when talking with a client who is seated in a wheelchair or on the plinth?

Answer

Sit or crouch at eye level, face the client, and keep an open posture. Standing over a seated client emphasizes a power imbalance and can make them feel rushed or talked down to.

#900 · Communication

What should you do before moving into a client's personal space or touching them for assessment?

Answer

Explain what you will do and why, and ask permission. Comfort with distance and touch varies by person, culture, and history, and unexpected contact can cause anxiety or withdrawal of consent.

#901 · Communication

How can eye contact differ across cultures, and what mistake should you avoid?

Answer

Direct eye contact signals respect in some cultures and disrespect in others, including some Indigenous communities. Do not assume that avoiding eye contact means disinterest, dishonesty, or lack of understanding.

#902 · Communication

What does incongruence between a client's words and their body language suggest, and what is the best response?

Answer

A mismatch, such as saying "it's okay" while tearful, suggests unspoken concerns. Check in with a gentle open question rather than assuming the meaning of the cue.

#903 · Communication

What does the SPIKES protocol provide for sharing difficult news with a client?

Answer

A step-by-step structure: Setting, Perception, Invitation, Knowledge, Emotions, and Strategy/Summary. It helps you prepare, learn what the client knows, deliver information in chunks, and respond to emotion.

#904 · Communication

Before giving difficult information such as a poor prognosis, what should you ask first?

Answer

Ask what the client already understands and how much they want to know. This tailors the message, avoids overloading them, and respects their preferences about information.

#905 · Communication

How should you discuss a lack of progress with a client?

Answer

Share the objective findings honestly, acknowledge their frustration, explore possible reasons, and agree on next steps such as changing the plan or referring. Avoid vague reassurance.

#906 · Communication

During assessment you find a red flag that needs prompt medical review. How do you tell the client?

Answer

Use calm, factual language, explain why you recommend prompt evaluation, and arrange the referral. Avoid speculating about a diagnosis you cannot confirm, and check that the client understands the urgency.

#907 · Communication

What are the core steps when disclosing an error that occurred in a client's care?

Answer

Tell the client promptly and honestly what happened, apologize, explain what is being done to address it and prevent recurrence, and document it. Follow employer and regulatory college reporting requirements.

#908 · Communication

Which plain language practices improve understanding for clients with low health literacy?

Answer

Use short sentences and everyday words instead of jargon, limit instructions to a few key points, give concrete examples, and add pictures or demonstrations. Written material should be simple and clear.

#909 · Communication

What is the teach-back method, and why is it better than asking "Do you understand?"

Answer

The client explains or demonstrates the instruction in their own words. Many clients say yes out of embarrassment, so teach-back reveals gaps and lets you re-explain without blaming them.

#910 · Communication

Which strategies help when communicating with a client who has hearing loss?

Answer

Reduce background noise, face the client in good light, speak clearly at a normal pace without shouting, avoid covering your mouth, and give written backup. Confirm that hearing aids are in use.

#911 · Communication

How should you adapt instructions for a client with cognitive impairment?

Answer

Give simple one-step instructions, repeat them, demonstrate, and use written or pictorial cues. With the client's permission, involve a caregiver, and keep the routine consistent between visits.

#912 · Communication

What adjustments help when communicating with a client who has low vision?

Answer

Use large print and high contrast materials, good lighting, and verbal description. Tell the client what you are about to do and where you are before touching or moving them or equipment.

#913 · Communication

When using an interpreter, to whom should you speak and how?

Answer

Speak directly to the client, not the interpreter, using short sentences and first person. Pause for interpretation, and watch the client's responses rather than looking only at the interpreter.

#914 · Communication

Why is a professional interpreter preferred over a family member, particularly for consent or sensitive topics?

Answer

Family members may filter or change information, lack medical vocabulary, and compromise confidentiality and the client's freedom to speak openly. A trained interpreter is neutral and bound by confidentiality.

#915 · Communication

How should you prepare with an interpreter before and after a session?

Answer

Before: brief them on the session purpose and key terms so they can prepare. After: debrief about any difficulties with understanding. Document that an interpreter was used.

#916 · Communication

What does cultural humility mean in physiotherapy practice?

Answer

Ongoing self-reflection on your own assumptions, recognition of power imbalances, and learning from each client about their values, rather than claiming to have mastered another culture.

#917 · Communication

What is cultural safety, and who determines whether it has been achieved?

Answer

Care in which the client feels respected and free from discrimination, with the client's experience defining safety. It is widely emphasized in Canada, especially in care for Indigenous peoples.

#918 · Communication

What does the OARS acronym stand for in motivational interviewing?

Answer

Open questions, Affirmations, Reflections, and Summaries. These core skills build rapport, highlight client strengths, and help the client hear their own reasons for change.

#919 · Communication

What is ambivalence in motivational interviewing, and how should you respond to it?

Answer

Ambivalence is wanting and not wanting to change at the same time. It is normal, so explore both sides without arguing for change, which can strengthen the client's resistance.

#920 · Communication

What is the righting reflex, and why should you resist it in behaviour change conversations?

Answer

The urge to fix, advise, and persuade. It tends to provoke defensiveness, so instead elicit the client's own reasons and ideas for change.

#921 · Communication

What is change talk, and how should you respond when you hear it?

Answer

Client statements that favour change, expressing desire, ability, reasons, need, or commitment. Reinforce it with affirmations and reflections and ask for elaboration, since it predicts actual change.

#922 · Communication

In motivational interviewing, why ask a client who rates importance 4 out of 10 "Why not a lower number?"

Answer

The question prompts the client to voice their own reasons for change instead of reasons against it. It is more effective than asking why they are not higher, which invites excuses.

#923 · Communication

How should you explain an exercise or procedure to a young child?

Answer

Use short, concrete language matched to their developmental level, demonstrate on a doll or on yourself first, and let the child see and touch equipment. Avoid abstract explanations and medical jargon.

#924 · Communication

Who gives consent for treatment of a young child, and what varies across Canada?

Answer

A parent or legal guardian usually consents. Provincial and territorial law determines when a mature minor can consent, and most jurisdictions rely on capacity rather than a fixed age.

#925 · Communication

What is assent, and how does it differ from consent?

Answer

Assent is a child's agreement to participate, given when they lack legal capacity to consent. Seek it and respect a child's distress or refusal where safe, while consent still comes from the legal decision maker.

#926 · Communication

How can play-based approaches improve a child's cooperation during therapy?

Answer

Turn exercises into games, let the child handle equipment first, and use the tell-show-do approach. Play builds rapport, reduces fear, and lets you observe movement and function naturally.

#927 · Communication

What should a physiotherapist do on suspecting a child is being abused or neglected?

Answer

Report to the child protection authority as required by provincial or territorial law. The duty applies to everyone, does not require proof, and overrides client confidentiality.

#928 · Communication

What does the SOAP note structure stand for, and what does it organize?

Answer

Subjective, Objective, Assessment, Plan. It organizes the client's report, measured findings, your clinical impression of progress, and the intended treatment and follow-up in a consistent order.

#929 · Communication

What belongs in the objective section of a note versus the subjective section?

Answer

Objective holds measurable findings, such as range of motion in degrees, strength grades, and outcome measure scores. Subjective records what the client reports, such as pain, symptoms, and goals.

#930 · Communication

How should you record an entry made after the fact, a late entry?

Answer

Label it clearly as a late entry, with the current date and time and the date of the event it describes, then sign it. Never backdate it to look as though it was written at the time.

#931 · Communication

How do you correct an error in a paper clinical record?

Answer

Draw a single line through the error so it remains legible, then initial and date it and add the correct entry. Never use correction fluid, erase, or obliterate the original.

#932 · Communication

What makes a treatment goal SMART, and why are functional goals preferred in documentation?

Answer

Specific, Measurable, Achievable, Relevant, and Time-bound. Functional goals, such as walking 500 metres, show meaningful change, guide the plan, and support the need for continued treatment.

#933 · Communication

What should you do before communicating with a client by email or text?

Answer

Explain the risks of unencrypted messaging, obtain the client's consent, and document it. Confirm the correct address or number and agree on what content may be sent this way.

#934 · Communication

What is the safest approach to sending sensitive health information by text message?

Answer

Avoid sending it over regular unsecured text. Use a secure, encrypted platform approved by your employer or organization, and keep messages brief and non-identifying, such as appointment reminders.

#935 · Communication

What are the privacy and boundary rules for physiotherapists on social media?

Answer

Never post identifiable client information, including photos, without written consent. Decline friend requests from clients, avoid giving individual clinical advice online, and keep a professional boundary.

#936 · Communication

Which practices protect the integrity and security of an electronic health record?

Answer

Use a unique login, never share passwords, and log out of shared computers. Records should keep an audit trail so any change to an entry can be traced, including who made it and when.

#937 · Communication

What should you do if a privacy breach, such as a misdirected email of client information, occurs?

Answer

Report it promptly to your privacy officer or employer, contain it, and notify affected clients. Follow the applicable provincial health privacy act or PIPEDA, which may require reporting to the privacy commissioner.

#938 · Communication

What should a reply letter to a referring physician include?

Answer

A concise summary of assessment findings, treatment provided, the client's response or outcome, and recommendations or follow-up. Keep it clear and relevant to the reason for referral.

#939 · Communication

What principles guide writing a report for an insurer or third party?

Answer

Be factual and objective, base statements on your documentation, include only information the client has consented to release, and avoid opinions beyond your scope. Sign with your name and credentials.

#940 · Communication

What does a discharge summary typically contain?

Answer

The reason for referral, treatment provided, the client's status at discharge compared with initial goals, and recommendations such as a home program or follow-up. It should be dated and signed.

#941 · Communication

Which wording is preferred in a professional report, "client refused" or "client declined", and why?

Answer

"Declined" is preferred, as it records the choice neutrally. Use objective, respectful language and avoid judgmental labels such as "non-compliant" or "difficult" in records and letters.

#942 · Communication

Before sending a report to another party, what must you confirm about release of information?

Answer

Confirm valid client consent or legal authority to release, and share only what the recipient needs. Verify the recipient's identity and use a secure delivery method.

#943 · Practice Management

Before starting treatment that a third-party payer will not fully cover, what must the physiotherapist discuss with the client?

Answer

Fees and the expected out-of-pocket cost, as part of informed financial consent. The client should know the charges and who pays before treatment begins, so there are no billing surprises.

#944 · Practice Management

A client's private insurer pays for only 10 visits per year. What is the physiotherapist's duty regarding the treatment plan?

Answer

Base the plan on clinical need, not the coverage limit. Tell the client about the limit, discuss options such as a home program or self-pay, and document the discussion.

#945 · Practice Management

What must a billing record or receipt for physiotherapy accurately reflect?

Answer

The services actually provided, the date, the fee charged and the provider. Billing for services not delivered, or inflating fees, is professional misconduct and may be fraud.

#946 · Practice Management

A client asks you to bill a different service code so insurance will pay. What is the correct response?

Answer

Decline. Bill only for the service actually provided. Knowingly submitting false billing information to a payer is dishonest, misconduct, and can lead to discipline by the college.

#947 · Practice Management

Why must a physiotherapist disclose any financial interest in a product or device recommended to a client?

Answer

It is a conflict of interest. Disclosure lets the client judge whether the recommendation is in their interest, and the client's needs must take priority over personal financial gain.

#948 · Practice Management

What does a clinical audit of physiotherapy records or services compare against?

Answer

A defined standard or criterion, such as a practice standard or guideline. The aim is to find gaps between actual and expected practice, then act on them.

#949 · Practice Management

Name the steps of a basic quality improvement (Plan-Do-Study-Act) cycle.

Answer

Plan the change, Do it on a small scale, Study the results against the goal, Act by adopting, adjusting or abandoning the change. The cycle then repeats.

#950 · Practice Management

Why should a service pair client satisfaction surveys with objective outcome measures?

Answer

Satisfaction reflects experience, such as communication and access, but not necessarily clinical improvement. Valid outcome measures show whether function actually changed, so both are needed to evaluate quality.

#951 · Practice Management

What is benchmarking when evaluating a physiotherapy service?

Answer

Comparing your service's outcomes or processes, such as wait times or functional gains, with those of similar services or with recognized standards, to identify where improvement is possible.

#952 · Practice Management

When reporting aggregated outcome data from a clinic for quality improvement, what privacy step is usual?

Answer

De-identify the data so individual clients cannot be recognized, and share only what is needed. Aggregate reporting protects confidentiality while still allowing service evaluation.

#953 · Practice Management

With limited treatment time, what principle guides how a physiotherapist prioritizes a caseload?

Answer

Clinical need and urgency first, such as risk, acuity and potential to benefit, applied consistently and fairly. Priority should never depend on a client's ability to pay or personal preference.

#954 · Practice Management

When is a group exercise program a reasonable way to balance client needs and limited resources?

Answer

When clients share similar goals and can be safely supervised in a group. Individual assessment still comes first, and anyone needing close one-on-one care should not be placed in a group.

#955 · Practice Management

Your clinic lacks the equipment normally used for a client's condition. What is the appropriate action?

Answer

Use a safe, evidence-informed alternative if one exists, or refer the client elsewhere. Do not provide unsafe or substandard care because of a resource shortage, and tell the client the reason.

#956 · Practice Management

How should a physiotherapist justify a request for new equipment in a clinic budget?

Answer

Link it to client outcomes, safety and use data: number of clients who benefit, evidence for effectiveness, and cost compared with alternatives. A reasoned case is stronger than personal preference.

#957 · Practice Management

What does reallocating a physiotherapist's time toward clients with higher need illustrate?

Answer

Equitable resource use: allocating time by need rather than equally to everyone. It supports fairness while keeping care safe for all clients on the caseload.

#958 · Practice Management

What is the purpose of triage criteria for a physiotherapy waitlist?

Answer

To order clients by clinical urgency, such as risk of deterioration, severity or time-sensitive conditions, using consistent, transparent criteria, rather than simply by first come, first served.

#959 · Practice Management

A client on your waitlist reports new bladder dysfunction and progressive leg weakness. What is the correct action?

Answer

Treat this as urgent: these are red flags for serious spinal pathology. Arrange prompt assessment or refer for urgent medical evaluation rather than leaving them in the routine queue.

#960 · Practice Management

Why should clients on a waitlist be given periodic communication?

Answer

They should know the expected wait, interim self-management advice, and when to seek urgent care if symptoms worsen. Clear communication maintains trust and reduces risk during the wait.

#961 · Practice Management

Is it acceptable to give priority on a waitlist to a friend or a referral source's client without clinical reason?

Answer

No. Queue-jumping without a clinical reason is unfair and a conflict of interest. Access should follow transparent criteria applied equally to all clients.

#962 · Practice Management

What should a physiotherapist do if the waitlist becomes so long that clients are at risk of harm?

Answer

Raise the concern with management or the employer, document it, advise affected clients of alternatives such as other providers, and consider interim measures. Silence is not acceptable when clients may be harmed.

#963 · Practice Management

An employer sets a productivity target that would shorten visits below safe assessment time. Which obligation prevails?

Answer

The physiotherapist's professional standards and duty to the client. Employer targets cannot override the standard of care, so raise the concern with the employer and document it.

#964 · Practice Management

Your employer asks you to document treatment you did not provide to meet billing targets. What should you do?

Answer

Refuse. Records must be accurate and reflect actual care. Falsifying documentation is misconduct regardless of employer pressure, and the physiotherapist remains personally accountable.

#965 · Practice Management

Is a physiotherapist who is an employee still personally accountable to the regulatory college?

Answer

Yes. Employment does not remove personal professional accountability. The physiotherapist must meet college standards even when an employer's policy conflicts with them.

#966 · Practice Management

If an employer policy conflicts with a practice standard, what is the first step?

Answer

Raise the concern with the employer, explain the standard and the risk to clients, and seek a solution. Document the discussion and consult the college or a professional association if unresolved.

#967 · Practice Management

Why is copying forward a previous note without reassessment an unacceptable documentation shortcut?

Answer

The record must reflect the current assessment and treatment. Copy-forward can record findings that are no longer true, misleads other providers, and fails the standard for accurate, timely documentation.

#968 · Practice Management

Who decides how long physiotherapy clinical records must be kept in Canada?

Answer

The provincial regulatory college and provincial law set retention periods, so they vary. Ontario, for example, requires ten years after the last entry, or for a minor ten years after the client turns 18. Check your own college.

#969 · Practice Management

Why do retention periods for a minor's records usually run longer than for an adult's?

Answer

The clock typically starts when the minor reaches the age of majority, not at the last visit. This preserves the record through the time the person could still raise a complaint or claim.

#970 · Practice Management

How should paper records be destroyed once the retention period ends?

Answer

In a way that makes them unreadable and irretrievable, such as cross-cut shredding or secure destruction by a bonded service. Keep a log of what was destroyed and when, without client details.

#971 · Practice Management

What must a physiotherapist do with client records when closing a practice?

Answer

Arrange for continued secure custody for the full retention period, notify clients and the college as required, and give clients a way to access their records. Records must never simply be abandoned.

#972 · Practice Management

What is a basic requirement for securely storing paper client records?

Answer

Keep them in a locked cabinet in a locked or access-controlled area, with only authorized staff holding keys. Records should never be left unattended in open areas.

#973 · Practice Management

What is an audit trail in an electronic health record?

Answer

A log showing who accessed or changed a record, what was done and when. It lets the clinic detect and investigate inappropriate access, supporting privacy and accountability.

#974 · Practice Management

Which staff should have access to a client's electronic record?

Answer

Only those who need it to provide care or perform their job, using unique logins and role-based access. Browsing records out of curiosity is a privacy breach, even for a colleague.

#975 · Practice Management

A client asks to see their physiotherapy record. What is the general rule?

Answer

Clients have a right to access their own record under provincial health privacy law, with limited exceptions, and the request should be answered within the legal time frame. A reasonable fee may apply.

#976 · Practice Management

Which federal law governs personal information in private-sector commercial activity where no provincial health privacy act applies?

Answer

PIPEDA, the Personal Information Protection and Electronic Documents Act. Provinces with substantially similar health privacy laws apply those to health information instead.

#977 · Practice Management

What is the correct first response if a client's records are lost or accessed without authorization?

Answer

Contain the breach, assess the risk, notify the affected client and the privacy commissioner as the applicable law requires, notify the college if required, and document the response and corrective steps.

#978 · Practice Management

What are the safeguards for sending client information by email or fax?

Answer

Confirm the recipient and number or address, use encryption or secure systems, limit the content to what is necessary, and include a confidentiality notice. Unencrypted email carries a risk of disclosure.

#979 · Practice Management

What does WHMIS 2015 require employers to provide for hazardous products at work?

Answer

Supplier and workplace labels, safety data sheets (SDS) that are accessible, and worker education and training. Workers need to know the hazards and how to handle the products safely.

#980 · Practice Management

How many sections does a WHMIS 2015 safety data sheet have?

Answer

Sixteen sections in a standardized format. They cover identification, hazards, composition, first aid, firefighting, spill response, handling and storage, exposure controls and more.

#981 · Practice Management

A physiotherapist is struck by a used needle. What is the immediate first aid?

Answer

Let the wound bleed freely, wash with soap and water, do not squeeze or scrub, then report immediately and seek prompt medical assessment, since post-exposure treatment is time-sensitive.

#982 · Practice Management

Where should used sharps be disposed of in a clinic?

Answer

In a puncture-resistant, leak-proof sharps container located close to the point of use. Never recap needles by hand and never overfill the container.

#983 · Practice Management

What is the principle behind good ergonomics when providing hands-on treatment?

Answer

Reduce the risk of work-related musculoskeletal injury by adjusting plinth height, keeping a neutral spine, staying close to the client, using body weight and varying postures, rather than working in sustained awkward positions.

#984 · Practice Management

A client becomes verbally aggressive and threatens you during a session. What is the appropriate response?

Answer

Prioritize safety: stay calm, create distance, leave the room if needed and call for help. Then report the incident, document it and follow the workplace violence policy.

#985 · Practice Management

What does Canadian occupational health and safety law require employers to have regarding workplace harassment and violence?

Answer

A written policy and program, a reporting process, and training for workers. Workers have the right to a workplace free from harassment and violence.

#986 · Practice Management

What is the purpose of reviewing an incident in a clinic after an adverse event or near miss?

Answer

To find system causes and prevent recurrence rather than to blame an individual. The review should be documented, with any needed changes made and shared with staff.

#987 · Practice Management

Why must clinic equipment have a maintenance and inspection log?

Answer

It shows that equipment such as electrical modalities and plinths was checked and serviced as scheduled. Faulty equipment is a safety risk, and the log is evidence of due diligence.

#988 · Practice Management

What should a physiotherapist do with a piece of equipment found to be defective?

Answer

Remove it from use immediately, tag it as out of service, report it, and have it repaired or replaced and re-inspected. Do not use it on a client until it is confirmed safe.

#989 · Practice Management

Why should a physiotherapist carry professional liability insurance, and who often requires it?

Answer

It protects the clinician and clients if a claim of negligence is made. Regulatory colleges generally require it as a condition of registration, even for employed physiotherapists.

#990 · Practice Management

What should a clinic's emergency plan cover?

Answer

Procedures and roles for medical emergencies, fire and evacuation, with posted exits, accessible emergency equipment such as a first aid kit, and regular staff drills and training.

#991 · Practice Management

When a physiotherapist delegates a task to a support worker, who remains accountable for the outcome?

Answer

The physiotherapist. They must ensure the worker is competent for the task, give clear instructions, provide appropriate supervision and monitor the client's response.

#992 · Practice Management

Which tasks must a physiotherapist never delegate to support personnel?

Answer

Those requiring the physiotherapist's professional judgment, such as assessment, diagnosis, treatment planning, and reassessment or discharge decisions. Only suitable, defined tasks may be delegated.

#993 · Assessment and Diagnosis

How is the Thessaly test performed, and what does a positive result suggest?

Answer

The client stands on the affected leg with the knee bent about 20 degrees and rotates the body in and out. Joint-line pain, locking or catching suggests a meniscal tear.

#994 · Assessment and Diagnosis

What are the Ottawa knee rule findings that call for a knee X-ray after acute injury?

Answer

Age 55 or older, isolated tenderness of the patella, tenderness at the head of the fibula, inability to flex to 90 degrees, or inability to take four steps both at injury and in assessment.

#995 · Assessment and Diagnosis

What finding makes a Dix-Hallpike test positive in posterior canal BPPV?

Answer

Vertigo with torsional, upbeating nystagmus after a short delay, which builds and then fades within about a minute, and weakens when the test is repeated.

#996 · Care Planning

What are the usual settings for conventional (high-frequency) TENS for pain relief?

Answer

High frequency, roughly 80 to 120 hertz, short pulse duration, and sensory-level intensity: a strong but comfortable tingling without a visible muscle contraction, applied around the painful area.

#997 · Client Safety and Client-Centred Care

What are the four moments for hand hygiene in Canadian health care settings?

Answer

Before initial contact with the client or their environment, before an aseptic procedure, after risk of body fluid exposure, and after contact with the client or their surroundings.

#998 · Client Safety and Client-Centred Care

Which personal protective equipment does droplet precaution require when working within two metres of a client?

Answer

A surgical or procedure mask plus eye protection, with a gown and gloves for contact, and the client placed in a single room where possible, so droplets do not reach your mucous membranes.

#999 · Professional Responsibilities

When may a physiotherapist disclose a client's personal health information without consent?

Answer

When the law requires it, such as mandatory reporting of suspected child abuse, or when there is a risk of serious imminent harm. Share only the minimum information needed and document the reason.

#1000 · Professional Responsibilities

What makes consent to physiotherapy treatment valid?

Answer

It must be voluntary, given by a client with capacity, specific to the proposed treatment, and informed about its nature, benefits, risks, alternatives and the result of refusing. The client may withdraw it at any time.